

Osteoporosis: Risk Assessment 2026

Consultation on draft guideline - Stakeholder comments table
13 January to 23 February 2026

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Agile	Guideline	7	1.3.1	Most hospitals don't do routine DXA in over 75 yr olds, how would this recommendation apply/change the current situation	Thank you for participating in the consultation process. The committee agreed that DXA is still the best approach for assessing fracture risk for this group unless it is not tolerated, not technically feasible, or not needed for treatment or monitoring decisions. If a person has had a fracture they are at increased risk and may need treatment. The BMD t score would help aid that decision on whether to offer treatment and which treatment to offer.
Agile	Guideline	7	1.3.6	If both the scores come out with such differing determination of risk, then is there not an argument to use both and then select the one that says the higher risk or lower risk depending on the service agenda.	Thank you for participating in the consultation process. The committee recommend using either the FRAX or QFracture tool and do not think there would be benefit to using both tools. This would increase appointment times. The guideline recommends a full clinical risk assessment alongside the risk prediction tool to ensure all risk factors are considered.
Amgen	Guideline	General	General	Amgen welcomes the opportunity to comment on the draft guideline <i>Osteoporosis: risk assessment</i>. We appreciate NICE's continued commitment to evidence-based guidance development and the opportunity to contribute to this consultation. Overall, Amgen supports the guideline's focus on improving identification of people at increased risk of fragility fractures, earlier intervention, and consistent use of validated risk assessment tools across primary and secondary care. We agree that strengthening risk assessment is central to preventing fractures and reducing long-term burden on patients and the NHS.	Thank you for participating in the consultation process and for your comment. We have responded to your comments in turn.

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				Amgen considers that the draft recommendations broadly reflect current clinical practice and NICE methodological standards. In particular, we welcome: • The emphasis on systematic fracture risk assessment in both men and women, • Recognition of age, prior fracture history, and clinical risk factors beyond bone mineral density alone, • Alignment with existing NICE guidance on osteoporosis management and fracture prevention. However, we believe several areas would benefit from clarification to support consistent implementation across the NHS.	
Amgen	Guideline	General	General	Amgen supports the overall direction of the draft guideline and believes it provides a strong foundation for improving osteoporosis risk assessment. We encourage NICE to further strengthen clarity around risk stratification, pathway integration, and implementation considerations to maximise real-world impact. We thank NICE for the opportunity to comment and would be pleased to provide further clarification if helpful.	Thank you for participating in the consultation process. The recommendations have been updated throughout the guideline to make the pathway clearer.
Amgen	Guideline Consultation - Equality and	General	General	Amgen supports NICE's consideration of equality impacts. We encourage further emphasis on groups at risk of under-identification or undertreatment, including: • Older people living with frailty,	Thank you for participating in the consultation process. People with frailty and limited mobility are included as examples in the recommendations and discussion. People living in care homes are not specific to osteoporosis and their risk of fracture is

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	Health Inequalities			- People in care homes, - Individuals with limited mobility or cognitive impairment, - Socioeconomically disadvantaged populations who may have reduced access to diagnostic services. Explicit recognition of these groups within the guideline would help mitigate the risk of widening health inequalities.	covered by the risk factors listed in this guideline. People in care homes are covered in the NICE guideline on Falls where their risk of falls is addressed. That guideline is linked to as a key factor in this guideline's risk assessment recommendations. Socio-economically disadvantaged populations and access to bone health services have been identified within the equality impact assessment and discussed in the guideline discussion in evidence report A.
Amgen	Guideline Consultation - Implementation Considerations	General	General	We note that some draft recommendations may be challenging to implement in settings with limited access to dual-energy X-ray absorptiometry (DXA) scanning or specialist services. NICE may wish to further clarify acceptable approaches in such contexts, including the role of clinical risk factors alone where access to diagnostics is constrained. In addition, we suggest that implementation guidance should acknowledge workload and capacity pressures in primary care, particularly in relation to repeat assessments, monitoring, and referral pathways. There are also capacity constraints around specialist workforce availability, particularly relevant for the administration for parenteral therapies commonly needed for treatments within the osteoporosis pathway.	Thank you for participating in the consultation process. The NICE implementation team will consider these issues where relevant support activity is being planned. This issue related to the availability of scanners will hopefully be addressed by the government announcement of 13 extra DXA scanners in 2025, and a further 20 scanners promised in March 2026 with the aim to help cut waiting times.
Amgen	Guideline Consultation -	General	General	We encourage NICE to ensure that the guideline clearly distinguishes between patients at low, high, and very high	Thank you for participating in the consultation process. This part of the guideline, part 1, only covers who is at risk and who

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	Risk Stratification and Pathway Integration			fracture risk, and that the implications of these risk categories for subsequent management are explicit. In practice, variability in interpretation of "high risk" can lead to delays in treatment initiation or inappropriate sequencing of therapies. Clearer articulation of how risk assessment should inform downstream treatment decisions would support more equitable and timely care, particularly for individuals at imminent fracture risk.	will need fracture. Part 2 will aim to address stratifying people into different risk groups and recommending which treatments are suitable in which groups.
Aneurin Bevan University Health Board	Evidence Review E	6	14	to mention REMS scan as well	Thank you for participating in the consultation process. This evidence review did include quantitative ultrasound and although not explicitly listed in the protocol it did include REMS. Please note that no REMS effectiveness or cost-effectiveness studies were identified for review E.
Aneurin Bevan University Health Board	Evidence Review H	017	023	General comment: Highlighting the evidence and cost for DEXA with TBS	Thank you for participating in the consultation process. DEXA with TBS was not part of the protocol for when to risk assess people who are not on treatment. It is part of the protocol for monitoring people who are using pharmacological treatment and this review will be published in part 2 of the guideline. Evidence for DEXA with TBS as part of risk assessment has been referred to in the clinical benefit and harm section for DXA (1.2.3.1) in Evidence Report D. We have also made a research recommendation covering this.

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BAME Health Collaborative Charity (BHC)	Guideline	04	1.06	Consider including patients with long-term formal detention/hospitalisation who have a severe mental illness and their composite risk factors of other lifestyle and psychotropic medication use. <i>As well as considering concealing clothing, skin colour etc, note that there is no agreed standard range of 25-hydroxyvitamin D levels agreed across age and ethnicity (particularly dark skinned south Asian and black African ancestry, where some studies suggest a narrow range of 50-100nmol/L relatively safer. Reference: (Fuleihan,.GE et al Serum 25-hydroxyvitamin levels: variability, knowledge gaps, and the concept of a desirable range, Journal of Bone Mineral Research, 2015 vol 30 pages 1119-1133.)</i>	Thank you for participating in the consultation process. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors. Medicines associated with increased fracture risk are included in the table.
BAME Health Collaborative Charity (BHC)	Guideline	04	1.1	In considering fragility fracture risk, the list should specify people with chronic serious mental illness with a likely composite risk associated with long-term antipsychotic, antidepressant, mood-stabilisers used both on license- (valproate, lamotrigine, carbamazepine) and off-license (gabapentin, oxcarbazepine, topiramate), lack of exercise, and higher tobacco smoking rate).	Thank you for participating in the consultation process. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors. Medicines associated with increased fracture risk are included in the table.

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BAME Health Collaborative Charity (BHC)	Guideline	05	Table 1	In assessing fragility fracture risk, the table (Table 1) should include under "other causes of secondary osteoporosis people on long-term prolactin-raising antipsychotics.	Thank you for participating in the consultation process. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors. Medicines associated with increased fracture risk are included in the table. People on long-term prolactin-raising antipsychotics has not been added as they were not included previously, and we have not reviewed the evidence for this as a risk factor.
BAME Health Collaborative Charity (BHC)	Guideline	06	1.1.4	Inclusion of patients with persistent hyperprolactinaemia in patients receiving long term prolactin raising antipsychotics as an at risk group of people	. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). They noted we were unlikely to find strong

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					evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors. Persistent hyperprolactinaemia in patients receiving long term prolactin raising antipsychotics has not been added as they were not included previously and we have not reviewed the evidence for this as a risk factor.
BAME Health Collaborative Charity (BHC)	Guideline	06	1.1.4	Inclusion of patients with epilepsy receiving long term treatment as an at risk group of people	Thank you for participating in the consultation process. Anticonvulsants are listed in the table as an example of medicines associated with increased one loss. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors.
BAME Health Collaborative Charity (BHC)	Guideline	06	1.2	People with a history of chronic severe mental illness have composite risk of fragility fractures by virtue of long-term antipsychotics, poor nutrition, lack of exercise, higher smoking rates and where there is concomitant use of cannabis and opioids. 20-50% of bone mass changes are associated with lifestyle risk factors. Heavy tobacco use has a direct effect on bone turnover and indirect effect by lower body weight/BMI. Heavy cannabis use is associated	Thank you for participating in the consultation process. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors. People with a history of

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				with low bone mineral density and increased risk of fractures. <i>Reference:(Sophocleous, A et al, Heavy Cannabis Use is associated with low bone mineral density and an increased risk of fractures The American Journal of Medicine, 2017 vol 130 p21-221)</i> People with a history of methadone maintenance treatment are also at risk as found to have low bone mineral density. These should be specifically mentioned. Studies should that when DXA examinations were performed in a group of patients on methadone maintenance, over 80% had below normal bone density either within the osteopenia or osteoporosis range with strong associations with concurrent tobacco and heavy alcohol use. The study showed that none of this cohort had been clinically screened for bone mineral density status and none were aware of having a clinical state of osteoporosis. <i>Reference: (Kim, TW et al, Low Bone density in patients receiving methadone maintenance treatment. Drug and Alcohol Dependence Journal ,2006 vol 85, p258-262)</i>	chronic severe mental illness or people with a history of methadone maintenance treatment has not been added as they were not included previously and we have not reviewed the evidence for this as a risk factor.
BAME Health Collaborative Charity (BHC)	Guideline	07	1.3.2	The risk prediction tool (FRAX or QFracture) data is predominantly derived from white populations, black populations have higher bone mineral density and South Asian populations have higher hip fracture risk. Is there a risk of misclassification in minority groups	Thank you for participating in the consultation process. Ethnicity was not a significant predictor of major osteoporotic fracture nor hip fracture in either FRAX or QFracture and so was not included in QFracture. FRAX is country specific and its performance is calibrated using national fracture incidence and

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					mortality data. The committee think that the risk tools are likely sufficient to classify risk on a population level. However, they acknowledge that this could risk misclassification in minority groups on an individual level. This has been added to the equality impact assessment form. We have included a recommendation to complete a full clinical risk assessment alongside a risk prediction tool so that other factors can be considered.
BAME Health Collaborative Charity (BHC)	Guideline	08	1.3.14	QFracture score does not include heavy cannabis or opioid use, tobacco	Thank you for participating in the consultation process. The committee are aware that the risk prediction tools do not take heavy cannabis or opioid use into account and have recommended doing a full clinical risk assessment alongside completing them. Tobacco use is included and in QFracture can be categorised as light, medium or heavy smoker.
BAME Health Collaborative Charity (BHC)	Guideline	08	1.3.2	When assessing fragility fracture risk using the QFracture tool, consider the need to interpret the findings in people on long-term antipsychotics or opioids carefully, and apply clinical judgement with a lower threshold to use DXA scan. Also consider people with dementia and long-term use of antidepressants and antipsychotics to treat non-cognitive symptoms (Behavioural, Psychological Symptoms in Dementia).	Thank you for participating in the consultation process. The committee agreed that every case warrants careful interpretation. The recommendation to ensure that a full clinical risk assessment is done alongside the use of risk assessment tools has been moved to the first recommendation in the section. The rationale highlights that this is because not all factors are included in the risk assessment tools.
BAME Health Collaborative Charity (BHC)	Guideline	08	1.3.9	Consider using the DXA for patients with composite risk where the QFracture tool omits these variables.	Thank you for participating in the consultation process. The committee have made a recommendation to complete a full clinical risk assessment alongside the risk prediction tools to

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					take into account that the risk tools do not include all the factors.
BAME Health Collaborative Charity (BHC)	Guideline	12	8	<i>Data indicates that while black women generally have higher bone mineral density and lower rates of bone loss compared to white and Asian women, they still experience osteoporosis. However, black women are less likely to be started on anti-resorptive treatment compared to white women, highlighting the need for better access of care. Reference:(Lau,EM et al, Romosozumab or alendronate for fracture prevention in East Asian patients: a sub analysis of phase 3 ,randomised ARCH study, Osteoporosis International ,2020 vol 31 p677-685.)</i>	Thank you for participating in the consultation process. We have noted that there is ethnic variation in bone mineral density, osteoporosis prevalence and fracture outcomes in the equality impact assessment. We have not added differences in treatment uptake as the committee noted this would depend on level of fracture risk. For the study cited, we are unable to find information about black women being less likely to start anti-resorptive treatment. The committee recommended that a full clinical risk assessment should be completed alongside risk assessment tools to consider additional risk factors.
British Association For Nutrition And Lifestyle Medicine (BANT)	EIA	1	General	3.1 Should include “Metabolic and Genetic Factors: Consideration should be given to metabolic and genetic factors influencing homocysteine levels, which differentially affect certain population groups, including older adults, people with nutritional insufficiency, and those of ethnic backgrounds with higher prevalence of MTHFR C677T polymorphism” Rationale: Prevalence of MTHFR C677T and hyper-homocysteinaemia varies by ethnicity and nutritional status. Failure to consider these factors may contribute to under-recognition of fracture risk in some groups, particularly where BMD alone underestimates skeletal fragility.	Thank you for participating in the consultation process. The guideline includes a recommendation to consider assessing fragility fracture risk in those with other causes of secondary osteoporosis. Inherited metabolic diseases known to be associated with osteoporosis, such as homocystinuria, is listed as an example of secondary osteoporosis.

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				Refs: 1) Gjesdal CG, Vollset SE, Ueland PM, et al. Plasma Total Homocysteine Level and Bone Mineral Density: The Hordaland Homocysteine Study. <i>Arch Intern Med.</i> 2006;166(1):88–94. doi:10.1001/archinte.166.1.88 2) BO Abrahamsen, Jonna Skov Madsen, Charlotte Landbo Tofteng, Lis Stilgren, Else Marie Bladbjerg, Søren Risom Kristensen, Kim Brixen, Leif Mosekilde, A Common Methylenetetrahydrofolate Reductase (C677T) Polymorphism Is Associated With Low Bone Mineral Density and Increased Fracture Incidence After Menopause: Longitudinal Data From the Danish Osteoporosis Prevention Study, <i>Journal of Bone and Mineral Research</i>, Volume 18, Issue 4, 1 April 2003, Pages 723–729, https://doi.org/10.1359/jbmr.2003.18.4.723	
British Association For Nutrition And Lifestyle Medicine (BANT)	Guideline	6	6	At 1.1.4 add extra bullet point: “impaired one-carbon metabolism resulting in elevated plasma homocysteine.” Rationale: Raised homocysteine has been independently associated with lumbar spine and hip osteoporosis and increased fracture risk. Measurement is inexpensive, widely available, and may identify a potentially modifiable contributor to	Thank you for participating in the consultation process. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors. Impaired one-carbon metabolism resulting in elevated plasma homocysteine has not

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				skeletal fragility, particularly in older adults and those with nutritional deficiencies or relevant genetic variants.	been added as they were not included previously and we have not reviewed the evidence for this as a risk factor.
British Association For Nutrition And Lifestyle Medicine (BANT)	Evidence Review A	8	28	To expand in this section: “Genetic variants affecting folate and homocysteine metabolism (for example, the methylenetetrahydrofolate reductase [MTHFR] C677T polymorphism) contribute to elevated homocysteine levels and have been associated with lower bone mineral density and increased fracture risk in some populations.” Rationale: The MTHFR C677T polymorphism reduces enzyme activity, leading to higher homocysteine concentrations, particularly in individuals with suboptimal folate status. Meta-analyses and longitudinal cohort studies suggest that the TT genotype is associated with lower BMD and increased fracture risk, particularly in women and certain ethnic populations, although heterogeneity exists. Including this as a contributory metabolic risk factor (rather than a screening recommendation) would better reflect current evidence. Refs: 1) Wang, H., Liu, C. Association of <i>MTHFR</i> C667T polymorphism with bone mineral density and fracture risk: an updated meta-analysis. <i>Osteoporos Int</i> 23, 2625–2634 (2012). https://doi.org/10.1007/s00198-011-1885-6	Thank you for participating in the consultation process and for the citations. The committee noted that the metabolic disease homocystinuria covers a small population and broadened this to include other inherited metabolic diseases with homocystinuria as an example. The list in the main text includes examples of risk factors and is not intended to be exhaustive. This guideline did not review the evidence on risk factors, including the association between TT Genotype, and BMD and fracture risk. As such, the citations provided would not have been included. The NICE Surveillance team accepts suggestions for new topics or updating existing guidance, please see: https://www.nice.org.uk/what-nice-does/our-guidance/prioritising-our-guidance-topics/topic-suggestion.

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				2) Gjesdal CG, Vollset SE, Ueland PM, et al. Plasma Total Homocysteine Level and Bone Mineral Density: The Hordaland Homocysteine Study. <i>Arch Intern Med.</i> 2006;166(1):88–94. doi:10.1001/archinte.166.1.88 3) BO Abrahamsen, Jonna Skov Madsen, Charlotte Landbo Tofteng, Lis Stilgren, Else Marie Bladbjerg, Søren Risom Kristensen, Kim Brixen, Leif Mosekilde, A Common Methylenetetrahydrofolate Reductase (C677T) Polymorphism Is Associated With Low Bone Mineral Density and Increased Fracture Incidence After Menopause: Longitudinal Data From the Danish Osteoporosis Prevention Study, <i>Journal of Bone and Mineral Research</i>, Volume 18, Issue 4, 1 April 2003, Pages 723–729, https://doi.org/10.1359/jbmr.2003.18.4.723 4) De Martinis M, Sirufo MM, Nocelli C, Fontanella L, Ginaldi L. Hyperhomocysteinemia is Associated with Inflammation, Bone Resorption, Vitamin B12 and Folate Deficiency and MTHFR C677T Polymorphism in Postmenopausal Women with Decreased Bone Mineral Density. <i>Int J Environ Res Public Health.</i> 2020 Jun 15;17(12):4260. doi: 10.3390/ijerph17124260. PMID: 32549258; PMCID: PMC7345373.	
British Geriatrics Society	Guideline	-	-	For Part 2 of guidance review, which will focus on people with a learning disability, would like you to be aware of this piece of work - https://pubmed.ncbi.nlm.nih.gov/30175427/	Thank you for participating in the consultation process and for the citation.

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British Geriatrics Society	Guideline	27	20	Aging should be ageing - aging is american english and as NICE is british, british english should be used throughout	Thank you for participating in the consultation process. We agree and have amended this.
British Society for Rheumatology	Guideline	General	general	Although we welcome these guidelines and recognise their many strengths, we ask NICE to consider the target audience (primary care) and the potential for confusion, which could hinder effective implementation. It is difficult to translate into a clear operational flowchart, which is likely to reduce its usability in everyday practice, and has several marked differences to NOGG, some of which are problematic and are highlighted in comments below numbered {2,7,11,12,13,15,18,20}	Thank you for participating in the consultation process. We have created a visual summary to go with the recommendations. Each comment is responded to in turn.
British Society for Rheumatology	Guideline	General	General	A recent international consensus study recommends using the term osteoporotic fracture instead of fragility fracture.	Thank you for participating in the consultation process. The committee discussed the terminology used within the guideline and noted that the use of fragility fracture is widely accepted and understood in the literature. The term is defined within the NICE guideline and includes the fact that fragility fractures occur from causes other than osteoporosis such as other bone diseases or malignancy. Therefore, the committee did not think it would be appropriate to use the term osteoporotic fracture.
British Society for Rheumatology	Guideline	General	General	It would be helpful to clearly define fragility fracture/osteoporotic fracture so that Primary care healthcare professionals do not need to rely on a secondary care code of fragility fracture (or osteoporotic fracture) to take action.	Thank you for participating in the consultation process. The guideline includes a section that defines terms that have been used in the guideline. Fragility fractures have been defined as fractures associated with osteoporosis, typically caused by low impact injuries that would not normally cause a fracture (such as a fall from standing height). Fragility fractures can occur

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					spontaneously, in people with no history of injury. Most vertebral fragility fractures are not caused by falls, instead happening after activities involving lifting, twisting, or bending. Fragility fractures most commonly affect the spine (vertebral column), hip, shoulder (proximal humerus), and wrist (distal forearm). But fragility fractures can happen in any bone, and some fractures (for example pelvis and rib fractures) are just as strongly associated with reduced bone strength as the most common fragility fractures. Fragility fractures can occur from causes other than osteoporosis such as other bone diseases or malignancy.
British Society for Rheumatology	Guideline	7/8	general	Clarity would be improved by placing the recommendations about DXA together (indications for DXA are: x,y,z) and all the recommendations about risk assessment together (including when and when not indicated). At the moment is confusing and difficult to follow	Thank you for participating in the consultation process. The recommendations have been restructured and a visual summary created to go with them.
British Society for Rheumatology	Guideline	4	1.1.2	Inclusion of these groups (listed below) will lead to lower value assessments and potential overwhelm healthcare systems diverting assessment from patients at higher risk: 1. Assessing fracture risk in all women aged 65 and over and all men aged 75 and over 2. Women 50 - 64 & men 50- 74 who are smokers, alcohol > 14 units per week, diabetes, COPD, CKD (which should instead be limited to GFR3b-5) anti-convulsants, SSRI, PPI, antiretroviral agents other than TDF	Thank you for participating in the consultation process. The recommendations are based on the previous version of the guideline and are similar to those listed in other guidelines and the committee do not think this will overwhelm the system. We liaised with the National Screening Committee (NSC) before the consultation. It was agreed that the recommendation to assess fracture risk in all women aged 65 and over and all men aged 75 and over relates to case finding and not screening. We will continue to liaise with the NSC to ensure there is no contradiction between our two organisation's guidance.

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				We also note assessing fracture risk in all women aged 65 and over and all men aged 75 and over does not align with recommendations from National Screening committee	
British Society for Rheumatology	Guideline	4	11	Different age ranges for men and women make clinical sense but make the guideline more confusing and complex to implement	Thank you for participating in the consultation process. The recommendations have been amended and simplified. A visual summary accompanies the recommendations to make them easier to follow.
British Society for Rheumatology	Guideline	5	Table 1	Key risk factors missing include neurological disease such as Parkinsons Disease and parkinsonism, dementia.	. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). Parkinson's Disease and Parkinsonism have been included in the table. Dementia has not been added as we have not reviewed the evidence for this as a risk factor. The list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors.
British Society for Rheumatology	Guideline	5	Table 1	Key risk factor missing is Systemic Lupus Erythematosus which has a similar inflation to fracture risk as Rheumatoid arthritis. We do not consider this is adequately covered by 'other inflammatory arthropathies'. Furthermore, risk of fracture in psoriatic arthritis is minimal	Thank you for participating in the consultation process. Systemic Lupus Erythematosus (SLE) has been added as an example of other inflammatory arthropathies and reference to psoriatic arthritis has been deleted.

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British Society for Rheumatology	Guideline	5	Table 1	CKD – Stage should be clarified Stage 3b-5	Thank you for participating in the consultation process. In line with the 2021 NICE guideline NG203 Chronic kidney disease: assessment and management (see recommendation 1.12.3), the table has been amended to clarify that CKD stages 4 and 5 (eGFR<30 mL/min) are associated with increased fracture risk.
British Society for Rheumatology	Guideline	7	2	Rec 1.3.1 Restricting DXA on basis of age is ageist and should be removed	Thank you for participating in the consultation process. The recommendation has been amended to apply to everyone aged 30 and over. For those under the age of 30 it is recommended that specialist advice is sought.
British Society for Rheumatology	Guideline	8	1.3.7	Should mention not just HRT but that all previous osteoporosis treatment alters fracture risk and should be taken into account when doing a fracture risk assessment	Thank you for participating in the consultation process. These recommendations are aimed at people who have not already had osteoporotic treatments. HRT was flagged by the committee because many women being assessed for fragility fracture will also be taking systemic HRT.
British Society for Rheumatology	Guideline	8	1.3.8	Clinical judgement should be used to decide how to assess fragility fracture irrespective of age.	Thank you for participating in the consultation process. The committee agree and the recommendations in this section on risk prediction tools have been simplified and this recommendation has been removed.
British Society for Rheumatology	Guideline	8	1.4.1	10% threshold would mean all ≥70 would automatically have a DXA without any risk factors Increasing burden on DXA. This is not dissimilar to FRAX without BMD which would also advocate DXA for patients with a fracture risk of about 10% (and lower in younger age groups).	Thank you for participating in the consultation process. The committee agreed this is best and current practice and did not find evidence to suggest changing this recommendation. This recommendation is to 'consider' BMD measurement with a DXA scan so may not be applied to all people with a 10-year risk of major osteoporotic fracture of 10% or more.
British Society for Rheumatology	Guideline	8	1.4.2	DXA is not needed when starting treatment for all patients, for many patients it may be possible but often not of high clinical	Thank you for participating in the consultation process. The committee agreed that although DXA is not always needed to

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				value. Will probably encourage patients to put off starting treatment until they have had a DXA resulting in unnecessary delay in initiating therapy.	start treatment a baseline BMD score is usually needed to compare against a later score to assess whether treatment has worked. A caveat has been made in the recommendations considering treatment without a BMD score if DXA is not tolerated, technically feasible, or needed for treatment or monitoring decisions. The committee also noted a BMD score is an important criterion for deciding whether treatment is recommended.
British Society for Rheumatology	Guideline	8	7	Suggest remove 'consider measuring BMD with a DXA scan' and amend line 6 to read 'seek specialist advice if you feel DXA may be required to further assess fracture risk'.	Thank you for participating in the consultation process. The recommendation has been updated and no longer includes consideration of DXA scans. The recommendation has changed to 'Seek advice from a specialist when assessing fragility fracture risk in people: aged under 30 or people with a suspected rare bone disease associated with fragility fractures, such as osteogenesis imperfecta or pregnancy-associated osteoporosis.' For people under 30 the committee agreed it is important to treat the underlying cause before any osteoporosis treatments are considered. The committee agreed that the specialist would recommend DXA if appropriate.
British Society for Rheumatology	Guideline	9	1.5.1	VFA in all patients over 50 years with no other risks is not evidenced based and potentially low value. Whilst this is an admirable target, the reality of adding 5 minutes scan time to each patient for purpose of VFA means that less DXA scans would be done each day increasing	Thank you for participating in the consultation process. The committee discussed this and did not agree that DXA capacity would be reduced because it was considered feasible to accommodate the additional time within the existing DXA slot in most cases. This was considered achievable because the time

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				backlogs further. VFA should remain targeted to those at higher risk of having a vertebral fracture (e.g. steroids, significant height loss, low vertebral T score etc).	to do the actual scan is a relatively small proportion of the overall time slot which includes time to meet and prepare the attendee. They agreed that there may be issues with reporting capacity due to additional time requirements and this would need to be considered when implementing the recommendation. To mitigate the impact the committee revised their recommendation from all people having DXA over 50 to men aged 70 and over and women aged 60 and over, with use under these ages limited to those with additional risk factors.
British Society for Rheumatology	Guideline	9	1.5.2	A definition of 'exceptionally' low BMD is needed for equitable implementation	Thank you for participating in the consultation process. The recommendation has been amended to read 'exceptionally low BMD for their age (for example, a Z-score of less than -2)'.
British Society for Rheumatology	Guideline	9	9	'Not needed for treatment decisions or monitoring' should be clarified	Thank you for participating in the consultation process. The recommendation has been updated with a cross reference to a more detailed recommendation in the treatment criteria section. Further information is also given in the rationale and people living with frailty is used as an example.
British Society for Rheumatology	Guideline	11	1.7.2	'Experienced' menopause is a potentially confusing term as it implies people have been symptomatic. Please use the term postmenopausal instead	Thank you for participating in the consultation process. The term 'experienced menopause' is aligned with NICE's style of writing and follows similar wording used in the menopause guideline.

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British Society for Rheumatology	Guideline	11	1.7.2	No inclusion of fracture risk incorporating DEXA values as a treatment threshold – which has value as giving patients the most accurate assessment of their risk, and our way as clinicians to stratify who gets what treatments. It is potentially a retrograde step to go back to T score thresholds. Unclear as to the justification of -1.5 as a BMD T score threshold here, yet -1.0 on line 26. This does not appear to be explained. NOGG intervention thresholds are incorporated into the NICE TAs for abaloparatide and should be included. Some patients we currently consider as very high fracture risk due to other factors eg multiple non hip non vertebral fractures would potentially be excluded from this approach.	Thank you for participating in the consultation process. The committee agree that the use of an age-based threshold in FRAX-NOGG to determine eligibility for pharmacological treatment leads to too few older people being treated and too many young people being treated because there is unclear long-term benefit for these treatments for a relatively small reduction in absolute risk. In contrast, the NICE approach is based on the premise that treatment is best targeted to people at high enough levels of risk who are likely to experience an absolute risk reduction in fracture that outweighs possible adverse events. It also incorporates BMD t score criteria in addition of absolute fracture risk as the committee agree this is important to ensure treatment is targeted at those most likely to achieve a risk reduction. However, although BMD score is a key criterion they also recommend treatment is considered without the need for a BMD score for people have a previous hip or vertebral fracture, people on very-high dose steroids (around 15mg per day), where the score is not needed for treatment decisions (e.g. elderly people living with frailty who have a few key risk factors, and where doing the DXA is not tolerated or feasible). The BMD score of <-1.0 has been removed following stakeholder consultation.

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British Society Of Skeletal Radiologists (Bssr)	Guideline	General	General	On behalf of the BSSR we are happy to support and endorse these guidelines and would look forward to be listed amongst the endorsers.	Thank you for participating in the consultation process.
Building Bones (Building Health Technologies Limited)	Guideline	General	General	Whole guideline (Part 1). We recognise that this draft guideline focuses on fracture risk assessment, identification of vertebral fractures and criteria for pharmacological treatment. However, the guideline does not include any explicit recommendations on non-pharmacological management for people at risk of fragility fracture or diagnosed with low bone density. We recommend that Part 2 of the guideline includes clear, explicit and actionable recommendations on non-pharmacological interventions, rather than these being treated as background or implicit advice.	Thank you for participating in the consultation process. Part 2 of the guideline will include reviews on the non-pharmacological interventions of exercise interventions and adjunctive calcium/vitamin D treatment.
Building Bones (Building Health Technologies Limited)	Guideline	General	General	We note that in previous consultation responses on the scope of this guideline, stakeholders raised concerns that non-pharmacological approaches, including exercise, are often de-prioritised in practice, and that clearer guidance is needed to support clinicians. NICE's published responses acknowledged exercise as a treatment within the scope of this guideline. We therefore recommend that Part 2 includes explicit recommendations on exercise as a core component of osteoporosis management.	Thank you for participating in the consultation process. Part 2 of the guideline will include a review on the clinical and cost-effectiveness of exercise interventions.
Building Bones (Building Health Technologies Limited)	Guideline	General	General	We recommend that Part 2 of the guideline includes structured recommendations on exercise for bone health, including progressive resistance training, impact-loading exercise and balance training (as appropriate based on an	Thank you for participating in the consultation process. Part 2 of the guideline will include a review on the clinical and cost-effectiveness of exercise interventions.

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				assessment), for people with low bone density, osteoporosis or at risk of fragility fracture. Exercise should be positioned as a core component of management, rather than an optional adjunct to pharmacological treatment.	
Building Bones (Building Health Technologies Limited)	Guideline	General	General	The guideline should explicitly recommend falls risk assessment and referral to evidence-based falls prevention exercise programmes for people identified as being at increased fracture risk. Clear recommendations are needed to support consistent implementation across services and to reduce unwarranted variation in access to falls prevention interventions.	Thank you for participating in the consultation process. The NICE guideline NG249 <i>Falls: assessment and prevention in older people and in people 50 and over at higher risk</i> is referred to in recommendation 1.1.2 and it is not NICE's general policy to repeat the recommendations of another NICE guideline.
Building Bones (Building Health Technologies Limited)	Guideline	General	General	We recommend that the guideline explicitly states that non-pharmacological interventions should be offered not only alongside pharmacological treatment, but also to people who decline pharmacological treatment and to those who are not eligible for drug therapy. Without explicit guidance, these groups risk receiving limited or inconsistent support.	Thank you for participating in the consultation process. Part 2 of the guideline will include reviews on the clinical and cost-effectiveness of exercise interventions and the addition of vitamin D/calcium to other pharmacological interventions. As such, if the committee recommends these interventions alongside pharmacological interventions, and/or for people who are not having treatment, any needed changes to the existing recommendations as proposed in Part 1 of the guideline will be made.
Building Bones (Building Health Technologies Limited)	Guideline	General	General	Implementation challenge: If non-pharmacological interventions such as exercise and falls prevention are not clearly specified in the guideline, there may be challenges in commissioning and service provision, particularly in primary and community care. Explicit recommendations and clear positioning of these interventions as part of standard care	Thank you for participating in the consultation process. Part 2 of the guideline will include reviews on the clinical and cost-effectiveness of exercise interventions and the addition of vitamin D/calcium to other pharmacological interventions. As such, if the committee recommends these interventions for people who are not having treatment, any needed changes to the existing recommendations as proposed in Part 1 of the

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				would support implementation and reduce variation in practice.	guideline will be made. Regarding falls prevention, NICE's guideline NG249 <i>Falls: assessment and prevention in older people and in people 50 and over at higher risk</i> is referred to in recommendation 1.1.2.
Building Bones (Building Health Technologies Limited)	Guideline	General	General	We recommend that the guideline recognises the role of technology-enabled and digital approaches in supporting delivery of non-pharmacological interventions, including exercise, education and ongoing support for people at risk of fragility fracture, those at a high fall risk and those with low bone density. Remote or hybrid delivery models may improve access for people who are unable to attend face-to-face services due to mobility, cost, geographical, work or caring constraints. It may also improve adherence to programs and enable remote monitoring and follow ups. Explicit recognition of digital and technology-enabled approaches may support implementation of exercise and falls prevention interventions, reduce inequalities in access, and increase patient choice.	Thank you for participating in the consultation process. Part 2 of the guideline will include reviews on the clinical and cost-effectiveness of exercise interventions. This includes investigating supervised exercises delivered online.
Cardiff and Vale University Health Board	Equality Impact assessment	4	4	Regarding section 3.4 on page 4. 'Age and disability'. ' DXA is not always technically feasible or tolerated (for example in older people living with frailty...). You may want to consider avoiding drawing a direct association with age and focusing more on an individual's frailty and function as a facilitator/inhibitor to them undergoing DXA. Some older people have very good function whilst some younger people do not.	Thank you for participating in the consultation process. The committee agree and 'older' has been removed from this statement so that it now focuses on frailty.

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Cardiff and Vale University Health Board	Draft guideline	4	7	The cutoff for high dose steroids being 5mg prednisolone equivalent rather than 7.5mg. The evidence review A document states that a decision was made to use 5mg 'as it aligns with the dose used in the FRAX prediction tool' However, FRAX recommend adjusting the hip fracture probability from a FRAX score by 20% and the MOF probability by 15% for doses equal to or above 7.5mg prednisolone equivalent.	Thank you for participating in the consultation process. The 5mg cut off recommended is the same as that in the FRAX tool which has the instructions 'Enter yes if the patient is currently exposed to oral glucocorticoids or has been exposed to oral glucocorticoids for more than 3 months at a dose of prednisolone of 5mg daily or more (or equivalent doses of other glucocorticoids)'. The committee agreed that people who use systemic glucocorticoids are likely to be at significantly higher risk of fragility fracture. So, they should have a risk assessment. FRAX assesses risk when glucocorticoids (such as prednisolone 5 mg or more daily or equivalent) are used for more than 3 months and the committee agreed to use this as a starting point for risk.
Cardiff and Vale University Health Board	Draft guideline	7	5	I am not sure it is fair or equitable to exclude someone over the age of 90 from a DXA purely based on age. These patients can be high risk of fracture and an anabolic may be suitable.	Thank you for participating in the consultation process. The recommendation has been amended to apply to people over 90 and notes some limitations for each of the fracture risk prediction tools for this age group. The mention of age 90 is to ensure healthcare professionals are aware of the limitations of the risk assessment tools at this age. It is not meant to deprive that group from risk assessment.
Cardiff and Vale University Health Board	Draft guideline	9	13	Whilst I agree with considering a VFA when doing a DXA in people aged 50 and over, this will increase the amount of time it takes to complete a DXA, which in turn will lead to reduced capacity within DXA services (i.e it'll take longer to do each scan and therefore less patients will be scanned per day).	Thank you for participating in the consultation process. The committee discussed this and did not agree that DXA capacity would be reduced because it was considered feasible to accommodate the additional time within the existing DXA slot in most cases. This was considered achievable because the time to do the actual scan is a relatively small proportion of the

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					overall time slot which includes time to meet and prepare the attendee. They agreed that there may be issues with reporting capacity due to additional time requirements and this would need to be considered when implementing the recommendation. To mitigate the impact, the committee revised their recommendation from all people over 50 having DXA to men aged 70 and over and women aged 60 and over, with use under these ages limited to those with additional risk factors.
Cardiff and Vale University Health Board	Draft guideline	10	12	Patients with scoliosis may still suffer vertebral fractures, therefore it would be reasonable to consider a VFA first if a fracture is suspected and if the VFA (AP and lateral images) insufficient, consider other imaging modalities rather than going straight to an alternative imaging modality.	Thank you for participating in the consultation process. The committee agree. The recommendation only advises against VFA if the scoliosis is likely to affect the interpretation, in other circumstances a VFA can be used.
Cardiff and Vale University Health Board	Draft guideline	11	17	The inclusion of treating patients with a BMD T score of -1.5 or less plus one of the listed risk factors will cast a wide net with regards to patients who can be treated. Whilst this is obviously beneficial to patients, this will make it far more complicated for a nurse lead fracture liaison service to identify those that should be treated and with what treatment to use. This is why the current FRAX/NOGG +BMD measurement approach is so useful for nurse lead FLS as it helps to focus and refine decision making, including helping to target who should receive an anabolic through identifying very high risk,	Thank you for participating in the consultation process. The committee agreed that it is important for this group of people should be considered for treatment. Anyone who has had a fragility fracture is likely to be at increased of fracture and therefore needs preventative treatment against a second fracture. The other two criteria relate to medicines known to affect bone density. As this group already has a relatively low bone density the medicines are likely to exacerbate this and increase the risk of fracture. Very high fracture risk patients will

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				high risk etc. Whilst these recommendations may help doctors in a clinic, it may create more uncertainty for allied healthcare professionals who rely on defined protocols to guide treatment decisions. In addition, clinical technologists who report DXA's can currently use FRAX/NOGG to guide recommendations. Reporting for non-medical DXA practitioners may become far more complex, leading to delays.	be defined in the multiple technology appraisal (MTA) which will be incorporated into part 2 of this guideline. The committee agreed that the decision to treat requires careful consideration weighing up several factors including the individual's risk factors (not just those in FRAX or QFracture), their risk score, BMD (if available) and the number and site of previous fractures. The committee agree that these decisions are complex and need to be interpreted with clinical knowledge (whichever tool is used). Clinical understanding is important for a meaningful discussion to make a shared decision on whether to treat. The committee do not think that this is more complicated than the FRAX-NOGG approach which also assumes a reasonable amount of background understanding to apply them appropriately.
Cardiff and Vale University Health Board	Draft Guideline	11	3	Treatment decisions may be more complicated for non-medical practitioners (e.g nurses in a fracture liaison service) and may affect the effectiveness of current services. As an example; if a patient is seen in an FLS and that patient cannot have a DXA or there is likely to be a large delay in getting a DXA, FRAX/NOGG allows assessment of risk factors and provides a clear intervention threshold from which the nurse can recommend treatment. It can of course be supplemented by other risk factors not in FRAX but ultimately	Thank you for participating in the consultation process. The recommendations have been updated to make them clearer and a visual summary of the pathway accompanies the guideline. The committee agreed that knowing who will benefit from osteoporosis treatments is a complicated process and anyone making the decision should be appropriately trained. There will need to be clinical judgement on top of recommendations

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				it allows non-medical practitioners a clear framework within which to recommend treatment. These recommendations may create far more uncertainty and may make it more difficult to create clear protocols for non-medical staff to follow.	regardless of whether the NICE or NOGG approach is used to deciding treatment.
Cardiff and Vale University Health BoardThi	Draft guideline	21	23-26	It is not possible to frequently fast track a DXA to within 6 weeks in our health board, due to capacity. As such, it is reasonable to have a discussion with patient's r/e their wishes to either wait for their DXA result or commence a PO bisphosphonate whilst they wait, informing them that this may blunt the affects of anabolics in the future should the DXA suggest an anabolic could be indicated. This way, the patient is able to make an informed decision r/e their treatment, fitting with their wishes.	Thank you for participating in the consultation process. The committee agreed that ideally DXA capacity should be sufficient to allow timely assessment for all people. However, they were aware that in some areas there are currently issues with DXA capacity leading to prolonged waiting times and so made this recommendation to help guide prioritisation to those with greatest clinical need where necessary. However, it is a 'consider recommendation' to reflect that it may not always be feasible.
FTWW: Fair Treatment for the Women of Wales	Guideline	General	General	We would like to see the Guideline strengthen cross-speciality guidance to healthcare providers who are likely to come into contact with patients at risk of osteoporosis, or presenting with related symptoms, so that they are better-informed about risk stratification and identification tools, and more equipped to refer for DXA scanning where appropriate. These should include healthcare professionals performing gynaecological surgery which can result in premature menopause (hysterectomy, oophorectomy) ensuring osteoporosis is part of informed consent, those who are managing hormone-related health issues (endocrinologists), those who are supporting conservative management of	Thank you for participating in the consultation process. NICE guidance applies to all settings where NHS care is delivered.

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				musculoskeletal symptoms (physiotherapists) etc, so that that there is ongoing monitoring for patients affected. Improving awareness and use of risk assessment tools across specialties would reduce the possibility of diagnostic delay, and enhance the likelihood of early intervention and improved patient outcomes.	
FTWW: Fair Treatment for the Women of Wales	Guideline	General	General	We would like to see the Guideline strengthen guidance to healthcare providers performing DXA scans and GP / consultant follow-up, so that patients identified as having osteopenia are immediately advised of preventative strategies, supplements, and pharmacological interventions as appropriate (patients in Wales are reporting an interval of 7 years in some instances, even where osteopenia has been identified previously and no interventions offered in the intervening period).	Thank you for participating in the consultation process. NICE guidance applies to all settings where NHS care is delivered. Treatment options, including lifestyle advice, will be covered in part 2 of the guideline.
FTWW: Fair Treatment for the Women of Wales	Guideline	General	General	We would recommend that links to the NICE Guideline on Menopause: Identification and Management are inserted as appropriate, including recommending public health information uniformly being made available to women from age 40-46 (46 being average age of onset of perimenopause) and strengthening advice around annual HRT reviews as a tool to enable early and ongoing risk assessment.	Thank you for participating in the consultation process. The NICE guideline NG23 Menopause: identification and management is referred to in recommendation 1.3.7 and a link has been added to the risk factors table. Please also see this guideline for recommendations about HRT (see sections 1.4, 1.6, 1.8, and 1.9).
FTWW: Fair Treatment for the Women of Wales	EIA	01	Disability	We would advise that reference is made here to evidence which suggests that women with learning disabilities are more likely to experience earlier menopause. Difficulties or differences in communication might make it more difficult for patients to articulate symptoms, so inclusion of menopause	Thank you for participating in the consultation process. The guideline committee were aware of a new risk assessment tool for people with learning disabilities. However, at the time of finalising the draft recommendations a peer reviewed article was not available to include the evidence in the guideline. The

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				and bone health in annual health checks should be recommended. We would also recommend consideration of physical impairments as posing additional challenges to accessing DXA scans.	article relating to the clinical and cost effectiveness of this tool are waiting peer reviewed publication. Once published this will be considered for inclusion in the guideline. Annual health checks were not included as part of the scope of this guideline. We have added physical impairments to the examples of challenges to having a DXA scan in the committee discussion.
FTWW: Fair Treatment for the Women of Wales	EIA	02	Sex	The document references that 'the risk of fracture is higher for postmenopausal women than men of a similar age'; however, we would suggest that as the age of menopause can vary (to include those experiencing medical, surgical menopause / premature ovarian function) that the EIA makes clear that this comment pertains to the typical age of natural menopause (approximately 51 years) and that females who experience menopause early are at greater risk if not identified or adequately treated.	Thank you for participating in the consultation process. The risk assessment recommendations have been amended from 'people over 50' to 'women who have experienced the menopause'. We have updated the EIA document to state that most women experience menopause between the ages of 45 and 55 years. We have also added to the EIA that untreated early menopause and premature ovarian insufficiency were identified to have a greater risk. In the section on risk assessment recommendations for men aged under 50 and women who have not experienced menopause we have included untreated early menopause and premature ovarian insufficiency as risk factors.
FTWW: Fair Treatment for the Women of Wales	EIA	02	Socio-economic factors	We would suggest that some consideration is given to both socio-economic factors and geographical area as being potential barriers to receiving care, particularly as DXA scans will be undertaken in specific clinical settings which may require access to transport.	Thank you for participating in the consultation process. We have added this to the EIA form and into the committee discussion for DXA in evidence report D.

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FTWW: Fair Treatment for the Women of Wales	Guideline	04	03	Given that those with vertebral fractures may not have been identified as having a history of fractures, we suggest it is important to include family history (first-degree relative) of kyphosis, patient medical history (such as medical / surgical menopause or ovarian insufficiency) and symptoms as an alternative.	Thank you for participating in the consultation process. Evidence was not identified to add this to the recommendation, so this has not been included.
FTWW: Fair Treatment for the Women of Wales	Guideline	05	01	Consider including extended use of GnRH analogues in women (particularly without use of add-back HRT) Consider including extended use of Depo-Provera injection for contraception in women. Consider amending 'untreated' to include those for whom HRT has been delayed or not been optimised, for example, where transdermal oestrogen preparations are inadequately absorbed, and menopause symptoms are not resolved and / or hormone blood-tests show low levels despite treatment.	Thank you for participating in the consultation process. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors. If treatment is delayed, then they are untreated and so are covered by the recommendation. For recommendations explicitly about the menopause, please see the 2015 NICE guideline NG23 Menopause: identification and management .
FTWW: Fair Treatment for the Women of Wales	Guideline	06	04	We are concerned that those who experience vertebral fractures are often unaware or not assessed adequately in primary care despite symptoms, including evidence of kyphosis. We would suggest that the guideline links here to recommendations on early identification of vertebral fractures made later on in the document and that 'family history (first-degree relative) of kyphosis' as a risk factor should be strengthened, both in terms of guidance to clinicians in	Thank you for participating in the consultation process. Evidence was not identified to add kyphosis to the recommendation, so this has not been included.

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				facilitating referrals for DXA scan and / or initiating treatment and in patient information.	
FTWW: Fair Treatment for the Women of Wales	Guideline	06	13	Consider amending 'untreated' to include 'treatment delayed or not optimised' (as per comment above on Page 05; line 01).	Thank you for participating in the consultation process. The committee agreed that if treatment is delayed, then they are untreated and so are covered by the recommendation. The committee also agreed that adding 'not optimised' would not be helpful. Menopause treatments often need optimising, but it does not mean the treatment is not providing adequate fracture protection.
FTWW: Fair Treatment for the Women of Wales	Guideline	07	19	Consider referencing GnRH analogues and Depo-Provera injection as both are commonly used in women and pose additional risk to bone density.	Thank you for participating in the consultation process. The recommendation would be too long to add all medicines and the two main ones are given as examples. The committee anticipate healthcare professionals will make a clinical judgement when assessing the risk of fragility fractures.
FTWW: Fair Treatment for the Women of Wales	Guideline	07	23	Consider amending to make clear that HRT needs to be optimised to provide benefit and reduce risk, for example, whether or not patients report HRT as resolving menopause symptoms and / or an assessment of how far trans-dermal preparations are being absorbed via blood test. We would advise that links to NICE Menopause Guideline are clearly signposted, particularly recommendations regarding annual HRT reviews, which would be a key enabler of osteoporosis risk assessment.	Thank you for participating in the consultation process. The committee agreed that it is not known if HRT needs to be optimised to offer fracture protection. Treatments could be offering adequate fracture while being optimised. A link to NICE's guideline on menopause has been added to the risk factors table and recommendation 1.3.4.
FTWW: Fair Treatment for the Women of Wales	Guideline	17	15	We are concerned that the draft guideline only references hip fracture in a first-degree relative as a risk factor for younger people and that there is inequitable emphasis placed on kyphosis. Given the significant impact on wider health and	Thank you for participating in the consultation process. Evidence was not identified to add kyphosis to the recommendation, so this has not been included.

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				quality of life incurred by this latter condition, including possible reduced lung capacity and breathlessness; reduced appetite and anorexia; reduced mobility and the well-evidenced consequences for bone density and fracture elsewhere in the body, we would urge that reference to it as a hereditary risk is strengthened. This would help to improve earlier identification and reduce the risks compounded by delayed diagnosis, particularly as vertebral fractures are not usually precipitated by a fall and so often go unrecognised by patient and undiagnosed by clinician.	
FTWW: Fair Treatment for the Women of Wales	Guideline	17	29	Consider amending 'untreated' to include 'treatment delayed or not optimised' (as per comment above on Page 05; line 01). We would also recommend that references to 'early menopause' here and elsewhere in the guideline clearly stipulate the inclusion of those patients undergoing treatment to induce a 'medical menopause', i.e., for a chronic gynaecological or menstrual health condition and who have not been offered or advised to utilise add-back HRT. It is important that both patient and prescribing clinician are aware of the longer-term risk to bone density and potential fracture and that this is monitored and assessed as appropriate.	Thank you for participating in the consultation process. The committee agreed that if treatment is delayed, then they are untreated and so are covered by the recommendation. The committee also agreed that adding 'not optimised' would not be helpful. Menopause treatments often need optimising, but it does not mean the treatment is not providing adequate fracture protection.
FTWW: Fair Treatment for the Women of Wales	Guideline	19	06	The QFracture risk assessment tool recommended for patients aged 30-39 does not ask about history of chemical menopause, for example, use of injectable GnRH analogues or Depo-Provera, both commonly used to treat menstrual / gynaecological conditions commonly reported by younger women and people assigned female at birth, including	Thank you for participating in the consultation process. The committee emphasised the importance of doing a full clinical risk assessment because neither tool includes all factors associated with an increased risk of fracture.

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				endometriosis, adenomyosis (both conditions affecting more than one in ten), premenstrual syndrome / premenstrual dysphoric disorder, etc. Both therapies are associated with increased risk of osteoporosis. This is a significant omission and has implications for the EIA (sex, age) and informed decision-making, identification, and treatment of younger female patients.	
Hyperparathyroid UK Action4Change	Evidence Review A		General	We believe this evidence paper needs to be reviewed, considering changes to risk factors that have been ignored or dismissed. It appears short sighted and has many gaps for updated risk factors, not least the guideline for the management of primary hyperparathyroidism which is also outdated and no longer current. Ignoring risk factors could potentially be a precursor to harmful outcomes and should be taken seriously.	Thank you for participating in the consultation process. The committee agreed to not update the risk factor evidence review and use the evidence from the previous guideline. They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors. Primary hyperparathyroidism is in the table of secondary causes. The committee has added a new recommendation to seek advice from a specialist for people with primary hyperparathyroidism and follow guidance in the NICE guideline on primary hyperparathyroidism.
Hyperparathyroid UK Action4Change	Evidence Review A		General	There is no mention of vitamin D or magnesium throughout the whole paper. We are aware calcium and Vitamin D will be included in part two, but in case we don't get the opportunity to mention it there, we believe both vitamin D and magnesium should be included in risk factors as a precursor for bone loss. Both vitamin D deficiency and hypomagnesemia will inevitably over stimulate parathyroid hormone and lead to	Thank you for participating in the consultation process. The committee agreed to not update the risk factor evidence review and use the evidence from the previous guideline. They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been

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				bone loss. We don't see how or why neither are listed as risk factors for osteoporosis. This may not have featured in your 2012 guideline but absolutely should feature prominently in a 2026 update without any doubt. Back in 2012, there were not the financial restrictions on vitamin D testing that are now in place throughout the NHS. In house vitamin D tests cost £4. Osteoporosis and fractures cost much more.	updated to make it clearer that this is a list of examples of risk factors.
Hyperparathyroid UK Action4Change	Evidence Review B		General	There is no mention of vitamin D or magnesium throughout the whole paper. We are aware calcium and Vitamin D will be included in part two, but in case we don't get the opportunity to mention them there, we believe calcium, vitamin D and magnesium should be included in search criteria for EHRs to potentially highlight many more people at potential risk of bone loss.	Thank you for participating in the consultation process and for your suggestions regarding the use of EHR to aid identification of people at risk of fracture. The review on electronic health records did not identify evidence to make a recommendation and the committee made a research recommendation.
Hyperparathyroid UK Action4Change	Guideline		General	Glucocorticoids are mentioned 16 times throughout the guideline, whilst Proton Pump Inhibitors are only mentioned once, at the end of table five. We would recommend that more prominence is applied to PPIs throughout the guideline as it is a significant contributory factor to bone loss and has been linked to parathyroid hyperplasia.	Thank you for participating in the consultation process. Glucocorticoid use is mentioned because it is a risk factor directly mentioned in the risk assessment tools, can lead to glucocorticoid induced osteoporosis and the committee agreed it needs particular attention. PPIs are mentioned as a risk factor in the table. However, the committee do not agree they need to be highlighted as much as glucocorticoids, nor do they agree that they should receive more attention than other medicines associated with increased risk of fragility fracture.
Hyperparathyroid UK Action4Change	1.0.7 DOC EIA (2019)	001-003	3.1	'Have the potential equality issues identified during the scoping process been addressed by the Committee, and, if so, how?'	Thank you for participating in the consultation process. The committee did not think that people with hyperparathyroidism should be included as an equality issue because it does not directly relate to a protected characteristic. Primary

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				The inclusion criteria; age, disability, gender reassignment, pregnancy and maternity, refugees, asylum seekers. people who are homeless, prisoners and young offenders have all been identified but there is no mention of people with primary hyperparathyroidism, who are all considered at greater risk of osteoporosis, regardless of age, disability, gender, pregnancy, maternity, refugee, asylum seeker, homeless or prisoner/young offender status. We are therefore concerned that excluding people with primary hyperparathyroidism amounts to inequality and should be included.	hyperparathyroidism is already listed as an example as a factor associated with increased risk when assessing risk of fragility fractures. To highlight this further a recommendation has been added advising seeking specialist advice for people with primary hyperparathyroidism and this links to the recommendations in NICE's guideline on primary hyperparathyroidism.
Hyperparathyroid UK Action4Change	1.0.7 DOC EIA (2019)	003	3.2	'Have any other potential equality issues (in addition to those identified during the scoping process) been identified, and, if so, how has the Committee addressed them?' 'No new issues identified.' We believe the committee has missed an equality issue by not identifying people with primary hyperparathyroidism. We would recommend the committee be referred to NG132 Hyperparathyroidism (primary): diagnosis, assessment and initial management, which references osteoporosis six times, with a link to this guideline. For the purpose of equality, we believe this guideline should also reference NG132 other than the brief mention in Table one on page five of the draft guideline under the heading, 'other causes of secondary hyperparathyroidism'. Osteoporosis is an inevitability to all people with untreated primary hyperparathyroidism by nature of the bone reducing aspect of the disease.	Thank you for participating in the consultation process. A recommendation has been added advising seeking specialist advice for people with primary hyperparathyroidism and this links to the recommendations in the NICE guideline 132 on hyperparathyroidism as suggested.

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Hyperparathyroid UK Action4Change	1.0.7 DOC EIA (2019)	003	3.3	'Have the Committee's considerations of equality issues been described in the guideline for consultation, and, if so, where?' 'Recommendations have been made by age groups to reflect the increased risk of fragility fracture as people get older, and by sex to reflect the increased risk of fragility fracture in women, particularly following the menopause. This is discussed in the rationales and committee discussions in the evidence reviews' By excluding people with primary hyperparathyroidism, the committee's recommendations are basing increased risk of fragility fracture as people get older, when the reality is those risks apply to people much younger who have undiagnosed and consequently untreated primary hyperparathyroidism. We've seen evidence of people in their thirties with bone loss and fractures who are being missed by this guideline recommendations. Recommendations based on sex to reflect the increased fragility fracture in women particularly following the menopause, excludes men with primary hyperparathyroidism yet we see cases of men with osteoporosis caused by undiagnosed primary hyperparathyroidism considerably younger than the age threshold given throughout this guideline. We also see pre-menopausal women with primary hyperparathyroidism who have bone loss, so these recommendations are prohibitive and go against equality.	Thank you for participating in the consultation process. Primary hyperparathyroidism has been taken into account in the guideline. It is listed as one of the examples to consider when assessing risk in table 1. An additional recommendation has been added to seek advice from a specialist and follow the recommendations in NICE's guideline on primary hyperparathyroidism. There is also a recommendation to seek advice from a specialist when assessing fragility fracture risk in people under 30. This is to ensure to identify and treat underlying causes before any osteoporosis treatments are considered.

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Hyperparathyroid UK Action4Change	Guideline	004	002	'People aged 50 and over' We believe this should begin with 'People under 50' for the benefit of those with primary hyperparathyroidism whose doctors might exclude them by age, when not having time to read as far as pages 5, 6, 11 and 28. We see this often in clinical practice with NG132 where doctors do not read beyond recommendation 1.3.1, which leads to long delays in diagnosis and treatment. We believe highlighting early in this section that people below the age of fifty may have a treatable cause of secondary osteoporosis would be beneficial to both clinicians and patients, either with the link to NG132 as posted in Table 1 or even better, referencing them directly to recommendation 1.3.2 of NG132. We see people in their forties with multiple fragility fractures who would benefit from an earlier DEXA scan and diagnosis of primary hyperparathyroidism if they or their doctors see this information here. Often hyperparathyroidism is not diagnosed until after fragility fractures. Hyperparathyroidism is mentioned twice on page 5, in Table 1: other causes of secondary osteoporosis, page 11; 'other factors known to be associated with accelerated bone loss (for example, aromatase inhibitors or androgen deprivation therapy, or having primary hyperparathyroidism not treated with surgery)', and page 28 (lines 16-18) 'This is particularly	Thank you for participating in the consultation process. The committee did not think that this order should be changed. This section begins with those at higher risk of fragility fracture (men aged 50 and over and women who have experienced menopause). Evidence was not available to recommend widespread risk assessment in people under 50. A cross reference to the NICE guideline on Primary hyperparathyroidism has been made with reference to the recommendations related to DXA.

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				the case in people with a significant risk of accelerated bone loss including people with primary hyperparathyroidism not treated with surgery.'	
Hyperparathyroid UK Action4Change	1.0.7 DOC EIA (2019)	004	3.4	'Do the preliminary recommendations make it more difficult in practice for a specific group to access services compared with other groups? If so, what are the barriers to, or difficulties with, access for the specific group?' 'Age and disability DXA is not always technically feasible or tolerated (for example, in older people who are living with frailty or are housebound for whom DXA is not suitable).' As a result of inequality in this guideline, men and pre-menopausal women with primary hyperparathyroidism find it difficult to persuade doctors to request DXA. We therefore recommend that men below 75 and pre-menopausal women who may potentially have primary hyperparathyroidism should be added as specific groups unable to access services which could be remedied if they were included here alongside age and disability. We would advise it as essential to not only include them in equality statements and guideline recommendations, but to recommend adding DXA of non-dominant forearm in patients with primary hyperparathyroidism, because osteoporosis/osteopenia is common in the wrist cortex of these patients.	Thank you for participating in the consultation process. The committee have added a recommendation for people with primary hyperparathyroidism (PHPT) to seek advice from a specialist and follow recommendations in the NICE guideline on primary hyperparathyroidism. This guideline recommends considering DXA scans. PHPT has not been added to the EIA form because it does not directly relate to a protected characteristic. The bone site for measuring DXA was not part of guideline recommendations.
Hyperparathyroid UK Action4Change	1.0.7 DOC EIA (2019)	004	3.6	Are there any recommendations or explanations that the Committee could make to remove or alleviate barriers to, or	Thank you for participating in the consultation process. The committee have recommended that a full clinical risk

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				difficulties with, access to services identified in box 3.4, or otherwise fulfil NICE's obligation to advance equality? When DXA is not always technically feasible or tolerated the committee recommend that a shared decision with the person should be made about the appropriateness of treatment without the DXA results if the person meets the other criteria for treatment. We believe it is imperative to rule out a diagnosis of primary hyperparathyroidism in these patients before instigating treatment for osteoporosis.	assessment should be completed alongside a risk prediction tool. They anticipate that healthcare professionals will consider other conditions when doing a full clinical assessment and that these are ruled out before osteoporosis treatment is considered. The committee also recommended seeking specialist advice when assessing fragility fracture risk in people under 30. This is to identify and treat underlying causes before any osteoporosis treatments are considered.
Hyperparathyroid UK Action4Change	Evidence Review A	005		'The committee discussed whether there was likely to be new evidence that was strong enough to change current recommendations from the NICE guideline on osteoporosis (published 2012). They agreed that the risk factor review from the previous version of this guideline was still relevant and should be used to inform the updated recommendations. It was agreed that an informal consensus approach was the most appropriate method to answer this question.' How can this be so when NG132 was published 23 May 2019?	Thank you for participating in the consultation process. Primary hyperparathyroidism was already included as a risk factor in the 2012 version of the guideline. The committee agreed to add a new recommendation to seek advice from a specialist for people with primary hyperparathyroidism and follow guidance in the NICE guideline on primary hyperparathyroidism.
Hyperparathyroid UK Action4Change	Evidence Review B	005	15	We support searching Electronic health records (EHRs) to potentially identify patients at increased risk of fragility fracture that should be assessed, but we know with certainty that excluding people fifty or under, will result in missed opportunities to identify patients at increased risk of fragility	Thank you for participating in the consultation process. The review was set at age 50 because the committee agreed that the risk of fragility fractures in younger people is uncommon and searching electronic health records in this group is unlikely to be cost effective.

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				fractures who may or may not yet be diagnosed with primary hyperparathyroidism, or have been enforced to watch and wait for increased calcium levels despite osteopenia or osteoporosis. We base this upon many cases known to us.	
Hyperparathyroid UK Action4Change	Guideline	006	004	We strongly believe an insertion referencing primary hyperparathyroidism should begin this section. Whilst hyperparathyroidism is mentioned twice in Table 1, it makes no sense not to mention it as a risk of fragility fractures. We would suggest adding this bullet point: • potential or confirmed diagnosis of primary hyperparathyroidism	Evidence was not available to make a strong recommendation for assessing everyone with hyperparathyroidism. It is included in the table of factors associated with increased fracture risk. A cross reference to the NICE primary hyperparathyroidism guideline has been included in recommendation 1.4.3.
Hyperparathyroid UK Action4Change	Evidence Review A	006	004-005	An updated evidence review was not prioritised for this question, so no new literature searches were run. We feel there is a missed opportunity to update evidence for primary hyperparathyroidism from NG132 (published 23/05/2019).	Thank you for participating in the consultation process. Primary hyperparathyroidism was already included as a risk factor in the 2012 version of the guideline. The committee agreed to add a new recommendation to seek advice from a specialist for people with primary hyperparathyroidism and follow guidance in the NICE guideline on primary hyperparathyroidism.
Hyperparathyroid UK Action4Change	Evidence Review A	006	008 -011	We are concerned the risk factors listed are outdated by fourteen years. Primary hyperparathyroidism (PHPT) should absolutely be added as a bullet point on the basis undiagnosed/ untreated PHPT will lead to bone loss by nature of the disease. With increased PHPT cases reported in teenagers from 13 to16 years in the last five years, this will	Thank you for participating in the consultation process. The committee agreed to not update the risk factor evidence review and use the evidence from the previous guideline. They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been

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				impact your first bullet point 'age as an independent risk factor'. We would also recommend Proton Pump Inhibitors be added as a bullet point before or after glucocorticoid use because of their impact on bone density, and likely more prolifically prescribed than glucocorticoids. The following four bullet points in our opinion are outdated and insufficient criteria. • Age as an independent risk factor • Previous fracture • Glucocorticoid use • Family history of fracture.	updated to make it clearer that this is a list of examples of risk factors. Primary hyperparathyroidism was already in the table of secondary causes. The committee have added a new recommendation to seek advice from a specialist for people with primary hyperparathyroidism and follow guidance in the NICE guideline on primary hyperparathyroidism. Similarly, PPIs were included in the previous guideline and are included in this update. The committee do not agree that the list is out of date.
Hyperparathyroid UK Action4Change	Guideline	007	006	We would recommend adding this bullet point: • potential or confirmed diagnosis of primary hyperparathyroidism	Thank you for participating in the consultation process. Seeking specialist advice for people with primary hyperparathyroidism has been added as recommendation 1.4.3 with a cross reference to the applicable recommendations in that guideline.
Hyperparathyroid UK Action4Change	Guideline	007	009	The link to QFracture is first presented on page 7, line 8, but not until line 14, 1.3.2, is the age range between 30 and 39 mentioned. We wonder if the links should be presented sooner in the guideline for example Table one page five.	Thank you for participating in the consultation process. These recommendations have been simplified with QFracture being linked to at its first mention in what is now recommendation 1.3.1.
Hyperparathyroid UK Action4Change	Guideline	007	010 - 014	We feel these age-based recommendations are a little disjointed. Line 10 (1.3.3) says, 'Use either FRAX or	Thank you for participating in the consultation process. The recommendations have been updated and simplified to make

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				QFracture when assessing fragility fracture risk in people aged between: • 50 and 90 who do not meet the criteria in recommendation 1.3.1 (Dexa between 50 and 90) • 40 and 49 Then line 14 (1.3.4) suggests to 'Use QFracture when assessing fragility fracture risk in people aged between 30 and 39.' We feel this would be less confusing to recommend using QFracture and/or FRAX for ages 30 – 90 whether or not they meet the criteria in 1.3.1: (Dexa between 50 and 90). QFracture was developed for the UK population and is intended for use in the UK, whilst FRAX models have been developed from studying population-based cohorts from Europe, North America, Asia and Australia. As you have stated on line 22 on page 7, 'FRAX and QFracture assess risk differently.'	them clearer. A visual summary also accompanies the recommendations to help with understanding how they fit in the pathway.
Hyperparathyroid UK Action4Change	Evidence Review H	007	016-024	1.2.1. The outcomes that matter most 'The committee agreed that the number or proportion of people meeting the threshold for treatment (having previously not met the threshold), number of people who subsequently developed a fragility fracture and generic health-related quality of life measures were important outcomes for this review. All outcomes were considered equally important for decision making and therefore were all rated as critical.	Thank you for participating in the consultation process. The NICE guideline on osteoporosis (2012) was limited to assessing the risk of fragility fractures and did not include a question on monitoring. Therefore, this is a new question that searched for evidence published up until 15 November 2024 (see Evidence report H Appendix B for further details). There were no studies identified that matched the outcomes listed in the protocol. The committee agreed that it was important to make recommendations for adults at risk of fragility fracture

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				We wholeheartedly agree with this statement. So, were surprised to read on line 022; 'No evidence was identified for any of the outcomes' Then lines 023-024; 1.2.2. The quality of the evidence No evidence was identified. We would like to ask if any evidence has been identified since this guideline was first produced in 2012 and what measures are being taken to secure evidence for all outcomes?	who are not on pharmacological treatment to ensure they are reassessed and managed appropriately; therefore, the committee made recommendations based on their experience and expertise. No further actions were taken to provide future evidence as active recommendations were prioritised over making recommendations for research.
Hyperparathyroid UK Action4Change	Guideline	007	020	We believe 'or having primary hyperparathyroidism not diagnosed or treated with surgery' needs to be added after 'androgen deprivation therapy', as it has been included on page 11, line 24 and page 28, line 17/18.	Thank you for participating in the consultation process. The text related to the reason for doing a full risk assessment has been moved to the rationale for the recommendation. The committee agreed that only 2 key examples need to be included and did not want to give the impression the list is exhaustive. It is anticipated the healthcare professional doing the assessment will use their clinical judgement.
Hyperparathyroid UK Action4Change	Evidence Review B App A	011	003-004	A.1 Review protocol for electronic health records for identifying adults for fragility fracture 4 risk We believe your inclusion and exclusion criteria to be flawed, which will potentially miss patients at risk of fragility fractures much younger than 50 years. We base this on current cases of people with primary hyperparathyroidism since their mid-twenties who were also excluded from an accurate diagnosis based on age. We have multiple cases of teenagers	Thank you for participating in the consultation process. The committee agreed that the review on the use of electronic health records to identify adults at risk of fragility fracture should be limited to adults aged 50 and over. The risk of fragility fractures in younger people is uncommon and

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				diagnosed from 13 years, who mostly began with vitamin D deficiency leading to overstimulation of parathyroid hormone. 'Inclusion: Adults who are 50 years and older. Exclusion: - children and young people less than 18 years. - Adults under 50 years old'	searching electronic health records in this group is unlikely to be cost effective. The committee agreed to add a new recommendation to seek advice from a specialist for people with primary hyperparathyroidism and follow guidance in the NICE guideline on primary hyperparathyroidism.
Hyperparathyroid UK Action4Change	Guideline	011	011	'Consider pharmacological treatment for men aged 50 and over and women who have experienced menopause, if the criteria for a DXA scan are met;' Pharmacological treatment is not advised before or after parathyroid surgery because bisphosphonate initiation after parathyroidectomy may interfere with the beneficial effects of parathyroidectomy on fracture risk in osteoporotic patients with Primary hyperparathyroidism which is why we would recommend emphasis about ruling out PHPT as a cause of osteoporosis before commencing pharmacological treatment. One of many studies (2020) describing this effect which we would recommend the committee to read: https://www.sciencedirect.com/science/article/abs/pii/S003960601930460X#:~:text=After%20parathyroid%20surgery%2C%20the%20use,bone%20density%2C%20and%20renal%20function.	Thank you for participating in the consultation process. The committee anticipate that healthcare professionals will assess all contraindications before prescribing treatment to an individual and therefore this would be taken into account.
Hyperparathyroid UK Action4Change	Guideline	011	024-025	We are concerned that this sentence is misleading and can lead to challenges in practice; 'or having primary hyperparathyroidism not treated with surgery'. It should be	Thank you for participating in the consultation process. This wording is included as an example of a condition that is associated with accelerated bone loss. The guideline rationale

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				more specifically worded 'unable' to be treated with surgery' as there will be many hundreds not treated 'yet' with surgery, because they are yet to have a PHPT diagnosis confirmed, or will be waiting for surgery.	and discussion in evidence report E have been updated to clarify that people with primary hyperparathyroidism not treated with surgery would also include those waiting for surgery or receiving alternative treatment. The guideline is unable to make recommendations for people as yet undiagnosed with PHPT.
Hyperparathyroid UK Action4Change	Guideline	014	007-014	'Wrist' is mentioned three times in the guideline, twice on page 14, and once on page 25. Distal forearm is mentioned on line 7 of page 14, but we don't see a mention of distal forearm being included in the DEXA scan recommendations. As bone loss in the distal forearm is commonly linked to primary hyperparathyroidism, we would appreciate a recommendation to include this in DEXA Scans, particularly when primary hyperparathyroidism is suspected. Currently having the distal forearm scanned is a bit hit and miss across NHS Trusts.	Thank you for participating in the consultation process. The bone site to scan with DEXA was not covered as part of the guideline and no recommendations have been made on this. In addition, a cross reference to the NICE guideline on Primary hyperparathyroidism has been made with reference to the recommendations related to DEXA.
Hyperparathyroid UK Action4Change	Guideline	016	019-021	'They also noted fracture liaison services should systematically identify people aged 50 and over who have had a fragility fracture with the aim of reducing further fractures.' We would recommend adding after 'aged 50 or over': or younger with primary hyperparathyroidism'	Thank you for participating in the consultation process. The committee's understanding is that Fracture Liaison Services are aimed at those over the age of 50.
Hyperparathyroid UK Action4Change	Guideline	017	010-011	'For all other risk factors the committee agreed that there was no evidence to change the 2012 recommendation.'	Thank you for sending the studies for our attention. We agreed not to update the evidence review on risk factors and

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				There have been many reports since 2012 on the effects of long-term use of PPIs (>12 months) and we would like reassurance this has been taken into consideration by the guideline committee. https://www.frontiersin.org/journals/pharmacology/articles/10.3389/fphar.2025.1582908/full 'PPIs-related osteoporosis events were more common in those aged 18–65 years. (n = 1,478, 39.9%). In addition, hospitalization (n = 1,040, 28.1%) and other serious adverse events (n = 633, 17.1%) accounted for a greater proportion of the outcomes in patients who developed osteoporosis while taking PPIs.' https://www.gov.uk/drug-safety-update/proton-pump-inhibitors-in-long-term-use-increased-risk-of-fracture 'Advice for healthcare professionals: • treat patients at risk of osteoporosis according to current clinical guidelines and ensure they have an adequate intake of vitamin D and calcium • take into account any use of PPIs obtained over-the-counter'	did not search for any new studies. The recommendation does already include proton pump inhibitors as an example for taking other medicines associated with increased fracture risk (Table 1). As noted, we will be covering vitamin D and calcium in the second part of this guideline.

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				We are aware pharmacological including calcium and vitamin D will be covered in part 2 in July 2026, but wanted to mention it as corticosteroids featured quite prominently here.	
Hyperparathyroid UK Action4Change	guideline	028	018	We are concerned that this sentence is misleading and can lead to challenges in practice; 'or having primary hyperparathyroidism not treated with surgery'. It should be more specifically worded 'unable' to be treated with surgery' as there will be many hundreds not treated 'yet' with surgery, because they are yet to have a PHPT diagnosis confirmed, or will be waiting for surgery.	Thank you for participating in the consultation process. This wording is included as an example of a condition that is associated with accelerated bone loss. The guideline rationale and discussion in evidence report E have been updated to clarify that people with primary hyperparathyroidism not treated with surgery would also include those waiting for surgery or receiving alternative treatment. The guideline is unable to make recommendations for people as yet undiagnosed with PHPT.
Keele University	Guideline	General	General	Although these guidelines are welcomed and have many strengths, we ask NICE to consider the audience for these guidelines and the potential for confusion which will impede implementation. The primary audience for these assessment guidelines is primary care. In a survey of primary care healthcare professional's (n=341) practice in osteoporosis Osteoporosis care in primary care settings: a national UK e-survey - PubMed, many respondents reported that confusing clinical guidelines were a key barrier to implementation of evidenced-based osteoporosis care. Respondents asked for a simple flowchart to follow. In our view the current guideline risks adding to the confusion. It is not easy to operationalise these into a	Thank you for participating in the consultation process. The committee appreciate that differences between guidance is not ideal however they agreed that the new recommendations are clinically justified. The committee includes representation from primary care. One of the aims of updating the guideline and the technology appraisals for medicines at the same time is to create a unified clear pathway of care. The NICE Osteoporosis Quality Standard is also being updated to ensure a consistent approach. A visual summary of the recommendations will be developed to help ensure clarity. It is anticipated that people in England, Wales and Northern Ireland will follow the NICE guidelines while the SIGN guidelines are for Scotland.

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				flowchart. Particular areas of confusion are highlighted in comments 3-6,8,11,16 As a primary care health professional reported in our recent INDEX (publication in draft) “The problem is you've got clinical knowledge summary, you've got SIGN [Scottish Intercollegiate Guidelines Network] guidance (...) We've got NICE [National Institue for Health and Care Excellence]. You've got the NOGG [National Osteoporosis Guidelines Group] and then you've got the Royal Osteoporosis Society (...) where do I look for it all?” (PCP07)	
Keele University	Guideline	General	General	Mix of use of term ‘fragility fracture’ and ‘major osteoporotic fragility fracture’ terms throughout the document. • We recognise there is a definition for ‘major’ osteoporotic fracture. • Patients in our research strongly disliked the term ‘fragility’ which contributed to connotations of frailty and was associated with avoidance behaviours. Evaluation of quality and readability of online patient information on osteoporosis and osteoporosis drug treatment and recommendations for improvement - PubMed • In our research with n=341 primary care health professionals, a key barrier to implementation of evidenced based care was uncertainty among GPs and other primary health care professionals over whether a fracture was a fragility fracture or not, as this was infrequently included in secondary care	Thank you for participating in the consultation process. The committee recognise that many people with fragility fracture do not like the term ‘fragility’. However, the term is widely accepted and understood in the literature. The use of the term ‘fragility fracture’, rather than ‘osteoporotic fracture’, accommodates the fact that most fragility fractures occur in people who do not have osteoporosis (as currently defined, a BMD less than -2.5). Also, the guideline is about identifying fragility fractures, and not all of these will be osteoporotic.

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				letters/communications. Osteoporosis care in primary care settings: a national UK e-survey - PubMed Given the confusion surrounding the diagnosis of osteoporosis and uncertainty among primary care HCPs about what constitutes osteoporosis and fragility fractures, we suggest osteoporotic fracture is used in place of fragility fracture throughout and clearly defined, in line with recommendations from our international consensus study. Time to reframe osteoporosis: a position statement to characterize the osteoporosis care gap JBMR Plus Oxford Academic For primary care it would be really helpful to add that if the trauma history is unknown, a wrist, humerus, pelvis, hip or vertebral fracture in a patient aged 50 and over could be coded as an osteoporotic fracture (fragility fracture). Uncertainty about what is a fragility fracture has been identified as a barrier to assessment Osteoporosis care in primary care settings: a national UK e-survey - PubMed. GPs have requested guidance on coding osteoporosis and fragility fracture (INDEX study, publication in draft)	
Keele University	Guideline	General	General	This guideline differs from the previous NICE guideline which recommended NOGG thresholds for DXA and treatment. We acknowledge the NOGG thresholds have some evidence of cost effectiveness, but have not to our knowledge been compared with other approaches.	Thank you for participating in the consultation process. Please note that the previous NICE guideline (CG146) did not recommend use of NOGG thresholds for DXA and treatment. We acknowledge that some other NICE products have referred to them when defining high risk however these are also being updated in due course so that there is a consistent NICE

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				- We also acknowledge thresholds for DXA and treatment are complex, and represent a set of characteristics rather than being distilled down to one risk threshold. - We acknowledge the threshold of 10% has face validity and appears acceptable and is used in Scotland - In the NIHR funded PROTECTS study Keele to lead new study to improve early diagnosis of osteoporosis - Keele University, the cost effectiveness of different thresholds for DXA is going to be compared for the first time. - It therefore seems inappropriate to ask England healthcare settings to change from evidence based threshold to another less evidence-based threshold before these research findings are known. This is going to add to confusion and implementation barriers, both now and in the future when the optimum approach is known.	pathway. However, the committee discussed the basis for existing criteria for DXA and treatment during guideline development and do not agree that the guideline represents moving from an evidence-based approach defined by NOGG to a less evidence-based approach in terms of a clinical or cost-effectiveness basis. As acknowledged in the comment, setting criteria for DXA and treatment is complex and there have not been comparative studies. The committee consider the recommendations to be justified clinically and that they are likely to be associated with lower DXA resource use than if using NOGG criteria. The basis for the recommendations are described in the Committee discussion section in Evidence report E. The committee welcomes additional research in this area but, given that the PROTECTS study has just started and it is a 6-year work program, did not consider it appropriate to delay updating the guideline. The next update of the NICE guideline will benefit from new research in this area. This will be monitored in line with the priority board manual (NICE-wide topic prioritisation manual).
Keele University	Guideline	4	10	Rec 1.1.2 Recommendation to consider screening in adults based on age alone is population screening which contradicts recommendations from national screening committee, adding to guideline confusion	Thank you for participating in the consultation process. We liaised with the National Screening Committee (NSC) before the consultation. It was agreed that the recommendation to assess fracture risk in all women aged 65 and over and all men aged 75 and over relates to case finding and not screening. We

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					will continue to liaise with the NSC to ensure there is no contradiction between our two organisation's guidance.
Keele University	Guideline	4	11	Different age ranges for men and women make clinical sense but make the guideline more confusing and complex to implement	Thank you for participating in the consultation process. The committee acknowledge that having different age ranges could be confusing but they are important clinically. The risk factors review in the previous version of the guideline CG146 (which is included on the published guideline page as supporting information) found that women are at an increased risk of fragility fracture at a younger age than men. The recommendation has been updated so that the ages are on different bullet points to help improve clarity.
Keele University	Guideline	4	9 onwards	Use of verb 'consider' instead of assess further risks opportunistic model of assessment and health inequities. There are several key indications in this list in which we consider the verb should read 'assess' instead of 'consider', including, but not limited to, low BMI, people with rheumatoid arthritis, people with coeliac disease, hyperparathyroidism etc. The 'consider' guideline contradicts with other guidelines referenced in Table 1. For example, NICE guidance on primary hyperparathyroidism states all patients should have a DXA	Thank you for participating in the consultation process. NICE uses the term 'consider' to reflect when there is weaker evidence, and the committee has less confidence in the evidence (more information on the wording of recommendations is available in the Guidelines Manual). We did not update this question and used the existing recommendations from CG146. This used the term 'consider' in line with their discussions of the evidence. Thank you for pointing out the recommendation about DXA from the NICE guideline on hyperparathyroidism. We have added a recommendation that people with hyperparathyroidism should seek advice from a specialist and follow relevant recommendations within that NICE guideline.

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Keele University	Guideline	5	Table 1	Key risk factors missing include neurological disease such as Parkinsons Disease and parkinsonism, dementia. Key risk factor missing is Systemic Lupus Erythematosus which has a similar inflation to fracture risk as Rheumatoid arthritis. We don't consider this is adequately covered by 'other inflammatory arthropathies'. There are a number of differences with NOGGs list of CRFs	Thank you for participating in the consultation process. We compared the risk factors across the NICE, SIGN and NOGG guidance and the committee agreed that there were similar. The table includes examples of risk factors for secondary osteoporosis, but it is not intended to be exhaustive, and it has been made clearer in the recommendations that these are examples. Please note that we have amended this table and added some of the suggestions when related to a previous example already on the list. We have added a neurological category which has Parkinsons Disease and parkinsonism as examples. We have also added systemic lupus erythematosus as an example under other inflammatory arthropathies. However, other examples where we have not looked at the evidence have not been included (for example, dementia).
Keele University	Guideline	5	Table 1	CKD – Stage should be clarified	Thank you for participating in the consultation process. We agree and have updated this.
Keele University	Guideline	6	2	Clarity on assessment indications for younger adults are welcomed.	Thank you for your support and participating in the consultation process.
Keele University	Guideline	7	2	Rec 1.3.1 Restricting DXA on basis of age is potentially ageist.	Thank you for participating in the consultation process. The recommendation has been amended to everyone aged 30 and over who meet the criteria (recommendation 1.4.1 in published guideline). For anyone under the age of 30 specialist advice is sought before deciding what to do (recommendation 1.3.5 in the updated version of the guideline).
Keele University	Guideline	7	2	The recommendation to offer DXA in people with hip and vertebral fractures without the need for fracture risk	Thank you for participating in the consultation process. This recommendation has been moved to the section on DXA as

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				assessment makes sense, but this was not immediately obvious on first reading, particularly as this recommendation is within the fracture risk assessment section. Suggest explicitly state "without need for fracture risk assessment first" or make the recommendation that fracture risk assessment is not needed in people with hip and vertebral fractures, 9and/or move this recommendation to later on in this section. Grouping DXA recommendations and fracture risk recommendations together would help	suggested. The recommendation has been updated to include 'with or without completing a risk prediction tool' to clarify this point. A visual summary laying out the recommendations has also been developed to help with following the main guideline recommendation pathway.
Keele University	Guideline	7	2	Recommendation 1.3.1 would be better placed in section 1.4	Thank you for participating in the consultation process. This recommendation has been moved as suggested and is now numbered 1.4.1.
Keele University	Guideline	9	13	1.5.1 does not align with the position statement of the International Society for Clinical Densitometry and risks being very costly to implement.	Thank you for participating in the consultation process. This recommendation has been amended so it applies to all men aged 70 and over and women aged 60 and over. This is based on a study (EPOS, 2002) showing the increased incidence of vertebral fractures around the above age cut offs. The committee agreed that a lot of vertebral fractures are missed and including VFA when doing DXA for this age group is likely to be cost effective. Incidence of Vertebral Fracture in Europe: Results from the European Perspective Osteoporosis Study (EPOS). Journal of Bone and Mineral Research, 17(4): 716–724,2002
Keele University	Guideline	9	9	'Not needed for treatment decisions or monitoring' should be defined/clarified	Thank you for participating in the consultation process. The recommendation has been updated with a cross reference to a more detailed recommendation in the treatment criteria section.

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					Further information is also given in the rationale and people living with frailty is used as an example.
Keele University	Guideline	10	8	Exceptionally low BMD for age should be defined/clarified	Thank you for participating in the consultation process. The recommendation has been amended as suggested.
Keele University	Guideline	11	17	Unclear as to the justification of -1.5 as a BMD T score threshold here, yet -1.0 on line 26. This does not appear to be justified in supporting narrative.	Thank you for participating in the consultation process. The recommendations and rationale have been updated and the BMD T score of -1.0 as a criterion has been removed and a very high-dose of systemic glucocorticoid added as an independent criterion. The committee think that using a T score <-1.5 is important because the bulk of evidence for treatment derives from studies in people with BMD below this level with some risk factors. The committee agreed that in people with osteopenia fracture risk was less clear and most people would not be at risk. They also agreed that treatment could be considered for people with a BMD T-score of -1.5 or less and a fragility fracture, current or frequent systemic glucocorticoid use, or medicines or secondary causes known to be associated with accelerate bone loss are likely to be at increased risk and should be considered for treatment. The committee selected BMD T-score of -1.5 for this recommendation as they did not think everyone within the osteopenia range would be at risk of fracture and narrowed the range down to those closer to the osteoporosis range. The committee made this recommendation based on their knowledge and experience.
Keele University	Guideline	11	general	The use of T score thresholds disadvantages people who cannot have DXA or have unreliable T scores due to	Thank you for participating in the consultation process. The recommendations have been updated to make it clearer that

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				osteoarthritis, or prior surgery. We acknowledge the guidelines take this into account but a clearer statement that BMD alone should not dictate treatment decisions and that limitations need to be considered could be made.	BMD alone is not the only criterion. The committee agreed that BMD is an important risk factor and the magnitude of change in BMD in response to treatment is associated with the magnitude of fracture risk reduction. Therefore, they agreed it is still a key criterion to consider when making treatment decisions.
Keele University	Guideline	14	16	We welcome the recommendation for research to examine the effectiveness of using electronic health records to help identify patients who should have fracture risk assessment. The recently funded NIHR PROTECTS study will address this question Keele to lead new study to improve early diagnosis of osteoporosis - Keele University	Thank you for participating in the consultation process. Thank you for signposting this study. The next update of the NICE guideline will benefit from new research in this area. This will be monitored in line with the priority board manual (NICE-wide topic prioritisation manual).
Medimaps Group	Evidence review / Rationale for DXA interpretation	Section relating to interpretation of DXA results	General	UK Reference Data (Implementation Feasibility) The guideline does not address availability of UK-specific reference data for adjunctive DXA-derived measures. UK adult reference curves for Trabecular Bone Score (TBS Osteo v4) have recently been developed in collaboration with Dr Swainson and Prof. Bukhari using NHS DXA clinic data (n = 3,496) and will be presented at the World Congress of Osteoporosis 2026 (Swainson et al.; abstract attached). 1147_Swainson_TBSReferenceUK_WC These data provide age- and sex-specific centiles for UK adults and enable contextual interpretation of TBS within NHS practice.	Thank you for participating in the consultation process. Evidence for the accuracy of DXA in combination with trabecular bone score (TBS) to predict fragility fracture was sought for evidence review D but only one prospective cohort study was identified (Leslie 2014). Consequently, a research recommendation has been added on the accuracy of DXA with trabecular bone score (TBS) assessment for predicting fragility fractures in adults, including those who have had a previous fragility fracture. Any relevant studies published after this guideline will be considered for inclusion in future updates of the NICE guideline. Please see the NICE-wide topic prioritisation manual for further information on topic selection.

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				Availability of UK reference ranges addresses potential concerns regarding population applicability and supports feasibility of implementation without additional patient burden. Recognition of this development may be relevant to the committee's consideration of research and implementation recommendations.	
Medimaps Group	Evidence Review /Rationale for Recommendations	Rationale section relating to DXA and treatment thresholds	General	Economic Impact and Threshold Efficiency The rationale does not consider economic evidence relating to refinement of fracture risk assessment following DXA. Recent health economic modelling evaluating incorporation of TBS into FRAX-based assessment demonstrates that TBS-adjusted risk assessment is economically dominant compared with FRAX plus BMD alone (World Congress of Osteoporosis 2026, Börgström et al., abstract submitted). In individuals within $\pm 5\%$ of the 10-year major osteoporotic fracture intervention threshold, incorporation of TBS resulted in: • Net reclassification improvement of 11.8% • Gain of approximately 60 QALYs per 10,000 individuals • Lower overall healthcare costs As NICE decision-making relies on efficient targeting at intervention thresholds, omission of consideration of adjunctive DXA-derived measures such as TBS may overlook	Thank you for participating in the consultation process. Evidence for the accuracy of DXA in combination with trabecular bone score (TBS) to predict fragility fracture was sought for evidence review D but only one prospective cohort study was identified (Leslie 2014). A search for economic evidence was also completed but no studies were identified. Please note that conference abstracts were not includable (see protocol in Evidence Report E, Appendix A.2). Any relevant studies published after this guideline will be considered for inclusion in future updates of the NICE guideline. Please see the NICE-wide topic prioritisation manual for further information on updates. Consequently, a research recommendation has been added on the accuracy of DXA with trabecular bone score (TBS) assessment for predicting fragility fractures in adults, including those who have had a previous fragility fracture.

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				opportunities for improved allocative efficiency within the existing pathway.	
Medimaps Group	Guideline	Terms used in the guideline section	General	Omission from Terms used in Guideline Trabecular bone score (TBS) is not defined within the guideline terminology. We propose adding: <i>“Trabecular bone score (TBS): a DXA-derived textural index intended to provide information related to trabecular microarchitecture that can be used alongside BMD to refine fracture risk assessment.”</i> Inclusion would improve clarity and ensure consistent interpretation within NHS practice.	Thank you for participating in the consultation process. A definition of trabecular bone score has been added to the glossary.
Medimaps Group	Guideline	Recommendations for Research	Research recommendation relating to REMS	Research Recommendation (Consistency with REMS) The guideline includes a research recommendation regarding REMS (Radiofrequency Echographic Multi Spectrometry) due to uncertainty in non-DXA methods. However, trabecular bone score (TBS) is not referenced in either recommendations or research priorities. TBS has a mature body of peer-reviewed evidence supporting: • Independent fracture prediction beyond BMD • Incremental value when combined with FRAX	Thank you for participating in the consultation process. A research recommendation has been added on assessing the discriminative accuracy of DXA with trabecular bone score (TBS) assessment for predicting fragility fractures in adults, including those who have had a previous fragility fracture?

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				• Reclassification near treatment thresholds • Data in glucocorticoid-treated and other secondary osteoporosis populations • Responsiveness to anti-osteoporosis therapy For proportionality and consistency, we propose the addition of the following research question: <i>“What is the clinical and cost effectiveness of using trabecular bone score (TBS), alongside DXA BMD and clinical risk assessment (including FRAX/QFracture), to improve fracture risk stratification and treatment targeting in adults, particularly those with osteopenia and/or secondary osteoporosis?”</i>	
Medimaps Group	Guideline	Section 1.4 Bone density assessment	General	Clinical Implementation Clarification The guideline discusses DXA as the primary bone density assessment tool but does not acknowledge validated adjunctive DXA-derived measures that may refine interpretation. TBS is derived from the lumbar spine DXA examination already recommended in the guideline and does not require additional imaging visits or radiation exposure. Its integration therefore represents refinement of an existing pathway rather than expansion of resource intensity.	Thank you for participating in the consultation process. Evidence was not available to recommend TBS and a research recommendation was made instead.

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				Clarification within the rationale that DXA-derived adjunctive measures may support clinical judgement would improve completeness.	
Medimaps Group	Guideline	Section 1.7	Recommendation 1.7.1	Omission of TBS from Recommendations Rec 1.7.1: Recommendation 1.7.1 does not reference validated DXA-derived measures of trabecular microarchitecture, such as trabecular bone score (TBS), which may refine fracture risk assessment following DXA. TBS is a DXA-derived index used adjunctively with BMD to improve fracture risk stratification, particularly in individuals with osteopenia or suspected secondary osteoporosis. We propose adding the following wording: <i>“Where a DXA scan has been performed, consider using additional validated DXA-derived measures of trabecular microarchitecture (for example Trabecular Bone Score [TBS]) to refine fracture risk assessment, particularly in people with osteopenia or suspected secondary osteoporosis.”</i> This wording is cautious (“consider”), positions TBS as refinement rather than replacement, and aligns with the guideline’s emphasis on full clinical risk assessment.	Thank you for participating in the consultation process. The discriminative accuracy of TBS assessment added to DXA to predict fragility fracture was included as in Review D. One prospective cohort study was identified in men aged 50 and over (Leslie 2014). This study showed poor accuracy (AUC 0.6-0.7) in predicting major osteoporotic fracture and moderate accuracy for predicting hip fracture (AUC 0.7-0.8). As such the committee did not recommend the addition of TBS assessment to DXA. However, the committee recognise the potential for DXA + TBS and therefore made a research recommendation on this topic.
National Osteoporosis	Evidence Review E			<i>‘This analysis cannot determine whether lower treatment numbers are better (reduced over prescribing) or worse (missed treatment), and results need to be considered</i>	Thank you for participating in the consultation process.

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Guideline Group UK				<i>alongside the different clinical rationales for the criteria for BMD assessment and treatment.'</i> This quote from Evidence review E is appropriate indicating the weakness of the review. Additionally, as noted above, there are errors in the calculations.	The economic analysis simply applied different clinical criteria and summarised the results in terms of DXA and treatment rates. The committee interpreted this taking into account the clinical basis for the different criteria being applied. Their considerations and interpretation are described in the committee discussion. Thank you for taking the time to consider the analysis methods. We do not think there to be errors in the analysis calculations and have responded to specific issues where they are raised.
National Osteoporosis Guideline Group UK	Committee member list	General	General	We note Scottish representation from SIGN within the NICE committee, but not English, Welsh or Northern Irish representation from NOGG. NOGG would be happy if NICE co-opted NOGG member representation at the next stage.	Thank you for participating in the consultation process. The committee is the independent advisory group that considers the evidence and develops recommendations, taking into account the views of stakeholders. The committee member is not representing SIGN. They are an independent committee member who also happened to be a committee member of the SIGN guideline.
National Osteoporosis Guideline Group UK	Guideline GID-NG10216	General	General	All members of NOGG firmly believe that the NICE draft proposal to (a) use fracture risk assessment purely as a gateway to DXA measurement of BMD, and (b) operationalise thresholds for intervention that rely almost exclusively on BMD assessment rather than on fracture risk, represents a retrograde step for the management of patients at increased risk of osteoporotic fractures. It would take the UK back more than 20 years to a care pathway determined by BMD (and access to DXA) rather than consideration of wider fracture risk. The field has moved on over the last 20+	Thank you for participating in the consultation process. The committee's recommendations for risk assessment and decision to treat do not solely focus on BMD. The recommendations require a comprehensive assessment of risk factors, of which BMD is just one. Recommendation 1.7.1 highlights all the factors that need to be taken into account when making a treatment decision. Overall, while the committee acknowledge that BMD is not the only criterion for a treatment decision, they agreed that it is still a key indicator in deciding whether treatment will be effective. It was agreed that

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				years and NOGG sincerely hope that NICE considers this global shift in osteoporosis care, when reviewing this draft. We present more specific and detailed feedback below.	clinical judgment would be required to optimise clinical decisions. The recommendations have been edited to improve clarity of this.
National Osteoporosis Guideline Group UK	Guideline	General	General	Eligibility for BMD assessment The guideline recommends that BMD be measured in individuals with a 10-year probability of a major osteoporotic fracture of 10% or more (with FRAX) or a 10-year incidence of 10% or more (with QFracture) [page 8, line 10]. This differs from the NOGG approach that uses age-specific thresholds up to the age of 70 years with FRAX. The argument for the NICE strategy is that it likely saves money by decreasing the number of BMD tests [page 22, line 25]. There are, however, several concerns resulting from the proposed approach. 1. NICE considers that a 10% threshold for a major osteoporotic fracture applies both to FRAX and QFracture. Whereas there is a measure of agreement between hip fracture risk, MOF is substantially underestimated using QFracture (Figure 1). Thus, a QFracture incidence of 10% is	Thank you for participating in the consultation process. The claim that a 10% threshold is recommended on the basis of 'saving money' is not an accurate reflection of the committee's reasoning. The rationale for a 10% risk threshold for DXA (except in those with hip, vertebral or multiple fractures who do not need to meet a risk criteria) was made on a clinical basis. The economic analysis found that it would likely also be associated with lower DXA resource use. 1. It is accepted that the interpretation of risk scores in each of the fracture risk assessment tools is not straightforward and that the fracture risk estimates are not commensurable (which is a function of the fact the FRAX and QFracture tools are different prediction models, constructed using different statistical methods and predictor variables). We note that unlike the SIGN guidance we do not indicate a preference for QFracture over FRAX but recommend either and that the same tool should be used throughout a person's care. We note that the enclosed figure cites a figure from Kanis 2016, which itself cites Crandall 2014 and NICE Technology Assessment on Bisphosphonates 2015. Crandall 2014

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				approximately equivalent to a FRAX probability of 15% or more. Figure 1. Comparison of the distribution of FRAX and QFracture model output by decile of risk in women for hip fracture (left panel) and major fracture (right panel). The diagonal lines are the lines of identity [Kanis et al 2016]. Reference: Kanis JA, Compston J, Cooper C, Harvey NC, Johansson H, Odén A, McCloskey EV. SIGN Guidelines for Scotland: BMD Argues against SIGN and use of BMD as treatment threshold: “The SIGN guidance for men at high risk of fracture provides a set of confused and inconsistent recommendations that are in direct conflict with regulatory authorizations and is likely	compares FRAX without BMD to OST (Osteoporosis Self-assessment Tool) and SCORE (Simple Calculated Osteoporosis Risk Estimation Tool) and not QFracture. Reading the article suggests that the reference should be to Hippisley-Cox & Coupland 2011, which itself reports data originally published in Hippisley-Cox 2009. We also note that the data used to depict the agreement (or lack thereof) between FRAX and QFracture is from the NICE technology assessment on bisphosphonates 2015 and is from a simulation of the UK population rather than a cohort study. The claim in the article that SIGN guidance recommends treatment in a small (compared to FRAX) proportion of the population and this precludes its implementation is not further explained (presumably the point is that not enough people are being treated). In any case, although there are similarities between the recommendations made in this guideline and SIGN (such as the use of a 10% fracture risk threshold in the pathway), there are clear differences including: recommending the use of either FRAX or QFracture; the use of different age groups and combinations of risk factors to determine risk category; and an emphasis on holistic consideration of a patient's full clinical fracture risk profile. 2. The committee agree that the use of an age-based threshold in FRAX-NOGG to determine eligibility for pharmacological treatment leads to too few older people being treated and too many young people being treated because there is unclear long-term benefit for these treatments for a relatively small

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				to increase further the large treatment gap in men. For women, the number of women eligible for treatment (i.e. with osteoporosis) is 81,700 with the use of FRAX but only 12,300 with QFracture representing 8.2 and 1.2 % of the total population at risk, respectively. We conclude that serious problems with the SIGN guidance preclude its implementation.” Versus FRAX Versus QFracture. Calcif Tissue Int. 2016 May;98(5):417-25. doi: 10.1007/s00223-015-0092-4 2. NOGG sets the testing threshold at the age-specific probability of fracture equivalent to a woman with no major clinical risk factors (CRFs) for fracture up to the age of 70 years; the threshold subsequently remains stable for ages above 70 years. The NICE approach of a 10% threshold for BMD testing will inequitably underserve younger individuals at a risk equivalent to or greater than that of a woman with a prior fragility fracture (e.g. a 50-year-old woman with a NHNV fragility fracture in whom treatment would be considered by a clinician, the 10-year probability of MOF is 7.3% (calculated	reduction in absolute risk. Although it is true that the relative risk of fragility fracture compared to a postmenopausal woman of the same age without clinical risk factors decreases as age increases, this is not the case for absolute risk, which increases with age. Older people therefore stand to benefit more from treatment than younger people, who if treated with current pharmacological options will not be able to receive them at a later time. It is important to note that the recommendations state the conditions under which the committee reason treatment should be considered but do not assert that people with BMD levels outside these ranges should not or cannot receive treatment. 3. The committee recognises that a large number of older people will be eligible for DXA measurement of BMD but think that this is appropriate given the current profile of available pharmacological treatments. Although there is well-known variation in DXA scanner waiting times, this is a capacity issue that the UK government is aware of (they have recently committed to the funding of 20 new scanners in England in addition to the 13 extra DXA scanners announced in 2025). 4. The committee agreed DXA resource use had been analysed appropriately. DXA resource was included if a scan were undertaken to determine treatment eligibility, treatment choice or to record a baseline measurement. This is important because a single DXA scan can serve multiple functions: a scan undertaken to inform treatment eligibility can also inform

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				by FRAX), meaning she would be denied consideration of treatment for her osteoporotic fracture). 3. It is important to appreciate that a decrease in BMD is a stronger risk factor for fracture at younger than older ages (see point 6 below), but the set threshold of 10% will result in BMD testing for the vast majority of individuals at older ages. 4. NOGG has an upper assessment threshold above which femoral neck BMD is not mandatory. The draft NICE guidance does not appear to consider this in their modelling of BMD resources. The net result of this is that using the draft NICE approach of 10% threshold, 96% of all women at 70+ years in the UK (compared with 59% in NOGG) would be recommended for BMD testing. If considering those age ≥ 75 years, 99% would need BMD testing following NICE, compared with 56% taking the NOGG approach using the same population base [Kanis 2021] (Table 1). Thus, any premise of more efficient DXA use, or cost-saving, has not been correctly modelled. Table 1: Number and percentage of women identified for BMD testing on basis of either NICE 10% MOF probability threshold or NOGG age-dependent threshold (Simulated UK population as per Kanis 2021).	treatment choice and provide a baseline for future monitoring. However, individuals who are considered eligible for treatment without requiring DXA (e.g., those at high fracture risk in NOGG) may still require DXA for these other clinical reasons. Including DXA scans undertaken for all these reasons is therefore important when comparing alternative strategies. A strategy that requires more DXA scans to inform treatment decisions may require fewer DXA scans for other reasons. Restricting the analysis to DXA scans performed solely to inform treatment decisions would therefore misrepresent differences in DXA use between strategies. The committee were aware that BMD assessment is not required under NOGG in people above the upper assessment threshold when determining treatment eligibility. However, since BMD measurement is still recommended in this group to inform treatment choice and/or provide a baseline measurement for monitoring - and because committee experience indicated that most people did not start treatment without a DXA - it was agreed that DXA use should be included in this group as well in the analysis. This approach is described in Evidence Report E in the analysis methods under 'Comparators' and we have expanded this to make the rationale clearer. We have also added this point to the committee discussion. The analysis also included a breakdown of reasons for DXA use for the committee's consideration: summary information is reported in the 'Results' section under Figure 1, and a detailed breakdown, including by age group was included in Appendix I.

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				<table><thead><tr><th>Age band (years)</th><th>Population, n</th><th>NOGG: n (%)</th><th>NICE: n (%)</th></tr></thead><tbody><tr><td>50-65</td><td>25735</td><td>16007 (62)</td><td>5636 (22)</td></tr><tr><td>65+</td><td>24898</td><td>14730 (59)</td><td>21571 (86)</td></tr><tr><td>70+</td><td>18227</td><td>10767 (59)</td><td>17415 (96)</td></tr><tr><td>75+</td><td>12435</td><td>6933 (56)</td><td>12301 (99)</td></tr></tbody></table> In summary, the NOGG approach targets DXA to the (younger) population in which BMD measurement has its greatest predictive value. In contrast the NICE approach targets DXA to the (older) population in which BMD measurement has its lowest predictive value [Johnell 2005]. References Kanis JA, Johansson H, Harvey NC et al (2021a) An assessment of intervention thresholds for very high fracture risk applied to the NOGG guidelines: A report for the National Osteoporosis Guideline Group (NOGG). Osteoporos Int Oct;32(10):1951-1960. doi: 10.1007/s00198-021-05942-2. Johnell O, Kanis JA, Oden A, Johansson H, De Laet C, Delmas P, Eisman JA, Fujiwara S, Kroger H, Mellstrom D, Meunier PJ, Melton LJ 3rd, O'Neill T, Pols H, Reeve J, Silman A, Tenenhouse A. Predictive value of BMD for hip and other	Age band (years)	Population, n	NOGG: n (%)	NICE: n (%)	50-65	25735	16007 (62)	5636 (22)	65+	24898	14730 (59)	21571 (86)	70+	18227	10767 (59)	17415 (96)	75+	12435	6933 (56)	12301 (99)	
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				fractures. J Bone Miner Res. 2005 Jul;20(7):1185-94. doi: 10.1359/JBMR.050304.	
National Osteoporosis Guideline Group UK	Guideline	General	General	Complexity poses a barrier to implementation The reasoning that a fixed threshold is simpler to use is questionable in UK clinical pathways. In the UK, the majority of online FRAX calculations link directly through to the NOGG website by a click on a button on the FRAX website. On the NOGG website the probability of fracture is then graphically presented against the assessment and treatment thresholds recommended by NOGG. In an audit of NOGG website activity between mid-2013 and mid-2014, a total of 253,530 sessions were recorded on the NOGG website, with the vast majority (208,766; 82%) arising from sites within the UK. The remainder of sessions were from other countries demonstrating that some users of FRAX in other countries make use of the NOGG guidance. Of the UK-sourced sessions, the majority was from England, but the session rate (adjusted for population) was the highest for Scotland. Almost all of the UK sessions arose from calculations being passed through from the FRAX tool website, and NHS sites/locations were identified as the major source of visits to the NOGG website, comprising 79.9% of the identifiable visiting locations (McCloskey 2015). In a more recent audit in 2024 (McCloskey 2025), just under a million (n=936,217) viewings of the NOGG website occurred over the year from a total of	Thank you for participating in the consultation process. The committee do not agree that the NICE approach is 'too tricky' to use. Some of the recommendations have been updated for clarity and a visual summary accompanies the recommendations to help follow the pathway. The committee note that it is important to include clinical judgement. Regarding McCloskey 2015, it is not clear to the committee what relevance statistics about the use of the FRAX and NOGG websites have to do with the claim that use of a fixed threshold is simpler than an age-based threshold. Regarding Gregson 2025, we have been unable to locate this reference and so are unable to assess relevance.

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				781,643 individual users. Of these users, 87.7% (685,427) were based in the UK; the number of views arising directly from the FRAX UK model webpage was 625,498, comprising 67% of all visits to the NOGG website, or up to 76% of views from within the UK. By contrast, the sub-segmentation within the NICE approach into primary and secondary fracture prevention in people with high and very high risk, with different pathways by fracture type, and MOF risks, and T-Score thresholds, makes it a really difficult read for the non-expert, who it is designed to inform. A potential risk is that its complexity could have unintended consequences of being ignored, or users may well resort to an inappropriate binary approach to osteoporosis care, or even neglect of patients because it is 'just too tricky'. Reference: McCloskey E, Johansson H, Harvey N, Compston J, Kanis J (2015) Online linkage of FRAX fracture risk assessment to management guidance is used by clinical practitioners. An analysis of access to National Osteoporosis Guideline Group Guidance in the UK (July 2013-June 2014). Journal of Bone and Mineral Research 30 (Suppl 1), S290 C. Gregson, D. J. Armstrong, J. Bowden, C. Cooper, J. Edwards, N. J. L. Gittoes et al, (2025) Online Access to	

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				NOGG Guidelines in 2024; Interaction with FRAX And International Visibility. International Osteoporosis Foundation Conference, P1578.	
National Osteoporosis Guideline Group UK	Guideline	General	General	Discordance between guidelines poses a barrier to implementation Primary care health professionals are the audience for this guideline. The complexity and differences with existing NOGG and SIGN guidance are going to be a source of confusion and a barrier to implementation and are likely to increase the already large treatment gap in the UK. Research (survey of primary care health professionals, n=341) has demonstrated that complexity and dissonance across guidelines is a barrier to primary care assessment and treatment in osteoporosis Osteoporosis care in primary care settings: a national UK e-survey - PubMed The disparity between NICE and NOGG guidance is particularly confusing given the support of NICE for NOGG thresholds in recent NICE documentation, especially the NICE Osteoporosis Quality Standard QS149, published: 28 April 2017, and the updated fracture risk assessment document CG146 where NICE stated “Healthcare professionals should follow local protocols or other national guidelines for advice on intervention thresholds.”	Thank you for participating in the consultation process. The committee appreciate that differences between guidance is not ideal however they agreed that the new recommendations are clinically justified. The committee includes representation from primary care. One of the aims of updating the guideline and the technology appraisals for medicines at the same time is to create a unified clear pathway of care. The NICE Osteoporosis Quality Standard is also being updated to ensure a consistent approach. A visual summary of the recommendations will be developed to help ensure clarity.

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National Osteoporosis Guideline Group UK	Guideline	General	General	Gender inequality arising from the proposed NICE guidance The scenarios recommended by NICE cover a wide range of fracture probabilities that give rise to inequalities between men and women. For example, in individuals without risk factors and a BMI of 24kg/m² but a femoral neck T-score of -2.5, men would not be eligible for consideration for BMD and thus treatment, since fracture probability derived by FRAX is less than 10% irrespective of age, whereas women are eligible from the age of 62 years. An additional example of disparity and inequity is for a man or woman (BMI 24kg/m²) with a parental history of hip fracture; both would qualify for BMD assessment under NOGG from age 50 upwards. Under NICE, however, women would qualify with this risk factor from the age of 58 but men wouldn't qualify for BMD evaluation until the age of 73 years. Similar inequalities arise from the clinical scenarios given by NICE set at a T-score of -1.5 (prior fracture or glucocorticoid exposure). Similar inequalities arise with the clinical scenarios given by NICE set at a T-score of -1.0 (e.g., prior MOF) in that women	Thank you for participating in the consultation process. The committee acknowledges that there will be slight differences between both approaches. Clinical judgment plays an important role for both approaches. There are some minor gender differences in reaching DXA and intervention thresholds. However, the committee think that using the NOGG approach would underdiagnose older men and women. In the first example, these people would not be risk assessed without a risk factor present. Therefore, they would not have BMD measurement.

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				are eligible from the age of 62 years whereas no men are eligible for consideration for treatment. Note BMI set at 24kg/m ² here and below.	
National Osteoporosis Guideline Group UK	Guideline	General	General	Eligibility for treatment is unclear The draft NICE approach utilises FRAX or QFracture as an entry for the consideration of treatment but does not use estimated incidence or probability of fracture as intervention thresholds. Rather it chooses criteria mainly based on the T-score. This is a step back to the RCP guidance a quarter of a century ago [RCP 1999] rather than advancing the cause of osteoporosis. There are a number of problems that arise with the recommendations that relate to using a BMD T-score as the gateway to treatment [Kanis 2015, Johansson 2017] and how the T-score should be used. 1. The method of calculating the T-score is not given and, for example, it is unclear whether the calculation is based on a UK reference range or on NHANES III. 2. It is not reported whether T-scores are sex specific. 3. It is unclear whether the T-score relates to BMD at the femoral neck or at other sites. 4. It is unclear which T-score is used if more than one site is measured.	Thank you for participating in the consultation process. The committee have amended the recommendations to make clear that although an individual can be eligible for pharmacological treatment on the basis of BMD T-score, it is not the only factor to be taken into account. 1. Regarding points 1-4 regarding BMD T-score, how BMD is measured is beyond the scope of the guideline and we would hope that DXA services act in accordance with established guidance (e.g. ISCD or ROS guidelines). 2. Regarding point 5, the committee accept that from the age of 78, the average T-score in the general population is lower than -2.5. However, this is a reflection of the increased incidence of osteoporosis and high fracture risk in the elderly. 3. Regarding point 6, the recommendation for treatment criteria with a T-score of <-1 has been removed. The recommendation that a T-score of <-1.5 can be used as a criterion for treatment is consistent with the fact that the majority of evidence for pharmacological treatment has been conducted in people with BMD below this level. Indeed, a recent IPD meta-analysis (https://doi.org/10.1093/jbmr/ziae068) of all anti-fracture

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				5. The significance of a given T-score varies according to age [Kanis 2000]. At the age of 65 years, a T-score of –2.5 confers a modest increase in the probability of fracture compared with women with no clinical risk factors and in whom BMD is not measured. With advancing age, the difference in the probability of fracture between the general population and those with a T-score of –2.5 diminishes, and, indeed, from the age of 78 years and above in the UK, the fracture probability becomes progressively lower than that of the age- and sex-matched general population without CRFs (Fig. 2). In other words, a T-score of –2.5 becomes a protective factor from the age of 78 years and the T-score required to achieve a “fracture threshold” (i.e. a fracture probability equivalent to that conferred by a prior fracture) becomes progressively lower with age [Grigorie 2013].	treatments by baseline BMD-FN T-score quintiles showed that although there was overall efficacy for vertebral fractures at all quintiles, this was not the case for other types of fractures (non-vertebral, hip, all clinical). In particular, there was no difference for any anti-fracture treatment (nor when the analysis was restricted to bisphosphonates) with the 95% CIs crossing the line of no association for these other fracture types. (That is, the treatments did not significantly increase time to non-vertebral/hip/all clinical fracture for people in the highest quintile). 4. The committee have not excluded people with a previous fragility fracture from receiving treatment. Rather they recommend treatment for the people with a previous fracture they think are at risk from further fractures and who will benefit from treatment – treatment will only be cost-effective if there is benefit from treatment. With this in mind they noted that most the evidence on the effectiveness of treatment is for people with a BMD t score of -1.5 or less. However, they have also clarified the recommendations that make provision on making a shared decision to treat for people who are not able, for whatever reason to have a DXA measurement of BMD, and recommendation 1.7.1 also advises that a shared decision is made on whether to treat with the person based taking into account several factors including any previous fragility fracture. We have updated the recommendation to offer a DXA scan to people over 30 who have had a single MOF in the last two

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				Figure 2. 10-year probabilities (%) of a major osteoporotic fracture in women from the UK according to a T-score of -2.5, a prior fracture, or the combination. BMI set at 24 kg/m² [Kanis 2016]. 6. This T-score paradox is compounded at a T-score of -1 which is largely protective (i.e. associated with a lower fracture risk than the average risk in a woman with no clinical risk factors from the age 52 years and becomes increasingly so thereafter). The draft guidance does, however, qualify the T-score threshold of <-1 with a requirement of age >65 years and high dose exposure to glucocorticoids; however, no explanation for this particular combination over any other	years. This will mean that this group will be eligible for DXA scan even if their absolute risk score is below 10%.

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				combination of risk factors is provided. Using a database of a simulated population of 50,633 women from the UK that was provided to NICE to explore the impact of such decisions, 24,898 would be >65 years of age, of whom 21,571 (86.6%) would have a FRAX MOF probability >10%. Of the latter, 20,798 (96.4%) would have a femoral neck T-score <-1. Given that the proportion with high dose glucocorticoid use is not discussed, the number with glucocorticoid exposure fulfilling the FRAX criteria is 2,090 (10.0% of those qualifying for a BMD test) with a minimum 10-year FRAX MOF probability of 7.4% and an average probability of 27.4%. However, another 7,753 women not on glucocorticoids (a 3.7-fold higher proportion) would have an identical average MOF (27.4%) but a higher minimum MOF (>18.2%) fracture probability than those on glucocorticoids and yet be unlikely to be considered for treatment. Selection for treatment based on categorisation rather than fracture risk is inequitable. Ethical issues arise from the draft NICE guidance in that many individuals with a prior fragility fracture are considered ineligible for consideration for treatment despite the availability of cost-effective interventions. The denial of treatments in individuals with the clinical expression of their disease is counterintuitive to clinical medicine.	

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				References Royal College of Physicians (1999) Osteoporosis: clinical guidelines for the prevention and treatment. London: Royal College of Physicians; 1999. Kanis JA, McCloskey EV, Harvey NC, Johansson H, Leslie WD (2015) Intervention thresholds and the diagnosis of osteoporosis. J f Bone Miner Res 30: 1747-1753. doi: 10.1002/jbmr.2531 Johansson H, Azizieh F, Harvey NC, McCloskey E, Kanis JA (2017) FRAX- vs. T-score-based intervention thresholds for osteoporosis. Osteoporosis International 28: 3099-3105. doi: 10.1007/s00198-017-4160-7. Kanis JA, Compston J, Cooper C et al (2016) SIGN Guidelines for Scotland: BMD Versus FRAX Versus QFracture. Calcified Tissue International 98: 417-25. doi: 10.1007/s00223-015-0092-4 Kanis JA, Johnell O, Oden A, Jonsson B, De Laet C, Dawson A (2000) Risk of hip fracture according to World Health Organization criteria for osteoporosis and osteopenia. Bone 27;585–90	

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				Grigorie D, Sucaliuc A, Johansson H, Kanis JA, McCloskey E (2013) Incidence of hip fracture in Romania and the development of a Romanian FRAX model. Calcif Tiss Int 92:429–36	
National Osteoporosis Guideline Group UK	Guideline	General	General	The definition of an osteoporotic fracture requires clarification Fragility fracture needs to be clearly defined in order that Primary care healthcare professionals do not need to rely on a secondary care code of fragility fracture (or osteoporotic fracture) to take action. Primary care health professionals have cited uncertainty about what constitutes a fragility fracture as a barrier to assessment [Hawarden 2025]. The degree of trauma that precipitates a fracture is not of paramount importance as research has demonstrated that fracture risk is increased even if the fracture was high trauma [Leslie 2020] Of note a recent international consensus article has recommended the term osteoporotic fracture instead of fragility fracture, as a more acceptable term for patients and overcoming issues with the operational BMD diagnosis of osteoporosis [Paskins 2026].	Thank you for participating in the consultation process. GP education and coding are outside the scope of this guideline. We recognise that many people with fragility fracture do not like the term ‘fragility’. We note that the term is widely accepted and understood in the literature. Its use rather than ‘osteoporotic fracture’ also accommodates the fact that most fragility fractures occur in people who do not have osteoporosis (as currently defined, a BMD less than -2.5). Also, the guideline is about identifying fragility fractures and not all of these will be osteoporotic. Fractures caused by trauma are not within the scope of this guideline. For recommendations on other types of fracture see the NICE guideline on Hip fracture: management (CG1234), Fractures (complex): assessment and management (NG37) and Fractures (non-complex): assessment and management (NG38). The reference of ‘2 or more fractures’ can mean different incident events or two fractures occurring at the same incident event. We have updated the NICE guideline rationale and evidence report committee discussion to clarify this.

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				Furthermore, at 6 points the text says '2 or more fractures', clarify whether that means different incident events, or could it mean two fractures occurring at the same incident event. References A Hawarden, L Bullock, N Marie Cox, E Nicholls, J Protheroe, C Jinks, Z Paskins. Osteoporosis care in primary care settings: a national UK e-survey Arch Osteoporos. 2025 Aug 6;20(1):109. doi: 10.1007/s11657-025-01591-8. W D Leslie , J T Schousboe, S N Morin, P Martineau, L M Lix, H Johansson, E V McCloskey, N C Harvey, J A Kanis. Fracture risk following high-trauma versus low-trauma fracture: a registry-based cohort study. Osteoporos Int. 2020 Jun;31(6):1059-1067. doi: 10.1007/s00198-019-05274-2. Z Paskins, L Bullock, A Hawarden, F Blackman, DJ Armstrong, I Bentley, E M Clark, et al Time to reframe osteoporosis: a position statement to characterize the osteoporosis care gap. <i>JBMR Plus</i>, Volume 10, Issue 3, March 2026, ziag006, https://doi.org/10.1093/jbmrpl/ziag006	
National Osteoporosis Guideline Group UK	Guideline	General	General	Comments on modelling and cost-effectiveness thresholds The ratio of vertebral fractures to other fractures that is used in modelling is incorrect in that it relates to clinical vertebral fractures. NICE supports the use of VFA. The prevalence of	Thank you for participating in the consultation process. VFA detected fractures will not be identified until the time of DXA, and so in terms of calculating numbers eligible for DXA

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				VFA detected fractures is more than three-fold greater than for clinical fractures [Schousboe 2016]. The argument for a high threshold on the basis of cost-effectiveness is inconsistent with the fact that NICE considers that alendronate is cost-effective in women with a 10-year probability of MOF of 1% or more [NICE 2017]. The cost of alendronate has not increased in the subsequent 9 nine years. References Schousboe JT (2016) Epidemiology of Vertebral Fractures. J Clin Densitom 19:8-22 National Institute for Health and Care Excellence (2017) Bisphosphonates for treating osteoporosis. Aug 9, 2017. https://www.nice.org.uk/guidance/ta464/resources/bisphosphonates-for-treating-osteoporosispdf-82604905556677 (accessed Jan 17, 2026).	we consider that using data that doesn't incorporate this would be appropriate. We agree that VFA detected fractures could impact treatment numbers. However, this will likely increase treatment numbers in all comparators. We have added discussion of this limitation of the economic analysis which was discussed by the committee, however it did not change committee conclusions. The rationale for a 10% risk threshold for DXA (except in those with hip, vertebral or multiple fractures who do not need to meet a risk criteria) was made largely on a clinical basis. The economic analysis found that it would likely also be associated with lower DXA resource use. The committee were aware that oral bisphosphonates had been found to be cost effective at very low risk levels (1% in the 2017 NICE technology appraisal analysis) but did not consider treatment to be clinically appropriate at that level. It was noted that this threshold was removed from the TA464 recommendation due to concerns raised by the MHRA that its inclusion may lead to use in low-risk populations outside the evidence base. The committee discussion is described in Evidence report E in Section 1.2 The committee's discussion and interpretation of the evidence.
National Osteoporosis Guideline Group UK	Guideline	General	General	Learning from the management of other non-communicable diseases The absolute risk of a clinical outcome is increasingly playing a role in the management of non-communicable diseases, most notably cardiovascular diseases (CVD), including	Thank you for participating in the consultation process. The committee agreed that treating younger adults with a low absolute risk would make sense on a population level when the treatment is safe, effective and works long-term (e.g statins for CVD). However, they noted that treatment for osteoporosis cannot easily be used long-term, and some treatments are only

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				hypertension. For example, in a paper from the Blood Pressure Lowering Treatment Trialists’ Collaboration in the [Lancet in 2021] that described a meta-analysis of 48 randomised controlled studies of blood pressure lowering in patients at risk of CVD, they reached several conclusions that mirror developments within the field of osteoporosis. For example, they concluded that their analysis dismissed “the suggestions that blood pressure-lowering treatment is only effective when blood pressure is above a certain threshold”, a finding also reflected in the field of osteoporosis. Further, they concluded that “Our study calls for a revision of..... guidelines. The finding.... calls for consideration of blood pressure-lowering treatment for any individual who has a sufficiently high absolute risk of cardiovascular disease.” The European Society for Hypertension embraced this approach in its 2024 guidance [McEvoy 2024]. A similar approach is taken by NICE (NG238) to prescribing of statins to prevent CVD where the main instrument used to help patients decide about treatment is the QRisk 10-year risk score, not their individual cholesterol values or other individual risk factors (https://www.nice.org.uk/guidance/ng238/resources/patient-decision-aid-on-should-i-take-a-statin-pdf-243780159). In a similar way NICE should follow the same approach to prescribing anti-osteoporosis treatment to prevent fracture risk where the main instrument used to help patients decide	recommended once in a lifetime. This would mean treatment options for later in life when a person is more likely to be at a high level of risk would be limited.

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				about treatment is their probability/risk of fracture, not their individual BMD value. References Blood Pressure Lowering Treatment Trialists' Collaboration (2021) Age-stratified and blood-pressure-stratified effects of blood-pressure-lowering pharmacotherapy for the prevention of cardiovascular disease and death: an individual participant-level data meta-analysis. Lancet 398:1053-1064. DOI: 10.1016/S0140-6736(21)01921-8 McEvoy JW, McCarthy CP, Bruno RM, Brouwers S, et al (2024) ESC Guidelines for the management of elevated blood pressure and hypertension. Eur Heart J 38:3912-4018	
National Osteoporosis Guideline Group UK	Guideline	General	General	Femoral neck Quantitative Computed Tomography (QCT) can measure bone mass Diagnosing osteoporosis with QCT (Quantitative Computed Tomography) is missing from the draft NICE guideline, despite being justifiably present in NOGG guideline. This is a substantial omission and should be rectified. NICE scope included, “Bone mass measured by dual energy x-ray absorptiometry (DXA) and quantitative computed tomography (QCT) will be considered”. The draft guideline, however, excludes all mention of QCT. Recently the Royal College of Radiologists (plus UKAS) accredited the first NHS clinical QCT service. It is advised that NICE committee include reference to femoral neck QCT where T-scores	Thank you for participating in the consultation process. In review D, we reviewed the accuracy of QCT to predict major osteoporotic or hip fracture where the reference standard to confirm fracture is clinical review, self-report, and/or radiographs, and not BMD as measured by DXA. We identified 2 small studies (one of which was added following consultation) in restricted populations providing low or very low-quality evidence. The committee did not think that they could make a recommendation for its use to predict fragility fracture from this limited evidence. However, the committee agreed to add a research recommendation on this bone accuracy assessment

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				calculated from 2D projections have proven equivalence to femoral neck DXA T-scores and can be used in FRAX, as recommended by the International Society for Clinical Densitometry guidelines (Krueger 2023) and NOGG. Lumbar Spine trabecular BMD as measured by QCT can be used to predict fractures, but derived values are not equivalent to DXA derived T-scores and therefore cannot be used to diagnose osteoporosis. Reference Krueger, D., Tanner, S.B., Szalat, A., Malabanan, A., Prout, T., Lau, A., Rosen, H. & Shuhart, C. DXA Reporting Updates: 2023 Official Positions of the International Society for Clinical Densitometry, Journal of Clinical Densitometry 101437. doi: 10.1016/j.jocd.2023.101437	technique so that new studies could be considered for any updates of this guideline. Please note that the aim of review D was not to assess whether QCT can be used for osteoporosis diagnosis (that is, whether it is a good measure of bone mineral density) nor the degree to which femoral neck scores from QCT and DXA are correlated.
National Osteoporosis Guideline Group UK	Guideline	General	General	Most osteoporotic fractures occur in patients with a BMD T-Score > -2.5 This fundamental fact is not acknowledged in this draft guideline.	Thank you for participating in the consultation process. The committee agree that most fragility fractures occur in people with a BMD T-score > -2.5. Indeed, this is one of the main reasons that the recommendations made by the committee on assessment, prevention, and management of fragility fracture include consideration of more than just BMD.
National Osteoporosis Guideline Group UK	Guideline	General	General	Feedback from patient representatives <i>'The guideline states it is for 'people at risk, their carers and families', so it should have a Plain English Summary. This should include treatment options/information links to assist shared decision making.'</i>	Thank you for participating in the consultation process. There will be an information for the public section on the NICE website when the guideline is published. It is a brief text which explains why the guideline is important and the main ways it should improve a person's care. The document will include

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					advice about shared decision making and links to other sources of advice and support. A visual summary accompanies the recommendations to make them easier to follow. Treatment will be covered in part 2 of the guideline.
National Osteoporosis Guideline Group UK	Guideline	General	General	Feedback from patient representatives An excessive reliance on DXA as the gateway to treatment discriminates against those who are older, frailer and disabled. <i>'DXA is not always technically feasible or tolerated ...How many people are not having scans when appropriate assistance (possibly from an HCA) or the use of some judicious equipment might achieve a satisfactory outcome? Leaflets say scans are painless and I agree the scanner does not inflict pain. But I've heard from many people about the difficulties of being in pain during or after a scan. This leads to reluctance to have further scans.</i> <i>My recent experience raises concerns. When being put in the supine position my head was totally unsupported because of kyphosis. I asked if the pillow could be doubled over. The answer was, 'no'. So, I grabbed my hair to provide some support. Then I was told, 'You must keep your arms by your side'. Explaining the difficulty and pain resulted in something small being tucked under the pillow. I asked if I could bend my legs (as on previous occasions) and a wedge was put under my knees, which did help a little. Then came the comment, "If you can't stay still, we'll have to abandon this".</i>	Thank you for participating in the consultation process. The committee agreed that it is important to have a BMD t-score to help guide treatment decisions for most people. However, they have also clarified the recommendations that make provision for people who are not able, for whatever reason to have a DXA measurement of BMD. Therefore, not creating inequalities in access to treatment based on age, frailty or disability. This feedback from the patient representative is a poor experience of care. It is anticipated that imaging services would ensure that patients are comfortable and that reasonable adjustments are made to achieve this. The NHS Constitution for England states that you have the right to be treated with a professional standard of care, by appropriately qualified and experienced staff, in a properly approved or registered organisation that meets required level of safety and quality.

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				<i>It was well over an hour after I left before I could move enough to drive home.'</i>	
National Osteoporosis Guideline Group UK	EIA	2	n/a	3.1 Pregnancy and maternity: We were unable to find a reference for the 2024 surveillance review.	Thank you for participating in the consultation process. The 2024 surveillance review of evidence regarding whether pregnancy and postpartum is a fracture risk factor for NICE guideline CG146 can be downloaded here . We have added the link to the EIA version 3 document.
National Osteoporosis Guideline Group UK	Equality impact assessment	2	n/a	3.1 Gender reassignment: "The committee did not make specific recommendations for trans and non-binary people because the information available at the time of development for these groups of people was too limited." This should inform a research recommendation	Thank you for participating in the consultation process. The committee have added recommendations on risk assessment and treatment criteria for trans and non-binary people rather than adding a research recommendation.
National Osteoporosis Guideline Group UK	Guideline	4	15	History of hip fracture in a parent is of no less relevance in people over the age of 80 years Most studies of parental history of hip fracture have not been able to adjust for the age at fracture in the parent. Recently, however, McCloskey 2025, in a large sample size showed that the hazard ratio for fracture associated with self-reported parental fracture did not vary by baseline age, follow-up time, or parent affected, i.e. there is no age interaction for the individual undergoing fracture risk assessment. With regard to the conclusion in the draft guidance that parental hip fractures occurring above the age of 80 are less important in risk assessment than those below the age of 80,	Thank you for participating in the consultation process. McCloskey 2025 shows that the hazard ratio for fracture associated with self-reported parental fracture did not vary by baseline age, follow up time or parent affected. This refers to age of the individual and not parental age at the time of their hip fracture. History of hip fracture in a first degree relative, particularly if the family member was aged under 80 at the time of hip fracture, was considered a risk factor for fragility fracture. The relationship was dependant on the age at the time of the first-degree relative's hip fracture. The risk is increased when the parental fracture occurred at a younger age and those after the age of 80 had no significant impact on offspring's risk.

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				this has been derived from a single study (Yang et al, JBMR 2016 ;31(9):1753-9) with a relatively small proportion of participants aged over 60 years (over 75% of offspring were between 40-44 years of age at study start). Notably, the conclusion also ignores the observation that parental history of hip fracture over the age of 80 years was still associated with a higher risk of hip fracture in offspring. The recommendation is an incorrect interpretation of the data based on a single study. Finally, it should be noted that this study was based on verified parental fractures which is not translatable to clinical practice where they are almost always self-reported. Reference McCloskey EV, Johansson H, Liu E, Åkesson KE, Anderson FA, et al Family history of fracture and fracture risk: a meta-analysis to update the FRAX® risk assessment tool. Osteoporos Int. 2025 Sep;36(9):1725-1741. doi: 10.1007/s00198-025-07607-w.	Yang 2016 included participants aged 40 years and over. The study reported that at baseline 75% of subjects with a parent who had a hip fracture were aged between 40-44 years old. 86% of subjects who reported no parental hip fractures were also aged 40—44 years. This refers to subject's age rather than parental hip fracture age. Parental fractures were obtained from hospital records. It was thought that most hip fractures would be picked up by hospitalisation records. Parental hip fracture is still a risk factor at any age, however, the committee wanted to highlight that the risk appears to be less significant after the age of 80.
National Osteoporosis Guideline Group UK	Guideline	4	9	Revise recommendation 1.1.2 from consider to assess Conditional recommendations begin with the term 'consider' and apply where the clinician examines the evidence within the wider health and social context and discusses the choices with the patient, taking into account the patient's values and preferences, or where documentation of the discussion of the pros and cons of an intervention is the indicator of quality, rather than the course of action itself.	Thank you for participating in the consultation process. Evidence was not available to make a stronger recommendation and 'consider' reflects NICE process on the wording of recommendations.

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				Strong recommendations begin with action verbs like 'advise', 'assess', 'conduct', 'measure', 'offer', 'plan', 'refer', 'review', 'start', 'treat' and similar. These apply where the clinician reasons that most patients ought to receive the intervention, or where adherence to the recommendation could be used as a performance or quality indicator and that deviation from this recommendation would prompt documentation of a clinician's rationale for doing so. In this basis recommendation 1.1.2 should be phrased 'Assess fragility fracture risk in....' Use of verb 'consider' instead of assess risks an opportunistic model of assessment which will widen health inequities. There are several key indications in this list in which we consider the verb should read 'assess' instead of consider, including, but not limited to, low BMI, people with rheumatoid arthritis, people with coeliac disease, hyperparathyroidism etc.	
National Osteoporosis Guideline Group UK	Evidence Review C	5		It would have been appropriate to include osteoporosis medication use as an outcome. Given that osteoporosis medication is well evidenced to increase total hip bone density, and total hip bone density is now recognised as a surrogate endpoint for fracture by the FDA.	Thank you for participating in the consultation process. Review C assesses the validity of fracture risk prediction models, not their impact and includes relevant outcomes including discrimination and calibration. As such, osteoporosis medication use is not an appropriate outcome for this purpose.
National Osteoporosis Guideline Group UK	Evidence Review B	5	3	"What is the clinical and cost effectiveness of searching and analysing electronic health records, imaging archives (PACS) and social care records (including GP 6 practice lists) to facilitate identification of adults who should be assessed for fragility fracture risk?".	Thank you for participating in the consultation process. Imaging archives (PACS) have been added to the research recommendation.

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				PACS stands for Picture archiving and communication system	
National Osteoporosis Guideline Group UK	Guideline	5	Table 1	Table 1 is missing a number of secondary causes of osteoporosis Key risk group missing include neurological disease such as Parkinsons Disease. Parkinsonism, dementia, multiple sclerosis, epilepsy, stroke etc Another group relates to muscle disorders, that increase risk of falls and therefore fractures, e.g. myositis, myopathies and dystrophies, sarcopenia Key risk factor missing is Systemic Lupus Erythematosus which has a similar inflation to fracture risk as Rheumatoid arthritis. This is not adequately covered by 'other inflammatory arthropathies'. HIV is a well-established risk factor for fracture Bariatric surgery should be specified under 'Gastrointestinal' Osteomalacia should be added under 'Rheumatological' Asthma should be added under 'Respiratory' Consider Adult learning disabilities: e.g., Down's Syndrome CKD Stage should be clarified Nuance is needed re. Hypogonadism - if managed and well controlled this is not a risk factor, whilst if unmanaged and/or uncontrolled it is.	Thank you for participating in the consultation process. Please note that we have amended this table and added some of the suggestions when related to a previous example already on the list. Added risk factors include multiple sclerosis, Parkinson's Disease, SLE, and stroke. We have also clarified that CKD stages 4 and 5 are associated with increased fracture risk. However, other examples where we have not looked at the evidence have not been included (for example, dementia, asthma, osteomalacia, bariatric surgery, Down's syndrome and epilepsy). Hypogonadism is included as a risk factor to consider and then can be further assessed to determine if well managed and controlled. Whilst HIV is not listed as a specific example, but the committee included antiretrovirals medicines which would cover people with HIV. We have included 2 or more falls as a risk factor and also linked to the NICE guideline on falls. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors.

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National Osteoporosis Guideline Group UK	Evidence Review F	6		The text indicates VFRAC is for use by a nurse, but this is not correct, it is designed for use by any primary care healthcare professional	Thank you for participating in the consultation process. The text has been updated replacing 'nurse' with 'primary care healthcare professional' as suggested.
National Osteoporosis Guideline Group UK	Guideline	6	14	We strongly propose that NICE advocate an EPR solution to embed fracture risk assessment into all primary care systems, which will help with the complexity mentioned above.	Thank you for participating in the consultation process. Evidence was not available to make a recommendation in this area and a research recommendation was made instead.
National Osteoporosis Guideline Group UK	Guideline	6	2	Clarity on assessment indications for younger adults are welcomed, as this is out of the scope of NOGG guidance	Thank you for participating in the consultation process and support for these recommendations.
National Osteoporosis Guideline Group UK	Guideline	7	23	HRT needs to be defined as systemic (not topical oestrogen)	Thank you for participating in the consultation process. We have amended the recommendations to explicitly refer to 'systemic' HRT.
National Osteoporosis Guideline Group UK	Guideline	7	27	Clarification is needed by what is meant by 'the benefit may continue for longer in those who take HRT for longer'. How does this link to the preceding sub-bullet? HRT works while it is being taken, when you stop it, the benefit stops.	Thank you for participating in the consultation process. The recommendation has been updated removing the bullet points and simplified to "When assessing risk, take into account that fragility fracture risk is decreased while taking systemic hormone replacement therapy (HRT)."
National Osteoporosis Guideline Group UK	Guideline	7	6	We appreciate the focus on the anabolic-first strategy. But we query 1.3.1 offering DXA in people 50-90 with a previous hip or vertebral fragility fracture or 2 or more fragility fractures. We understand why this is there, to encourage identification of high-risk patients who may fulfil guidance for parenteral	This recommendation has been updated and moved to the section on DXA so that all recommendations related to BMD assessment come together. A visual summary also

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				treatments, ensuring they have BMD assessment. However, the order and wording is awkward, since readers might read this sentence right at the start of guidance to limit DXA to those with these either/or conditions “Offer a dual-energy X-ray absorptiometry (DXA) scan to measure bone mineral density (BMD) when assessing fragility fracture risk (unless not needed for treatment decisions or monitoring) in people aged between 50 and 90 who have had either: • a previous hip or vertebral fragility fracture or • 2 or more fragility fractures”. For instance, for romosozumab, eligibility is presently a MOF within 24 months and T-score <−2.5. Obviously, in clinical practice DXA requests are determined by very high/imminent risk from MOF, then by FRAX risk with NOGG recommendation based on FRAX. Clarity would be improved by placing the recommendations about DXA together (indications for DXA are: x,y,z) and all the recommendations about risk assessment together (including when and when not indicated).	accompanies the recommendations to make them easier to follow. The recommendation has also been updated to cover everyone over the age of 30 who meet these criteria.
National Osteoporosis Guideline Group UK	Evidence Review A	8	10	We support the amendment to the more specific term of first-degree relative's hip fracture.	Thank you for participating in the consultation process and your support for the recommendation.
National Osteoporosis Guideline Group UK	Evidence Review A	8	16	Reduced balance in people with osteoporosis can also be due to vertebral fractures causing hyperkyphosis which changes one's centre of gravity.	Thank you for participating in the consultation process. Thank you for your suggestion, we have added this into the committee discussion in evidence review A.

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Stakeholder	Document	Page No	Line No	Comments	Developer's response
National Osteoporosis Guideline Group UK	Guideline	8	3	Guard against ageism and therapeutic nihilism Re: 1.3.8 'Use clinical judgement to decide how to assess fragility fracture risk for people aged over 90' Greater information is needed to qualify such a statement to prevent it being used as an ageist excuse to ignore fracture risk in those 90+. At the age of 90, men live on average a further 4 years and women on average a further 5 years in the UK (see ONS data 2025). This provides plenty of time for anti-osteoporosis medicines to prevent a painful and expensive death by hip fracture. We strongly urge NICE to guard against ageism and therapeutic nihilism in clinical practice. At the age of 90 your fracture risk is greater than at any other preceding age. Note that FRAX, rather than Qfracture, integrates the death hazard and the fracture hazard so that fracture probabilities in older people reflect average remaining lifetime risk.	Thank you for participating in the consultation process. The recommendations were not intended to deprive older people of treatment, rather to highlight the limits of the risk assessment tools. The recommendation has been amended to make clear that the risk prediction tools are intended for use in specific age ranges: FRAX from 40-90; QFracture 30-100 with note that after aged 90, the estimated risk decreases by 1 year for every additional year (e.g. the fracture risk for a 91-year old is 9-year fracture risk). The point about FRAX taking into account mortality where QFracture doesn't is mentioned in the committee discussion of the full evidence report C.
National Osteoporosis Guideline Group UK	Guideline	8	7	Consideration of measuring BMD in someone under the age of 30 needs careful justification under IR(ME)R regulations.	Thank you for participating in the consultation process. The recommendation has been amended to remove reference to DXA and specifies that specialist advice should be sought.
National Osteoporosis Guideline Group UK	Guideline	9	12	While promotion of VFA is welcomed, the reality is that VFs are identified mainly by lateral radiograph in England and Wales. VFA is of very variable quality depending on the scanner type and lateral radiograph is the definitive (gold standard) diagnostic method to identify fractures. VFA on	Thank you for participating in the consultation process. Committee experience was while some areas do not use VFA

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				every DXA is not evidenced as cost-effective and will significantly increase burden on DXA services. Substantial investment would be needed in expanding capacity of trained operators and DXA reporters for this to be implemented. VFA in mobile adults adds 5-7 minutes to scanning time and a further 5-7 minutes to reporting time for an experienced reporter. VFA in frailer, less mobile adults adds 7-10 minutes to scanning time and 7-10 minutes to reporting time (as osteoarthritic degeneration of the spine makes reporting more difficult). Cumulatively this will place substantial extra pressures on DXA services. NOGG makes the following recommendation, which NICE might consider: Vertebral fracture assessment (VFA) is indicated in postmenopausal women, and men age ≥ 50 years, if there is a history of ≥ 4cm height loss, kyphosis, recent or current long-term oral glucocorticoid therapy, a BMD T-score ≤ -2.5 at either the spine or hip, or in cases of acute onset back pain with risk factors for osteoporosis	currently, other areas are already using it in selected patient groups having DXA. Cost-effectiveness evidence was available in people 50 years and over with fracture having DXA and people aged 65 to 80 years at high risk of vertebral fracture having DXA. However, the committee agreed that due to current low identification rates of vertebral fractures wider recommendations to consider VFA when doing DXA were also likely to be cost effective. They have now prioritised recommendations for groups where risk of vertebral fracture is higher – women aged 70 and over, men aged 60 and over and selected groups of younger people with additional risk factors. The committee acknowledge that implementation of the recommendations will require additional training and ensuring sufficient resourcing related to VFA, especially reporting capacity. It was considered feasible to accommodate the additional scanning time within existing DXA slots in most cases. A substantial resource impact to the NHS overall was not anticipated as increased costs related to VFA were considered likely to be offset by reductions in spinal radiographs and reductions in fractures. Health outcomes for patients would also be improved through increased and better targeted treatment. Vertebral fractures are normally identified in clinical practice using conventional radiography, but vertebral fractures are underdiagnosed. DXA uses a lower dose of ionising radiation compared to conventional radiography to image the spine and

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					provides additional information on bone health. The specificity of DXA with VFA was very high with the point estimates for the per-person and per-vertebra analyses both greater than 0.9, with a low probability of misidentifying vertebral fractures. Additionally, the committee made a research recommendation on accuracy of DXA based VFA scans for identifying vertebral fractures in adults having a DXA scan. This question is to determine the accuracy of the newer generation scanners which have increased image quality and resolution which should result in higher discriminatory accuracy for detecting vertebral fractures.
National Osteoporosis Guideline Group UK	Evidence Review A	9	14	It seems confusing that NICE is using – ‘multiple fracture or a hip or vertebral fracture’, and not using the term ‘Major Osteoporotic Fracture’ (MOF) for hip, vertebral, humerus, wrist fractures when it is known that MOFs confer the higher imminent risk of further fractures.	Thank you for participating in the consultation process. The recommendation uses the term previous fragility fracture which includes spine, hip, shoulder and wrist because the committee agreed the recommendation applies to any fragility fracture, not just major osteoporotic fractures. The evidence report is stating that previous fragility fractures increase the risk especially if it is spine, hip or multiple fractures. This review and corresponding recommendations are about identifying people at risk of fracture and happens before osteoporosis has been identified as the potential cause.
National Osteoporosis Guideline Group UK	Evidence Review A	9	15-16	‘with risk being greatest in younger people and diminishes with time’ – This sentence is unclear. Does this mean – risk being greatest in older people? The risk of subsequent fracture does diminish with time, but it would be clearer as a	Thank you for participating in the consultation process. The committee agree and have clarified the wording.

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				separate sentence. Here it may be helpful to discuss Imminent Fracture Risk.	
National Osteoporosis Guideline Group UK	Guideline	9	4	1.4.4 reads: Make a shared decision with the person about whether to treat without a DXA scan for people... for whom a DXA scan is not needed for treatment decisions or monitoring. This seems to be a paradoxical sentence? 'Not needed for treatment decisions or monitoring' should be defined/clarified	Thank you for participating in the consultation process. The recommendation has been updated with a cross reference to a more detailed recommendation in the treatment criteria section. Further information is also given in the rationale and people living with frailty is used as an example.
National Osteoporosis Guideline Group UK	Evidence Review A	9	46	Section 1.1.10.2. For people age under 50, this suggests that - eg a 48 year old woman with a fragility fracture of her wrist (and not on glucocorticoids or early menopause) would not need to have her osteoporosis risk assessed, with which all clinicians would disagree. Any person with a fracture of the wrist or humerus should have their osteoporotic risk assessed, particularly for women in their 40's, who may be perimenopausal, or e.g. a 46 year old post-menopausal woman with a wrist fracture, where in clinical practice risk assessment may lead to consideration of osteoporosis treatment/HRT to reduce fracture risk.	Thank you for participating in the consultation process. The guideline has recommended risk assessment for anyone who has had a previous fragility fracture. This is a strong recommendation for anyone who has had a previous hip or vertebral fracture or multiple major osteoporotic fractures and for all men over 50 and women who have experienced menopause. It has also recommended considering risk assessment in men under 50 and women who have not experienced the menopause with either a previous non hip non vertebral fracture, (which would include a wrist fracture), or untreated menopause (recommendation 1.1.4). Evidence was not available to make a stronger recommendation for this group. These recommendations on risk factors have been updated from people above and below 50 to men above and below 50

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					and women who have and have not experienced the menopause.
National Osteoporosis Guideline Group UK	Guideline	10	1	Height loss needs quantifying, and the method of assessment needs to be described	Thank you for participating in the consultation process. The committee agree and have added the International Society for Clinical Densitometry (ISCD) criteria of a height loss of more than 4 cm.
National Osteoporosis Guideline Group UK	Evidence Review A	10	39	As currently many patients on glucocorticoids have not had an osteoporosis risk assessment, if this guideline is implemented, it will likely increase demand for osteoporosis services and cost.	Thank you for participating in the consultation process. Previous recommendations also advised that people using glucocorticoids are risk assessed and therefore this does not represent a substantial change. However, the rationale does acknowledge that the recommendations may raise awareness and increase the number of people being assessed for fracture risk. If this is the case there may be increased resource use related to risk assessment, dual energy X ray absorptiometry (DXA) scans, and treatment, but savings because of reduced fragility fractures in the long term.
National Osteoporosis Guideline Group UK	Evidence Review A	10	44	Use of a fixed threshold of 10% is inequitable.	Thank you for participating in the consultation process. The committee do not agree this is inequitable and think it ensures people with a similar absolute risk are all equally risk assessed.
National Osteoporosis Guideline Group UK	Guideline	10	8	Exceptionally low BMD for age should be defined/clarified	Thank you for participating in the consultation process. The recommendation has been amended to read 'exceptionally low BMD for their age (for example, a Z-score of less than -2)'.
National Osteoporosis	Evidence Review A	11	11	There is likely to be an increase in resource needed, particularly (1.1.1) the recommendation regarding people	Thank you for participating in the consultation process. The committee noted that some areas have capacity issues with

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Guideline Group UK				over age 50 on glucocorticoids. This is important because there is currently low provision of DXA, osteoporosis clinics and provision of injectable osteoporosis drugs, which don't meet current demand. Services have also been stretched recently by the increased use of osteo-anabolic drugs.	DXA and so there can be long waiting lists. NHS England data from February 2026 reported a median waiting time for DXA of around 2.6 weeks with 12% of people on the waiting list having been so for 6 weeks or more and 3% for 13 weeks or more. They noted that NHS England committed to funding 13 new scanners in 2025 and another 20 new scanners in 2026 – this included a mix of replacement machines and additional machines to increase capacity. This has been added to the resource use discussion in Evidence report A.
National Osteoporosis Guideline Group UK	Guideline	12	1	BMD Z-Scores rather than T-Scores are recommended to be used in this age group by the ISCD. Reference International Society for Clinical Densitometry (ISCD). Adult Official Positions of the ISCD. <u>https://iscd.org/learn/official-positions/adult-positions/</u>, 2019.	Thank you for participating in the consultation process. The specific BMD score to use is not mentioned in the recommendation and the committee anticipate the healthcare professional assessing the person's risk will choose which score to use.
National Osteoporosis Guideline Group UK	Guideline	12	14	This recommendation should include advice re. lifestyle modifications and exercise	Thank you for participating in the consultation process. Part 2 of the guideline will include reviews on the clinical and cost-effectiveness of exercise interventions and the addition of vitamin D/calcium to other pharmacological interventions.
National Osteoporosis Guideline Group UK	Guideline	12	9	This recommendation should include advice re. lifestyle modifications and exercise	Thank you for participating in the consultation process. Part 2 of the guideline will include reviews on the clinical and cost-effectiveness of exercise interventions and the addition of vitamin D/calcium to other pharmacological interventions.

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National Osteoporosis Guideline Group UK	Guideline	14	16	Use of electronic health records Research recommendation 1 <i>Using electronic health records to help identify adults who should have a fragility fracture risk assessment.</i> Please expand this to include imaging (PACS) as well as 'health records'. UK research is world leading in opportunistic identification of fragility fractures and osteoporosis from <i>imaging PACS records</i> and in several centres (due to NHS Digital and NIHR funded research) this has now crossed over into routine clinical practice. Revised statement would read, "What is the clinical and cost effectiveness of searching and analysis of electronic health records, imaging archives (PACS) and social care records (including GP 6 practice lists) to facilitate identification of adults who should be assessed for fragility fracture risk?". PACS stands for Picture archiving and communication system.	Thank you for participating in the consultation process. PACS has been added as an example of an electronic health record to the research recommendation.
National Osteoporosis Guideline Group UK	Guideline	25	3	The draft guideline states: "However, the committee agreed predicted risk alone should not be used to determine whether treatment is appropriate, beyond being used to determine eligibility for DXA scans. While it gives a probability of fracture, it still needs to be put into the context of a person's risk factors and clinical circumstances." The reasoning behind this is unclear: why is BMD T-Score, particularly a BMD threshold, seen as largely 'immune' from consideration of fracture probability (person's risk factors) and	Thank you for participating in the consultation process. The committee have amended the recommendations to make clear that although an individual can be eligible for pharmacological treatment on the basis of BMD score, it is not the only factor to taken into account. The committee recommend that a shared decision is made with the person about whether to start treatment. This should be a holistic approach including discussion about risk factors (including those not in the tools) previous fractures and BMD if

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				clinical circumstances? This is not logical and is not adequately justified in the draft guideline. Very many studies have shown BMD as a variable included in risk algorithms outperforms the use of BMD alone in predicting fracture risk. To simply say later on that "Overall, the committee agreed that a BMD measurement allows clinicians and people at risk of fragility fracture to make informed choices about whether treatment is appropriate, and which one would be the best option" ignores the assessment of the "risk of fragility fracture" included in the statement. This reinforces the lack of an underlying evidence base in support of the conclusion that BMD is the major driver of treatment decision-making.	available. The subsequent recommendation (1.7.3) gives more specific criteria to consider. Although fracture risk is not included in the criteria, risk factors would have been assessed to determine the need to measure BMD and to be considered for treatment. It is also clear that people without BMD can still start treatment and this recommendation has been moved to sit next to the other recommendations on treatment to clarify this.
National Osteoporosis Guideline Group UK	Evidence Review G	26	20 and throughout 1.3.5.	The proposed utility of 'routine' VFA in subgroups undergoing DXA is likely to positively address the treatment gap in osteoporosis, identifying more higher risk patients, allowing access to effective treatments to prevent additional fractures. The opportunity cost benefit of performing VFA during a DXA examination has not been adequately captured within the committee discussion. The ability of a patient to receive two risk assessments (DXA + VFA) at one sitting is powerful, offering convenience to the patient and the wider health system. This is a practical/real-world advantage of 'bundling' VFA with DXA in those who meet subgroup criteria. To avoid follow-on plain radiographs in unequivocal VFA-determined fractures is helpful. Again, this has cost savings associated and reduces demand on radiology departments Furthermore, and not highlighted, there will be reduced use of ionising radiation. In older patients, vertebral fractures are	Thank you for participating in the consultation process. The discussion has been amended to emphasise the noted benefits of conducting routine DXA with VFA scans in 1.3.5. Please note that benefits in terms of ionising radiation are covered in section 1.3.4.

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				often associated with multimorbidity that requires additional independent (ionising) imaging for parallel pathologies, thus to avoid unnecessary plain X-rays will reduce total exposure.	
National Osteoporosis Guideline Group UK	Guideline	General (Recommendations 1.7.1 to 1.7.3 24)	General	Deciding whether treatment is appropriate requires clearer reasoning The sections on deciding whether treatment is appropriate and the reasons behind the committee’s decisions require clarification. For example, 1.7.1. recommends joint decision-making about the need for treatment with consideration of fracture risk (where available) and BMD (where available), but 1.7.2 on pharmacological treatment for men aged 50 years or over and women who have experienced menopause doesn’t mention fracture risk at all. It returns to an approach to intervention from the late 1990s where decisions were based largely on BMD thresholds and other, mainly fracture-related criteria, despite the fact that both of these parameters are also included in risk assessment algorithms and contribute to the magnitude of risk (using FRAX rather than Qfracture). The rationale for this appears to be based on opinion rather than an assessment of the available evidence – “The committee agreed that determining who would benefit from treatment is not straightforward. Based on their <i>knowledge and experience</i> they agreed that several factors need to be taken into account.”	Thank you for participating in the consultation process. The committee recommend that a shared decision is made with the person about whether to start treatment. This should be a holistic approach including discussion about risk factors (including those not in the tools) previous fractures and BMD if available. The subsequent recommendation (1.7.3) gives more specific criteria to consider alongside the previous recommendation (1.7.1). Although fracture risk is not included in the criteria, risk factors would have been assessed to determine the need to measure BMD and to be considered for treatment. It is also clear that people without BMD can still start treatment and this recommendation has been moved to sit next to the other recommendations on treatment to clarify this. The SCOOP study showed a benefit in reduced hip fractures with FRAX and BMD compared to usual management. However, there was no difference for osteoporotic fractures, clinical fractures, mortality or quality of life outcomes between groups. The number of people at very high risk (14%) had a mean BMD -2.6, which would include some below and above the T-score -2.5 threshold. However, the study only included women between 70-85.

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				The limitations of this BMD-centred approach has been illustrated in many published studies. For example, in the SCOOP study in England [Shepstone 2018], a small group of women age 70-85 years at very high risk of hip fracture was identified within primary care using the FRAX tool. This very high-risk group, comprising only 14% of women in the intervention arm, contained a higher proportion of women with FRAX risk factors not just confined to previous fractures (Table 2) with a mean femoral neck T-score of -2.5 (i.e. half were above the T-score osteoporosis threshold). Intervention reduced the overall hip fracture incidence by 28% despite treatment being targeted only at this small group [Shepstone 2018; Chotiyarnawong 2022]. References Shepstone L, Lenaghan E, Cooper C, et al. (2018) Screening in the community to reduce fractures in older women (SCOOP): a randomised controlled trial. Lancet 391:741–747 Chotiyarnwong P, McCloskey EV, Harvey NC, et al (2022) Is it time to consider population screening for fracture risk in postmenopausal women? A position paper from the	The committee were aware of a recent pooled analysis of individual patient data from 25 trials (Schini 2024) that found that anti-osteoporotic medications reduced the risk of fractures regardless of baseline BMD, although the effect was greater for those with lower BMD scores. The post hoc analysis of the FRAME study showed a higher incidence of nonvertebral fractures in romosozumab group compared to placebo for people recruited from Latin America and North America. However, there was a risk reduction in the other countries (central/Eastern Europe, Western Europe, Australia/New Zealand, Asia Pacific). This shows a risk reduction in people recruited from Western Europe of 77% vertebral fractures and 79% non-vertebral fractures with treatment for people with BMD T-scores <-2.5 which support our treatment criteria. The committee have amended the recommendations to make clear that although an individual can be eligible for pharmacological treatment on the basis of BMD T-score, it is not the only factor to take into account.

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				International Osteoporosis Foundation Epidemiology/Quality of Life Working Group. Arch Osteoporos 17:87 <table><tr><th>Characteristic</th><th>Question completion (%)</th><th>Control*</th><th>Screening*</th><th>High risk subgroup</th></tr><tr><td>Number</td><td></td><td>6250</td><td>6233</td><td>898</td></tr><tr><td>Age (y)</td><td>100</td><td>75.6 ± 4.1</td><td>75.6 ± 4.2</td><td>77.7 ± 4.4</td></tr><tr><td>BMI (kg/m²)</td><td>100</td><td>26.7 ± 4.8</td><td>26.7 ± 4.7</td><td>24.4 ± 4.1</td></tr><tr><td>Prior fracture (%)</td><td>99.1</td><td>23.4</td><td>22.4</td><td>45.5</td></tr><tr><td>Parental history of hip fracture (%)</td><td>92.6</td><td>9.2</td><td>9.4</td><td>39.4</td></tr><tr><td>Glucocorticoid use (%)</td><td>94.8</td><td>5.0</td><td>5.1</td><td>12.6</td></tr><tr><td>Rheumatoid arthritis (N, %)*</td><td>94.7</td><td>6.6</td><td>6.8</td><td>8.8</td></tr><tr><td>Smoking (%)</td><td>99.9</td><td>4.6</td><td>4.7</td><td>9.6</td></tr><tr><td>Alcohol (%)</td><td>99.7</td><td>3.6</td><td>3.5</td><td>6.7</td></tr><tr><td>Secondary osteoporosis (%)</td><td>99.8</td><td>22.5</td><td>23.8</td><td>29.7</td></tr><tr><td>History of falls (%)</td><td>99.0</td><td>27.2</td><td>28.0</td><td>32.9</td></tr><tr><td>FRAX 10 year MOF probability (no BMD) (%)</td><td>-</td><td>19.3 ± 8.8</td><td>19.3 ± 8.9</td><td>30.0 ± 10.1</td></tr><tr><td>FRAX 10 year Hip probability (no BMD) (%)</td><td>-</td><td>8.5 ± 7.3</td><td>8.5 ± 7.4</td><td>17.9 ± 10.9</td></tr><tr><td>Femoral neck BMD T-score</td><td>-</td><td>-</td><td>-1.70 ± 1.02</td><td>-2.55 ± 0.68</td></tr></table> Table 2. Baseline characteristics of SCOOP study participants recruited from primary care using the FRAX questionnaire. Note that all FRAX risk factors, not just prior fracture, are present at a higher prevalence in the high-risk subgroup targeted for intervention. At the other end of the fracture risk spectrum, a T-score of -2.5 does not always mean high fracture risk; for example, in	Characteristic	Question completion (%)	Control*	Screening*	High risk subgroup	Number		6250	6233	898	Age (y)	100	75.6 ± 4.1	75.6 ± 4.2	77.7 ± 4.4	BMI (kg/m²)	100	26.7 ± 4.8	26.7 ± 4.7	24.4 ± 4.1	Prior fracture (%)	99.1	23.4	22.4	45.5	Parental history of hip fracture (%)	92.6	9.2	9.4	39.4	Glucocorticoid use (%)	94.8	5.0	5.1	12.6	Rheumatoid arthritis (N, %)*	94.7	6.6	6.8	8.8	Smoking (%)	99.9	4.6	4.7	9.6	Alcohol (%)	99.7	3.6	3.5	6.7	Secondary osteoporosis (%)	99.8	22.5	23.8	29.7	History of falls (%)	99.0	27.2	28.0	32.9	FRAX 10 year MOF probability (no BMD) (%)	-	19.3 ± 8.8	19.3 ± 8.9	30.0 ± 10.1	FRAX 10 year Hip probability (no BMD) (%)	-	8.5 ± 7.3	8.5 ± 7.4	17.9 ± 10.9	Femoral neck BMD T-score	-	-	-1.70 ± 1.02	-2.55 ± 0.68	
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				the FRAME study of romosozumab, the treatment was found not to significantly reduce fracture risk in patients recruited in Latin America despite all having BMD T-scores <-2.5; this reflected a low absolute risk of fracture as reflected by FRAX assessments [Cosman 2018; McCloskey 2021]. References Cosman F, Crittenden DB, Ferrari S et al (2018) Romosozumab FRAME Study: A Post Hoc Analysis of the Role of Regional Background Fracture Risk on Nonvertebral Fracture Outcome. J Bone Miner Res 33:1407–1416 McCloskey EV, Johansson H, Harvey NC, Lorentzon M, Shi Y, Kanis JA (2021) Romosozumab efficacy on fracture outcomes is greater in patients at high baseline fracture risk: a post hoc analysis of the first year of the frame study. Osteoporos Int 32:1601–1608	
National Osteoporosis Guideline Group UK	Evidence Review D	34	5-8	Diagnosing osteoporosis with femoral neck QCT (Quantitative Computed Tomography) based on equivalence to DXA is missing from evidence Review D. The evidence review remit specifically included “<i>Bone mineral density (BMD), bone mass measured by dual energy xray absorptiometry (DXA) and quantitative computed tomography (QCT) for consideration</i>”. But this did not happen for QCT equivalence to DXA. In fact, all relevant research on QCT equivalence to	Thank you for participating in the consultation process. Review D is on the accuracy of various bone assessment methods to predict fragility fracture rather than (i) their use to diagnose osteoporosis, and (ii) the accuracy of imaging methods compared to DXA to estimate bone mineral density. As such the reference standard for this review was clinical review, self-report, and radiographic confirmation of fracture.

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				DXA was omitted or excluded from Evidence review D. This is the work that led, in 2025, to UK adoption of QCT by the NHS, Royal College of Radiologists and UKAS. Specifically, what needs rectifying is to include the work showing that femoral neck QCT BMD is equivalent to DXA, since Evidence Review D only looked at one Leonhardt paper from 2020. Leaving the equivalence work out disadvantages patients and effectively invalidates existing CE marked technologies currently in use in the NHS. If left unaddressed, current NHS clinical practice (using the DXA-equivalent QCT femoral neck BMD via the specific drop-down menu within FRAX) would be likewise invalidated by exclusion by this NICE draft. Not only is that contrary to NOGG (previously NICE approved), it means of 61 countries worldwide using FRAX in this way, clinicians in England, Wales & NI could no longer do so 'officially'. Leaving out this proven diagnostic method for osteoporosis from NICE guidelines puts the UK at a disadvantage when adopting novel CE-marked, approved diagnostic technologies. Bonescreen SpineQ, Mindways QCT, Nano HealthOST, Virtuost are just a few operating in this space, making use of ICSD nationally-agreed equivalence guidelines. Specific omission: Evidence D literature review for NICE guidance summarily excludes all of the work on areal BMD and QCT to DXA equivalence as adopted by the	Thank you for bringing our attention to the studies cited. We have updated the review to include the study Yang 2012 which does fit our protocol. We have looked at all the references, please see below for specific responses: Adams 2010: This is a biomechanical CT case-cohort study in people with previous CT scans. This study uses DEXA BMD as the reference standard and therefore would not have been included in the review. Cann 2014: This study shows a correlation between DXA and 3D-QCT and does not report any outcome of interest to review D and therefore would not have been included in the review. Johannesdottir 2018: This is a review article which summarises the state and future of CT and finite element analysis in fracture prediction. We have identified one article from this review (Yang 2012) that we agree should be included in review D. The remaining studies cited in Table 1 of this article would not have been included in review (reasons include: mean age of male cohort was less than 75 years; inappropriate study design, e.g. association study, retrospective cohort study). Thank you for bringing this to our attention. Lenchik 2018: This is a start of the art review of CT screening that argues that CT can be used to opportunistically screen rheumatology patients who are having CT scan. There are no relevant articles cited in this review that would have been included in review D. Pickhardt 2015: This study shows a correlation between DXA aBMD and QCT aBMD measurements but is not a fracture

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				International Society for Clinical Densitometry (ICSD). See statements from NOGG 2024 guidelines Statements 9,10 and Evidence backing statements 11,12 below. Evidence review D took only a superficial look at the QCT evidence base, specifically missing the 5 key NOGG evaluated research findings below. As a reminder, these are the recommendations that appear in National UK guidelines (NOGG) concerning QCT, based on good quality evidence: Guideline statements: QCT-measured femoral neck areal BMD in postmenopausal women, and men age ≥50 years, can be used for opportunistic diagnosis of osteoporosis and to inform individual treatment decisions using FRAX (Conditional recommendation). Computer Aided Diagnostics (CAD) may be considered to improve standard reporting of CTs performed on postmenopausal women, and men age ≥50 years, to improve opportunistic identification of vertebral fractures (Conditional recommendation). Evidence backing statements: QCT-measured femoral neck areal BMD predicts osteoporotic fractures in men and women and is equivalent to DXA-derived areal BMD.	prediction study and so would not have been included in review D. Ziemlewicz 2016: This is CXDA and QCT correlation study and uses DXA as reference standard, and so would not have been included in review D. There was limited evidence identified for QCT and DXA with TBS. The committee have added two new research recommendations to address this evidence gap, although these research recommendations were not prioritised as key research recommendations. Please see section on research recommendations in evidence review D for further details.

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				Femoral neck and total hip T-scores calculated from two-dimensional projections of quantitative computed tomography (QCT) data are equivalent to the corresponding DXA-derived T-scores. This means femoral neck CT X-ray absorptiometry (CTXA) BMD measurements can be included in FRAX; (Evidence level IIa). References: • International Society for Clinical Densitometry (ISCD). Adult Official Positions of the ISCD. https://iscd.org/learn/official-positions/adult-positions/, 2019. • Adams AL, Fischer H, Kopperdahl DL, et al. Osteoporosis and Hip Fracture Risk From Routine Computed Tomography Scans: The Fracture, Osteoporosis, and CT Utilization Study (FOCUS). J Bone Miner Res 2018; 33(7): 1291-301. • Johannesdottir F, Allaire B, Bouxsein ML. Fracture Prediction by Computed Tomography and Finite Element Analysis: Current and Future Perspectives. Curr Osteoporos Rep 2018; 16(4): 411-22. • Pickhardt PJ, Bodeen G, Brett A, Brown JK, Binkley N. Comparison of femoral neck BMD evaluation obtained using Lunar DXA and QCT with	

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				asynchronous calibration from CT colonography. J Clin Densitom 2015; 18(1): 5-12. • Cann CE, Adams JE, Brown JK, Brett AD. CTXA hip--an extension of classical DXA measurements using quantitative CT. PloS one 2014; 9(3): e91904-e. • Ziemlewicz TJ, Maciejewski A, Binkley N, Brett AD, Brown JK, Pickhardt PJ. Opportunistic Quantitative CT Bone Mineral Density Measurement at the Proximal Femur Using Routine Contrast-Enhanced Scans: Direct Comparison With DXA in 355 Adults. J Bone Miner Res 2016; 31(10): 1835-40. Lenchik L, Weaver AA, Ward RJ, Boone JM, Boutin RD. Opportunistic Screening for Osteoporosis Using Computed Tomography: State of the Art and Argument for Paradigm Shift. Current rheumatology reports 2018; 20(12): 74.	
NHS Greater Manchester	Guideline	General	General	The guideline does not make specific reference to frailty. Patients living with high levels of clinical frailty are at increased risk of falls, osteoporosis and fractures. It would make sense for this cohort to be identified particularly as many of these patients would qualify for treatment based on their clinical history alone.	Thank you for participating in the consultation process. NICE's guideline NG249 <i>Falls: assessment and prevention in older people and in people 50 and over at higher risk</i> is referred to in recommendation 1.1.2. Frailty is also taken into account in the recommendation 1.7.4 related to treatment decisions to ensure people living with frailty are not deprived of treatment. It will also be discussed as part 2 of the guideline where the committee will consider whether specific recommendations are needed for this group.
NHS Greater Manchester	Guideline	General	General		Thank you for participating in the consultation process. Service structure is out of the scope of this guideline. However, these

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				As an ICS we would welcome advice on service structure and further clarity on where in the system these guidelines apply. Fracture liaison services are mentioned in the discussion but not the main guidance, should advice around these services be brought into the body of the guidelines themselves? The guidelines could be strengthened in their description of where and when they should be applied. Too often, bone health is seen as a specialist issue when in fact it can be assessed and treated by a wide range of staff who meet a patient who is at risk of osteoporosis.	guidelines apply wherever NHS care is delivered and not just specialist services.
NHS Greater Manchester	Guideline	7	2	Paragraph 1.3.1 is unclear. What does ' <i>unless not needed for treatment decisions or monitoring</i> ' mean? This paragraph could be reworded more clearly to exclude patients for whom risk prediction suggests treatment without a DXA, or patients who have suffered a fragility fracture and are over 90.	Thank you for participating in the consultation process. The recommendation has been updated with a cross reference to a more detailed recommendation in the treatment criteria section that provides further details about when DXA is not needed for treatment decisions or monitoring. This recommendation (1.7.4) clarifies when to start pharmacological treatment without a DXA scan if their condition meets the criteria but for whom 'a DXA scan is not tolerated or technically feasible or not needed to inform decision to start treatment or to provide a baseline for long-term monitoring (for example for people living with frailty)'. Further information is also given in the rationale and people living with frailty or who are housebound is used as an example.
NHS Greater Manchester	Guideline	8	3		Thank you for participating in the consultation process. We have amended recommendation 1.7.3 to specify that

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				People over 90 represent a large proportion of hospital inpatients who experience fragility fractures. Is there scope to expand guidance for this patient population beyond this line? In hip fracture care, many people over 90 with high levels of clinical frailty are given a single dose of IV zoledronic acid based on their age and hip fracture alone. Too many hip fracture patients are discharged without bone protection – see this consensus/call to action paper: Call to action: a five nations consensus on the use of intravenous zoledronate after hip fracture - PMC. This guideline has the opportunity to make it clear to clinicians that many older people living with frailty who have experienced previous fragility fractures can be considered for, and offered treatment for osteoporosis without onward referral for DXA or further assessment.	pharmacological treatment should be considered for anybody over 50 who has had a hip or fragility fracture, independently of a whether or not a BMD measurement is available. However, recommendation 1.7.1 makes clear that clinical assessment of fracture risk factors must be conducted when considering treatment. Specific treatments to offer, such as IV zoledronic acid is included in part 2 of the guideline.
NHS Greater Manchester	Evidence review D	9	10	Evidence Review D (page 34, line 18) states that <i>"Measurement of BMD by DXA rather than other bone assessment techniques was recommended from this review because it had the most evidence and it is widely used in current practice."</i> This does not make sense in light of the earlier paragraph in the same evidence review (page 33, line 36) which notes that <i>"Overall, the discriminatory power of bone assessment methods to predict MOF and HF were similar for DXA, CT and ultrasound."</i>	Thank you for participating in the consultation process. The committee agree that QUS technology is safe, portable, and can be appropriate in absence of a DXA scan. However as indicated in the review, the evidence base for the accuracy of QUS of the heel to predict major osteoporotic fracture and hip fracture is limited with AUC for MOF and HF prediction ranging from poor to moderate and the associated wide 95% CIs indicating substantive uncertainty. The committee decided on this basis not to make a positive recommendation to use it.

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				It might be that, all other things being equal, DXA should be preferred. NICE's conclusion does not take into account that heel ultrasound can be done in the community and will often be more convenient. It may even be better sometimes in the hospital eg fractures increase after a stroke and it may sometimes be easier to do an heel ultrasound for these patients on the ward than take the patient to have a DXA. It also ignores radiation emitted by DXA. It may be reasonable to go along with NICE's recommendation if it added that heel ultrasound was an acceptable alternative when it increases accessibility. This is an important consideration when addressing health inequalities.	However, a research recommendation was made for REMS to possibly inform future guidance.
NHS Greater Manchester	Guideline	11	11	The wording of this paragraph could be confusing. It may be better worded as '<i>if the criteria for osteoporosis assessment are met</i>', as this group of patients will include people who have met the criteria for assessment (which COULD include a DXA scan) but cannot have a DXA scan because they are very frail, cannot lie flat etc. It's important that the guideline clearly identifies that for some patients (eg 80+, living with frailty, previous hip fracture) osteoporosis treatment can be offered without a DXA.	Thank you for participating in the consultation process. This sentence has been removed. The recommendation relating to making a decision to treat without a DXA scan because it is not tolerated, technically feasible or not needed for treatment or monitoring decisions has also been moved into this section.

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NHS Greater Manchester	Guideline	13	16	Is there any reason this section does not also cover follow up DXA scans for patients who have been offered treatment?	Thank you for participating in the consultation process. Follow up for people on treatment will be covered in part 2 of this guideline which will be published in 2027.
Northumbria NHSFCT	Guideline	4	6	Would be helpful to include a frequency for intermittent high dose steroid, eg 3+ rescue packs per year	Thank you for participating in the consultation process. The committee have added the example of 'using 3 rescue packs per year' in the guideline rationale and committee discussion in evidence report A.
Northumbria NHSFCT	Guideline	8	10	Why is this cut-off different from FRAX/NOGG for patients who are under 70 years old?	The committee did not use the FRAX-NOGG thresholds to determine eligibility for pharmacological treatment for the following reasons: 1. It leads to the under treatment of older people who would benefit most from treatment options. Relative risk reduction from pharmacological treatment is similar at all ages. However, absolute risk increases with age, and older people will benefit more from treatment than younger people. The committee do not think older people should have to reach a higher level of absolute risk to warrant treatment. 2. It can lead to over treatment of young people, or lots of young people being assessed but not treated as they are deemed not to be at a high enough risk to warrant treatment. This in turn can result in an assessment 'treadmill' which also has an associated resource cost, impacting on DXA services and potentially causing patient anxiety because of this cycle.

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					3. The committee agree that in principle it might seem attractive to start treatment early with the aim of preventing further deterioration in bone health. However, they also agreed there is a lack of evidence of long-term treatments. Unlike other conditions which have treatments that are safe, effective and work long-term there is no 'forever treatment' in osteoporosis. Osteoporosis treatments are recommended for 5 (up to 10) years before a treatment break is required. These treatments are also known to be associated with the occurrence of rare but severe adverse events which are related to dose/duration of treatment and some are only recommended for use once in a lifetime. This would mean treatment options later in life when a person is more likely to be at a high level of risk would be limited. In contrast, the NICE approach is based on the premise that treatment is best targeted to people at high enough levels of risk who are likely to experience an absolute risk reduction in fracture that outweighs possible adverse events. It also incorporates BMD t-score criteria in addition to absolute fracture risk as the committee agree this is important to ensure treatment is targeted at those most likely to achieve a risk reduction. The committee noted that the majority of RCT evidence is in people with BMD T scores below -1.5. However, although BMD score is a key criterion they also recommend treatment is considered without the need for a BMD score for people have a previous hip or vertebral fracture, people on

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					very-high dose steroids (around 15mg per day), where the score is not needed for treatment decisions (e.g. elderly people living with frailty who have a few key risk factors) and where doing the DXA is not tolerated or feasible. It is important to note that our recommendations state the conditions under which the committee believe treatment should be considered but do not assert that people with BMD levels outside these ranges should not receive treatment.
Northumbria NHSFCT	Guideline	9	13	VFA for all patients over 50 will impact on national DEXA scanning capacity	Thank you for participating in the consultation process. The committee discussed this and did not agree that DXA capacity would be reduced because it was considered feasible to accommodate the additional time within the existing DXA slot in most cases. This was considered achievable because the time to do the actual scan is a relatively small proportion of the overall time slot which includes time to meet and prepare the attendee. They agreed that there may be issues with reporting capacity due to additional time requirements and this would need to be considered when implementing the recommendation. To mitigate the impact the committee revised their recommendation from all people having DXA over 50 to men aged 70 and over and women aged 60 and over, with use under these ages limited to those with additional risk factors.
Northumbria NHSFCT	Guideline	10	14	Sorry, don't understand why shouldn't use spinal imaging to confirm VFA suggestion of fracture. Depending on age of	Thank you for participating in the consultation process. A recommendation has been amended to emphasise that spinal

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				scanner, VFA images can lack detail (ours do); whilst you can use this image to rule out fractures, more detailed imagery is needed to rule them in. The committee acknowledged this (p23 line 29) but the screening recommendation shouldn't be based on the assumption everyone has the most up to date equipment (chance would be a fine thing)	imaging should not be routine practice in the presence of a VFA positive result unless there are uncertainties about the fracture or it is needed for differential diagnosis. This is because several studies suggested that doing a VFA when doing a DXA scan had a low risk of misdiagnosing vertebral fractures with high specificity reported in the per-vertebral analysis. Therefore, the committee agreed that if DXA-based VFA positively identifies a person as having a fracture, it is highly likely to be accurate, and further imaging is not needed.
Northumbria NHSFCT	Guideline	17	20	Whilst evidence for trans and nonbinary people is limited, they may well be taking additional hormonal treatment and DEXA scan can help clarify their unique circumstances with regards to their bone health and form the basis for a specialist treatment plan. As a general rule, tests are most useful when the result isn't known in advance – this is such a situation. I take a sympathetic approach to DEXA requests for these patients because the results can be unusual. DEXA needs to be reported in g/cm2 by scanner brand, and normalised to the female T score table for assessing fracture risk in all trans and nonbinary patients to ensure equality of approach.	Thank you for providing this information. We have added a recommendation for trans or non-binary people that takes into account hormonal history and current hormone treatment when assessing fracture risk.
Parkinson's UK	Guideline	General	General	In the section for risk factors for fragility fractures we note that neurological conditions including parkinsonism are not considered. However, compared to age and gender matched controls people with Parkinson's are at 2-fold increased risk for osteoporotic fracture, and 3-fold increased risk for hip fractures (Pouwels et al. 2013, Schini et al. 2020). Therefore	Thank you for participating in the consultation process. The list of risk factors has been updated to include Parkinson's disease and parkinsonism under the neurological category. Please note that the list of risk factors is not intended to be exhaustive, and the guideline has not included every factor that may be associated with increased fracture risk.

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				a diagnosis of parkinsonism should prompt an assessment of fracture risk (NOGG Guidelines 2024, Henderson et al. 2019, Naylor et al. 2025).	
Parkinson's UK	Guideline	General	General	In the section assessing for fragility fracture risk it would be beneficial to include further caveats and adjustments for the interpretation of FRAX scores when assessing people with parkinsonism. In the development of the BONE PARK 2 algorithm we recommended that parkinsonism is included using rheumatoid arthritis as a proxy, and recurrent falls are included using a mathematical adjustment of 30% to better capture the risk of fracture in this high risk population (Naylor et al. 2025)	Thank you for participating in the consultation process. Adjusting risk scores was not included as an area to look at in the guideline and therefore hasn't been included in the recommendations. The committee noted that it is important to include clinical judgment in decision making. Parkinson's disease and parkinsonism have been included in the list of risk factors.
Pregnancy associated osteoporosis UK (PAO UK)	EIA	02	12-17	It is stated that pregnancy associated osteoporosis (also known as Pregnancy Lactation Associated Osteoporosis) is an 'ultra-rare condition', hence it has not been considered within this draft guideline. Although rare, recent data suggest that the prevalence of pregnancy associated osteoporosis may be greater than previously thought (ref Orhadje et. al. Calcified Tissue International 113; 591-596), and it is very likely underrecognised due to lack of awareness among clinicians. Therefore, this guideline could provide a valuable opportunity to raise awareness of PAO, particularly in women who have had vertebral fractures in the context of current / recent pregnancy. We would simply suggest making it explicit in this NICE guidance that 'if individuals report fragility fractures during pregnancy or breastfeeding, then advise immediate referral to specialist'.	Thank you for participating in the consultation process. The committee agree that it is important to highlight pregnancy associated osteoporosis in the guideline. It has been included as an example of rare bone disease associated with fragility fractures that require specialist advice in recommendations for risk assessment and treatment options.

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Pregnancy associated osteoporosis UK (PAO UK)	Guideline	07	10-13	It is recommended that FRAX or QFracture be used to assess fracture risk in individuals aged 40-49, including when deciding whether or not to refer for a DXA scan. These risk assessment tools do not distinguish between vertebral / hip fracture and other types of fracture when assessing fracture risk, therefore may underestimate risk in individuals who have had a vertebral / hip fracture, or multiple fractures. It is our view that any adult with an unexplained low trauma vertebral or hip fracture should be referred for a DXA scan (as these fractures are uncommon in this age group, this should not have a significant impact on resource use).	Thank you for participating in the consultation process. The recommendations have been updated so anyone aged over 30 with a previous hip or vertebral fracture is referred to DXA. For people under 30 or with a suspected rare bone disease associated with fragility fractures, (such as osteogenesis imperfecta or pregnancy-associated osteoporosis) seeking specialist advice is recommended.
Pregnancy associated osteoporosis UK (PAO UK)	Guideline	07	14-15	It is recommended that QFracture be used to assess fracture risk in individuals aged 30-39, including when deciding whether or not to refer for a DXA scan. QFracture does not distinguish between vertebral / hip fracture and other types of fracture when assessing fracture risk, therefore may underestimate risk in individuals who have had a vertebral / hip fracture, or multiple fractures. It is our view that any adult with an unexplained low trauma vertebral or hip fracture should be referred for a DXA scan (as these fractures are uncommon in this age group, this should not have a significant impact on resource use). Furthermore QFracture also does not allow incorporation of BMD when assessing fracture risk, and (unlike FRAX) does not provide an intervention threshold for treatment.	Thank you for participating in the consultation process. The recommendations have been updated so anyone aged over 30 with a previous hip or vertebral fracture is referred to DXA. For people under 30 or with a suspected rare bone disease associated with fragility fractures, (such as osteogenesis imperfecta or pregnancy-associated osteoporosis) seeking specialist advice is recommended.
Pregnancy associated	Guideline	12	1-6	We agree that specialist assessment is required for premenopausal women and younger men who experience fragility fracture, however we are concerned that waiting	Thank you for participating in the consultation process. The committee acknowledge that waiting times can be a problem and this has been noted in the equality impact assessment.

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osteoporosis UK (PAO UK)				times for secondary care osteoporosis clinics can be long, and are variable across the country.	However, they have made the recommendations based on what they think is best practice. In these circumstances they agreed it is important to seek specialist advice because it is important to identify the underlying cause when a younger person has a major osteoporotic fracture, and rare bone diseases need specialist assessment and treatment.
Royal College of Nursing	General	General	General	Thank you for inviting the Royal College of Nursing to review and comment on the draft guideline - osteoporosis: risk assessment, treatment, and fragility fracture prevention (update) The draft guideline was shared with our members who work in this area of health for their review. The comments below reflect the view of our reviewer.	Thank you for participating in the consultation process. We have responded to each comment in turn.
Royal College of Nursing	General	General	General	The draft guideline looks comprehensive and provides sensible advice. There are no further comments to add at this stage.	Thank you for participating in the consultation process.
Royal Osteoporosis Society	Guideline	General	General	Overall comment on the draft guideline. The Royal Osteoporosis Society welcomes this draft update and supports NICE's focus on earlier identification of people at high risk of fragility fractures. Improving identification, particularly of vertebral fractures, has the potential to reduce avoidable fractures, long-term disability and substantial healthcare and societal costs if it leads to timely intervention. Independent analysis commissioned by the Royal Osteoporosis Society, undertaken by Lane Clark & Peacock,	Thank you for participating in the consultation process and your support for the guideline.

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				has highlighted the significant economic impact of fragility fractures across the NHS, social care and the wider economy, particularly when fractures are missed or untreated. ¹ ¹ <i>The Economic Impact of Fragility Fractures in the UK, Royal Osteoporosis Society and Lane Clark & Peacock.</i>	
Royal Osteoporosis Society	Guideline	General	General	Implementation considerations across the guideline. The cost-effectiveness of improved risk assessment and VFA depends on services being in place to act on the findings. Alignment with the Government's commitment to deliver high-quality Fracture Liaison Services across England by 2030 is essential to ensure that diagnostic investment translates into reduced fracture rates, avoided hospital admissions and long-term savings across the health and care system, as well as reduced productivity losses.	Thank you for participating in the consultation process. The committee agree that adequate service provision is important for the delivery of appropriate care.

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Royal Osteoporosis Society	Guideline	General	General	Confusion in primary care directly harms patients - Primary care teams are the frontline for identifying and supporting people at risk of osteoporosis. Yet evidence shows many primary care professionals already feel uncertain about how to follow existing osteoporosis guidance. In a UK survey of 341 primary care clinicians, confusing guidelines were identified as a major barrier to delivering evidence-based osteoporosis care, with many specifically requesting simple, workable flowcharts. (Hawarden A, Bullock L, Cox NM, Nicholls E, Protheroe J, Jinks C, Paskins Z. Osteoporosis care in primary care settings: a national UK e-survey. Archives of Osteoporosis. 2025;20(1):109. doi:10.1007/s11657-025-01591-8.) The current draft guideline is difficult to operationalise, and we fear this will widen the existing care gap. Confusion in primary care means patients may miss out on important assessments or face delays in diagnosis and treatment.	Thank you for participating in the consultation process. The recommendations have been updated to make them clear and a visual summary produced to go alongside them. Following publication of the guideline the technology appraisals and clinical knowledge summary will also be updated to ensure that there is a consistent set of recommendations. Your comments will also be considered by the NICE implementation team where relevant support activity is being planned.
Royal Osteoporosis Society	Guideline	General	General	Terminology matters — and current wording risks stigma and missed diagnoses - The guideline uses both “fragility fracture” and “major osteoporotic fracture.” Patients tell us consistently that the word “fragility” is upsetting and stigmatising; it can imply weakness or frailty and may discourage people from seeking help. Research has confirmed these negative associations.	Thank you for participating in the consultation process. The committee recognise that many people with fragility fracture do not like the term ‘fragility fracture’. The term is widely accepted and understood in the literature. Its use rather than ‘osteoporotic fracture’ also accommodates the fact that most fragility fractures occur in people who do not have osteoporosis (as currently defined, a BMD less than -2.5). It is also worth

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				Alongside the emotional impact for patients, primary care clinicians also report uncertainty about whether fractures should be labelled as “fragility fractures,” partly because this terminology is rarely used in hospital discharge letters. This ambiguity leads to missed opportunities for osteoporosis assessment and treatment. We strongly recommend using the clearer, more consistent, and less stigmatising term “osteoporotic fracture,” aligned with international consensus recommendations. To further support accurate diagnosis, we suggest explicitly stating that in adults aged 50+, fractures of the wrist, humerus, pelvis, hip, or spine can be coded as osteoporotic when the trauma history is unclear. This would substantially reduce under-diagnosis, which is a major patient safety issue. In addition ‘experienced menopause’ may lead to misunderstanding that symptoms are necessary – please amend to post-menopausal.	noting that this section of the guideline is about identifying fragility fractures and not all will be osteoporotic. The term is still used across other NICE guidance and external organisations and guidelines. Should there be a different and universally accepted term agreed then NICE will consider updating its recommendations across its guidance. The term women who have experienced menopause is an agreed NICE style terminology. It also aligns with the NICE guideline on menopause.
Royal Osteoporosis Society	Guideline	General	General	Changes to thresholds risk confusion and inconsistent patient care - The draft guideline moves away from the previous NOGG-based thresholds for DXA and treatment. While we acknowledge the challenges in defining thresholds, introducing a new approach without clear evidence that it	Thank you for participating in the consultation process. Please note that the previous NICE guideline (CG146) did not recommend use of NOGG thresholds for DXA and treatment. We acknowledge that some other NICE products have referred to them when defining high risk however these are also being updated in due course so that there is a consistent NICE

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				improves outcomes risks confusion in primary care and could exacerbate inequalities in access. Current NIHR-funded research (PROTECTS) will provide the first rigorous economic comparison of threshold strategies. Implementing new thresholds now, only to revise them again when evidence emerges, will create unnecessary disruption for both clinicians and patients. Stability and clarity are essential for patient-centred care.	pathway. The committee consider the recommendations to be justified clinically and that they are likely to be associated with lower DXA resource use than if using NOGG criteria. The basis for the recommendations are described in the Committee discussion section in Evidence report E. The committee welcomes additional research in this area but, given that the PROTECTS study has just started and it is a 6-year work program, did not consider it appropriate to delay updating the guideline. The next update of the NICE guideline will benefit from new research in this area. This will be monitored in line with the priority board manual (NICE-wide topic prioritisation manual).
Royal Osteoporosis Society	Guideline	General	General	Use of the word “consider” risks widening inequalities - Several key risk factors are linked with significantly increased fracture risk — for example, low BMI, rheumatoid arthritis, coeliac disease, and hyperparathyroidism. Using the instruction “consider” instead of “assess” risks leaving assessments to chance, especially in time-pressured primary care settings. Vague wording may also lead to inconsistent practice across regions, widening existing inequalities. We urge NICE to use stronger, clearer terminology for these well-established risk factors. Additionally, several important high-risk groups are missing, including people with Parkinson’s disease, dementia, and systemic lupus erythematosus (SLE). SLE carries a fracture	Thank you for participating in the consultation process. The committee agreed to not update the risk factor evidence review and use the evidence from the previous guideline. They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors. Consequently, following the NICE process for wording recommendations, the term ‘consider’ was used in the previous guidance, reflects the confidence in the available evidence, and is used again in this guideline.

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				risk comparable to rheumatoid arthritis and deserves explicit inclusion.	Parkinson's disease and SLE have been added to the list of examples as they were referred to in the original NICE guideline. However, we did not include dementia as we have not reviewed the evidence for this risk factor and are unable to add new suggested examples.
Royal Osteoporosis Society	Guideline	General	General	Age-based restrictions raise fairness concerns - Restricting access to DXA scanning based primarily on age risks disadvantaging older adults — the very population most likely to sustain osteoporotic fractures. This approach feels inequitable and may result in significant numbers of high-risk patients being under-assessed. We welcome clearer guidance on assessing younger adults but urge NICE to ensure that older adults are not excluded from essential diagnostic pathways.	Thank you for participating in the consultation process. The recommendations have been amended to make clear that age is not the determining factor in deciding whether to obtain DXA measurement of BMD but rather: 1. the presence of a previous hip or vertebral fragility fracture, 2. 2 or more fragility fractures, or 3. a 10-year MOF risk of 10% or more as assessed by FRAX or QFracture. This applies now to everyone over 30. Specialist advice is recommended for anyone under 30. The recommendations relating to risk assessment tools have also been updated to make it clear there are limitations when using this in people over 90.
Royal Osteoporosis Society	Guideline	General	General	DXA and fracture risk assessment recommendations need clearer structure Statements about fracture risk and DXA are currently intertwined. The guideline notes that people with hip and vertebral fractures should be offered DXA without requiring prior fracture risk assessment. While this is clinically appropriate, the recommendation's placement is unclear and risks being overlooked. A clearer separation — or a statement explicitly saying “without the need for a fracture risk assessment” — would support consistent practice.	Thank you for participating in the consultation process. The structure of the recommendations has been amended to make it clearer. The recommendation has been updated to include ‘with or without completing a risk prediction tool’ to clarify this point. A visual summary accompanies the recommendations. A statement has also been added to the rationale indicating that risk may still be calculated with risk prediction tools for this group if it is useful for clinical decision making or communicating risk to the person.

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Royal Osteoporosis Society	Guideline	General	General	Inconsistencies in T-score thresholds risk further confusion - Multiple T-score thresholds are used in different parts of the guideline (e.g., -1.5 in one place, -1.0 in another) without clear explanation. T-scores can also be unreliable for people with osteoarthritis or previous surgeries. These limitations should be clearly stated so that BMD is not used in isolation to determine treatment, potentially disadvantaging patients with difficult-to-interpret scans.	Thank you for participating in the consultation process. Recommendation 1.7.1 makes it clear that several factors need to be taken into account when making a decision to treat. The recommendation about making a decision to treat without a DXA has been moved down to the treatment criteria section to make it clearer the decision can be made without a BMD t score in some circumstances. The recommendations have been updated removing the BMD t score of <-1.0 and making the criteria clearer. A visual summary is provided to accompany the recommendations.
Royal Osteoporosis Society	Guideline	General	General	The ongoing disparity between NICE and NOGG guidance risks sowing unnecessary confusion and could contribute to expanding the already-large treatment gap for osteoporosis. Complexity or restrictive thresholds risks leaving more people untreated, clarity and consistency across national guidance is essential to reduce postcode variation. Any approach which unintentionally excludes high-risk patients is concerning.	Thank you for participating in the consultation process. We recognise that differences in clinical guidelines can confuse patients and clinicians alike but do not ultimately agree with the FRAX-NOGG system for reasons discussed in the guideline. The committee do not think that high risk patients will be excluded from assessment and treatment.
Royal Osteoporosis Society	Guideline	General	General	We would welcome further clarification on the rationale for applying a fixed 10% 10-year probability of major osteoporotic fracture as a cut-off for assessment or treatment consideration. A single threshold may not fully reflect the clinical significance of different fracture types. Hip fractures, for example, can occur at lower percentage risks but are associated with substantial morbidity, mortality and loss of	Thank you for participating in the consultation process. The committee agree that fracture site should be taken into consideration. People with hip or vertebral fragility fractures or multiple NHNV fractures do not need to meet the 10% risk threshold to be eligible for DXA and consideration for treatment (see recommendation 1.4.1 below).

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				independence. In addition, younger individuals with specific risk profiles may fall below a 10% major fracture threshold while still exceeding appropriate treatment thresholds when hip fracture risk is considered. We would encourage NICE to ensure that any fixed probability cut-off does not inadvertently exclude people at meaningful risk of serious fracture outcomes.	People with a previous hip or vertebral fragility fracture should be considered for pharmacological treatment without meeting a T-score threshold (see recommendation 1.7.3 below). Recommendation 1.4.1. Offer a DXA scan to measure BMD (with or without completing a risk prediction tool) when assessing fragility fracture risk in people aged 30 and over who have had either a previous hip or vertebral fragility fracture or 2 more fragility fractures. Recommendation 1.7.3 consider pharmacological treatment for men age 50 and over and women who have experienced menopause if they have any of the following: a previous hip or vertebral fragility fracture (other criteria not listed here).
Royal Osteoporosis Society	Evidence review H: Fracture risk monitoring in non-treatment groups	12-14	General	Sections 1.8–1.10: Follow-up for people not having treatment. We are concerned that people who do not initially meet treatment thresholds can be left without clear follow-up or safety-netting. Lack of planned reassessment risks people re-presenting only after a major fracture, which carries far higher acute, rehabilitation and long-term care costs than planned monitoring and early intervention.	Thank you for participating in the consultation process. The committee agree that it is important not to leave people who do not initially meet treatment thresholds without clear follow-up. Therefore, the guideline recommends to consider reassessing (by risk tool or DXA) at 5 years or sooner if there is a change in circumstances.
Royal Osteoporosis Society	Guideline	4-6	General	Sections 1.1 and 1.2: Identifying adults for fragility fracture risk assessment and use of electronic health records. We welcome NICE's recognition of systematic approaches to risk identification, including use of electronic health records. Proactive case-finding has the potential to be more cost-effective than reliance on opportunistic assessment by	Thank you for participating in the consultation process and for your supportive comment.

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				identifying high-risk individuals earlier and reducing avoidable fractures, emergency admissions and long-term care costs, which represent a significant proportion of the overall economic burden of osteoporosis-related fractures.	
Royal Osteoporosis Society	Guideline	9-10	General	Section 1.5: DXA-based vertebral fracture assessment (VFA). We strongly welcome the recommendation to consider adding vertebral fracture assessment (VFA) to routine DXA scanning in appropriate groups. Identifying vertebral fractures at the same appointment can reduce missed diagnoses and downstream costs associated with unmanaged pain, repeat GP consultations and subsequent fractures. Evidence on the wider economic impact of fragility fractures underlines the value of identifying vertebral fractures before further, more costly fractures occur.	Thank you for participating in the consultation process and for your supportive comment.
Royal Osteoporosis Society	Guideline	9-10	General	Section 1.5: DXA-based vertebral fracture assessment (VFA). The guideline would benefit from clearer signalling that identification of a vertebral fracture through VFA should trigger treatment consideration and follow-up. Without this link, the system risks incurring the costs of additional diagnostics without achieving downstream savings from fracture prevention, a concern highlighted in economic analyses of fragility fracture care pathways.	Thank you for participating in the consultation process. Recommendation 1.7.3 advises that treatment should be considered if a vertebral fracture is identified. A visual summary accompanies the recommendations to make them clearer to follow.

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Royal Osteoporosis Society	Guideline	11-12	General	Section 1.7: Criteria for treatment. Clear treatment thresholds are important, but patients frequently report being diagnosed without clear explanation or action. From an economic perspective, delayed or absent treatment following diagnosis increases the likelihood of further fractures, with substantially higher costs to the NHS, social care and employers than timely preventative treatment, as reflected in published estimates of the economic impact of fragility fractures.	Thank you for participating in the consultation process. The committee agree that appropriate and timely treatment is important in preventing further fractures and that this can be affected by inadequate communication. The committee think that, as a matter of good clinical practice, healthcare professionals should take the time to explain to a patient why they are recommending a course of action (such as starting anti-osteoporotic medication) and to answer any questions the person might have. Recommendations on effective communication and information about a person's diagnosis are covered in NICE's generic guidance on Patient experience in adult NHS services which is linked to from this guideline.
Royal Osteoporosis Society	Guideline	5	General	Diabetes is listed as a risk factor for osteoporosis; it would be helpful to note whether this is type 1, type 2 or both. COPD is also listed as a risk factor and is a very broad category that risks covering a lot of people unnecessarily; is it worth including something about severity and whether this overlaps with smoking and/or steroid use?	Thank you for participating in the consultation process. We have amended the table to make clear that both types of diabetes are associated with increased fracture risk. COPD was included as an example in the previous version of the guideline (CG146), so we have not changed it. However, we have added a reference to the NICE guideline on COPD for further information. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive.

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Society and College of Radiographers	Guideline	General	General	These recommendations will be difficult in clinical practice because there is a lot of responsibility on the DXA operator who may not be registered professionals with appropriate high-quality training to undertake extended scope of these proposals. Many DXA services are staffed with band 4 and band 5 roles and due to increase in responsibility career progression in DXA services should be considered.	Thank you for participating in the consultation process. The committee agreed that this shouldn't change current practice for DXA operators other than there may be a change in the number of people needing DXA scans. Appropriate training and career progression for DXA services were not part of the scope of this guideline.
Society and College of Radiographers	Guideline	General	General	Would be of use if the guidelines mentioned the importance of a requirement of a full clinical report rather than a technical report. Furthermore, who's responsibility it is to disseminate all this information and risk factors from these guidelines. Indeed, DXA reporting guidance incorporates the results from the INDEX study recommending reporting DXA with BMD and fracture risk as one diagnostic statement.	Thank you for participating in the consultation process. DXA reporting was not part of the scope of this guideline.
Society and College of Radiographers	Guideline	General	General	Consideration over recommending a clinical questionnaire to be undertaken by DXA operators to be able to obtain all the information required in these guidelines – clinical referrals don't always provide sufficient information.	Thank you for participating in the consultation process. Questionnaires for DXA operators were not part of the scope of the guideline.
Society and College of Radiographers	Guideline	General	General	These recommendations will be difficult in clinical practice without teaching and education to support these guidelines. No course/module currently available in the country to provide the education for the operational, analytic and reporting to provide sufficient training for VFA.	Thank you for participating in the consultation process. Reporting of results was not part of the scope of the guideline. Your comment will be considered by the NICE implementation team where relevant support activity is being planned.

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				If there is no reporting recommendation on the resultant image – what is the point of doing the scan – it will not be justified under IR(ME)R - A formal clinical evaluation and radiological report would be required to be IR(ME)R compliant.	
Society and College of Radiographers	Guideline	General	General	These recommendations will be difficult in clinical practice as the current scope of practice for some DXA operators does not cover what is required within these guidelines and therefore education would be required.	Thank you for participating in the consultation process. Your comment will be considered by the NICE implementation team where relevant support activity is being planned.
Society and College of Radiographers	Guideline	General	General	Considering the education this will have a short-term cost implication to ensure the training is in place.	Thank you for participating in the consultation process. The potential for short term training costs has been noted in the Committee discussion in Evidence report G and the Impact section in the Short Guideline. Your comment will be considered by the NICE implementation team where relevant support activity is being planned.
Society and College of Radiographers	Guideline	General	General	NET costing review: VFA is extremely beneficial for patients as documented, but consideration must be had on the costs of running the service and training costs. Even if VFA took a few minutes extra to scan and report (see comment 23) this would work out at potential of loss of capacity for at least one patient a day. When considering the more realistic time frames considered in comment 23 it could be around 1.5hours additional per day given almost everyone will require a VFA within these guidelines.	Thank you for participating in the consultation process and support for the recommendation. The committee discussed this and did not agree that DXA capacity would be reduced because it was considered feasible to accommodate the additional time within the existing DXA slot in most cases. This was considered achievable because the time to do the actual

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				Overall, the long-term goal will show cost improvement due to preventing fractures/hip fracture admissions in the short-term funding will be required.	scan is a relatively small proportion of the overall time slot which includes time to meet and prepare the attendee They agreed that there may be issues with reporting capacity due to additional time requirements and this would need to be considered when implementing the recommendation. To mitigate the impact the committee revised their recommendation from all people having DXA over 50 to men aged 70 and over and women aged 60 and over, with use under these ages limited to those with additional risk factors
Society and College of Radiographers	Guideline	General	General	There is evidence that there is: • Lack of workforce within DXA nationally. • National struggle to keep up with scanning demand and capacity • Long delays for patients getting a DXA scan • Long delays in getting the scan reported. The recommendations do not consider the impact on services both practically and from a financial aspect.	Thank you for participating in the consultation process. The committee noted that some areas have capacity issues with DXA and so there can be long waiting lists. NHS England data from February 2026 reported a median waiting time for DXA of around 2.6 weeks with 12% of people on the waiting list having been so for 6 weeks or more and 3% for 13 weeks or more. They noted that NHS England committed to funding 13 new scanners in 2025 and another 20 new scanners in 2026 – this included a mix of replacement machines and additional machines to increase capacity. The committee considered whether recommendations were a change in practice and the potential impact on NHS resource use as part of the guideline development process alongside any evidence of clinical and cost-effectiveness. Their considerations are described in the 'Committee discussion of

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					the evidence' in the relevant Evidence Reports and the Impact sections of the Short Guideline.
Society and College of Radiographers	Guideline	General	General	The document suggests there are lots of fracture liaison services across the country – the truth is there are still a lot of trusts that have not implemented a FLS and ROS reports that around 50% of the UK population is NOT covered by FLS, Trusts without FLS do not have the staffing model, funding or additional capacity to accommodate an FLS.	Thank you for participating in the consultation process. The provision of Fracture Liaison Services is beyond the remit of this guideline. It is noted that the government has committed to 100% coverage by 2030 in the NHS 10-year plan.
Society and College of Radiographers	Overall review	General	General	The guidance appears to be a step back from fracture risk-based management to DXA T-score based management. Reinforcing T score as the gatekeeper to treatment except for hip and spine fractures. The bone health community has been moving closer to reducing fracture risk and fracture risk-based management.	Thank you for participating in the consultation process. The committee's decision is based on following the pathway of risk assessment through to the decision when to offer treatment. The committee think that the use of an age-based threshold in FRAX-NOGG to determine eligibility for pharmacological treatment leads to too few older people being treated and too many young people being treated because there is unclear long-term benefit for these treatments for a relatively small reduction in absolute risk. Although it is true that the relative risk of fragility fracture decreases as age increases, this is not the case for absolute risk, which increases with age. Older people therefore stand to benefit more from treatment than younger people. It is important to note that the recommendations state the conditions under which the committee think treatment should be considered but do not assert that people with BMD levels outside these ranges should not receive treatment.

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					In contrast, the NICE approach is based on the premise that treatment is best targeted to people at high enough levels of risk who are likely to experience an absolute risk reduction in fracture that outweighs possible adverse events. It also incorporates BMD t score criteria in addition of absolute fracture risk as the committee agree this is important to ensure treatment is targeted at those most likely to achieve a risk reduction. However, although BMD score is a key criterion they also recommend treatment is considered without the need for a BMD score for people have a previous hip or vertebral fracture, people on very-high dose steroids (around 15mg per day), where the score is not needed for treatment decisions (e.g. elderly people living with frailty who have a few key risk factors, and where doing the DXA is not tolerated or feasible).
Society and College of Radiographers	Guideline	04 06 11	005 002 009	Rec 1.1.1 – States previous fragility fracture – however feel this needs defining further – time frame of age of fracture: fracture risk decreases with age of fracture (not considered over 5 years ago) In addition, there is evidence that fractures sustained within a 2-year period are an indicator for certain treatments (vertebral fractures).	Thank you for participating in the consultation process. For recommendation 1.1.1 the committee agreed that any previous fragility fracture should be assessed. However, the committee recommend recency of fracture is an important factor to take into account when deciding on treatment and this has been included in recommendation 1.7.1
Society and College of Radiographers	Guideline	04 06	015 004	There is evidence that pelvic fracture including public rami should be treated the same as hip fractures – all are a sign of fragility and included on the national hip fracture database	Thank you for participating in the consultation process. Evidence was not identified to single out pelvic fractures for these recommendations. The evidence related to family history only relates to hip fracture. The guideline recommends any

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					fragility fracture, including pelvic, is considered as part of risk assessment.
Society and College of Radiographers	Guideline	05	001	We agree with the table listed and would like the addition of contraceptive Depo Provera under Other 5 th bullet point - medicines	Thank you for participating in the consultation process. The committee agreed to not update the risk factor evidence review and use the evidence from the previous version of the guideline (CG146). They noted we were unlikely to find strong evidence to change the strength of the recommendation, and the list of risk factors is not intended to be exhaustive. The recommendations have been updated to make it clearer that this is a list of examples of risk factors. The contraceptive Depo Provera has not been added as they were not included previously and we have not reviewed the evidence for this as a risk factor.
Society and College of Radiographers	Guideline	07	019	We agree with the examples given but would like other hormonal medications to be added.	Thank you for participating in the consultation process. The text related to the reason for doing a full risk assessment has been moved to the rationale for the recommendation. The committee agreed that only 2 key examples need to be included and did not want to give the impression the list is exhaustive. It is anticipated the healthcare professional doing the assessment will use their clinical judgement.
Society and College of Radiographers	Guideline	07	024	Needs defining - Patches or oral HRT	Thank you for participating in the consultation process. The recommendation has been amended to specify that fracture risk is decreased during systemic HRT.
Society and College of Radiographers	Guideline	07	025	We agree with the use of HRT and would like other bone sparing medicines to be added	Thank you for participating in the consultation process. The recommendation concerns HRT because this can be included in QFracture. Whilst the committee agrees that other bone sparing medicines are likely to reduce fracture risk, they are not

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					included in FRAX and QFracture and so no recommendation about them was made.
Society and College of Radiographers	Guideline	07	026	Poor wording needs clarification. Benefit decreases once treatment stops	Thank you for participating in the consultation process. The recommendation has been updated removing the bullet points and simplified to "When assessing risk, take into account that fragility fracture risk is decreased while taking systemic hormone replacement therapy (HRT)."
Society and College of Radiographers	Guideline	08	007	If considering measuring BMD for patients under 30s – clinical indications will be required to be justifiable under IR(ME)R. What action will be done with the results. Cross reference to other guidelines coeliac and eating disorders as done in table.	Thank you for participating in the consultation process. The recommendation has been amended to remove reference to DXA and specifies that specialist advice should be sought.
Society and College of Radiographers	Guideline	08	012	NOGG guideline should be considered regarding 10%. NOGG management advice is evidence based and this guideline appears to be moving away from this. Currently 10% risks excluding younger older adults with elevated fracture risk from being investigated.	The committee did not use the FRAX-NOGG thresholds to determine eligibility for pharmacological treatment for the following reasons: 1. It leads to the under treatment of older people who would benefit most from treatment options. Relative risk reduction from pharmacological treatment is similar at all ages. However, absolute risk increases with age, and older people will benefit more from treatment than younger people. The committee do not think older people should have to reach a higher level of absolute risk to warrant treatment. 2. It can lead to over treatment of young people, or lots of young people being assessed but not treated as they are deemed not to be at a high enough risk to warrant treatment.

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					This in turn can result in an assessment ‘treadmill’ which also has an associated resource cost, impacting on DXA services and potentially causing patient anxiety because of this cycle. 3. The committee agree that in principle it might seem attractive to start treatment early with the aim of preventing further deterioration in bone health. However, they also agreed there is a lack of evidence of long-term treatments. Unlike other conditions which have treatments that are safe, effective and work long-term there is no ‘forever treatment’ in osteoporosis. Osteoporosis treatments are recommended for 5 (up to 10) years before a treatment break is required. These treatments are also known to be associated with the occurrence of rare but severe adverse events which are related to dose/duration of treatment and some are only recommended for use once in a lifetime. This would mean treatment options later in life when a person is more likely to be at a high level of risk would be limited. In contrast, the NICE approach is based on the premise that treatment is best targeted to people at high enough levels of risk who are likely to experience an absolute risk reduction in fracture that outweighs possible adverse events. It also incorporates BMD t-score criteria in addition to absolute fracture risk as the committee agree this is important to ensure treatment is targeted at those most likely to achieve a risk reduction. The committee noted that the majority of RCT evidence is in people with BMD T scores below -1.5. However,

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					although BMD score is a key criterion they also recommend treatment is considered without the need for a BMD score for people have a previous hip or vertebral fracture, people on very-high dose steroids (around 15mg per day), where the score is not needed for treatment decisions (e.g. elderly people living with frailty who have a few key risk factors) and where doing the DXA is not tolerated or feasible. It is important to note that our recommendations state the conditions under which the committee believe treatment should be considered but do not assert that people with BMD levels outside these ranges should not receive treatment. The recommendation to offer a DXA scan with or without completing a risk prediction tool has been amended to include people aged 30 and over who have a single major osteoporotic fragility fracture in the last 2 years. This will increase number of younger people with elevated fracture risk being investigated.
Society and College of Radiographers	Guideline	08	018	“Fast-tracking” what does this mean i.e. 6 weeks? – time scales required – further timescales for scan and report. Would these require “hot reporting”? Scan can often be undertaken quickly but report can take a while – needs linking to access to clinician that can prescribe. Radiography prescribing would assist with this. Note made to DXA radiology department to help facilitate the Fastrack and service provision. Could potentially impact achieving NHSE DM01 diagnostic waiting times of 6 weeks.	Thank you for participating in the consultation process. Fast-tracking means within 6 weeks and this has been added to the recommendation. The committee made this recommendation as they agreed it is the best recommendation for practice. Evidence was not available to make a stronger recommendation and ‘consider’ reflects NICE process on the wording of recommendations. The committee agreed to make the recommendations based on best practice and did not want to make a different

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					recommendation purely because of capacity issues. Also, they note that the government announced 13 extra DXA scanners in 2025 , and a further 20 scanners have been promised in March 2026 to help cut waiting times.
Society and College of Radiographers	Guideline	09	013	“Consider” poor wording, needs defining. Rec 1.5.1 – We are concerned these recommendations suggest VFA to be used as a screening tool of such which is not justified under IR(ME)R or capacity to undertake NOGG and ISCD have a set of criteria for patients over 50 years of age supporting the identification of people who may have elevated risk of vertebral fractures and who might benefit from fracture identification. [This text was identified as confidential and has been removed], Would be difficult to implement given capacity, demand, training needs of operators and reporters as well as reporting capacity. IR(ME)R justification is	Thank you for participating in the consultation process. In NICE guidelines, the wording of the recommendation reflects the strength, with ‘consider’ used to reflect a weaker recommendation. The committee note that the exposure to radiation from VFA is small and did not consider justification under IR(ME)R to be an issue because the benefit of identifying people at increased risk of fracture outweighs the risk from radiation exposure. To mitigate the impact the committee revised their recommendation from all people having DXA over 50 to men aged 70 and over and women aged 60 and over, with use under these ages limited to those with additional risk factors. These groups were selected due to their increased risk of vertebral fracture. The committee acknowledge that implementation would require consideration of appropriate training and expanding reporting capacity.
Society and College of Radiographers	Guideline	09	019	Back pain and radiating rib pain – requires more definition – many people over 40 complain of LBP – link with guideline (vfrac describes this) – need to assess if justified under IR(ME)R	Thank you for participating in the consultation process. The recommendation has been amended to read ‘history of back pain or radiating rib pain with no identified cause’.

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Society and College of Radiographers	Guideline	10	001	Physical height loss – how much, height needs defining.	Thank you for participating in the consultation process. The committee have added the International Society for Clinical Densitometry (ISCD) criteria of a height loss of more than 4 cm.
Society and College of Radiographers	Guideline	10	004	Rec 1.5.2 – We are concerned these recommendations have not thought about training capacity and demand issues– document suggests that the DXA operator can identify a VF from the DXA scan – further training would be required for: Operators: - To be skilled to understand signs and symptoms in patients who may benefit from VFA - To be skilled at identifying features on the DXA scan which might be suspicious for fracture - To be able to carry out the VFA accurately Reporters: - To be skilled at analysing vertebral morphometry from VFA - To be experienced and skilled to report vertebral fractures Further adding time to the analysis and report as well as and increasing the scanning time of a patient.	Thank you for participating in the consultation process. The committee considered the additional resource implications of the VFA recommendations and these are described in the Committee discussion of the evidence in Evidence report G and summarised in the Impact section of the Short Guideline. The committee acknowledge that implementation would require consideration of appropriate training and expanding reporting capacity. To mitigate the impact the committee revised their recommendation from all people having DXA over 50 to men aged 70 and over and women aged 60 and over, with use under these ages limited to those with additional risk factors. These groups were selected due to their increased risk of vertebral fracture.
Society and College of Radiographers	Guideline	10 24	014 010/011	There is evidence that VFA cannot differentiate between grade 1 fractures and non-fracture deformities such as normal variants / congenital anomalies. Grade 1 fragility fracture are still clinically important so need identifying and therefore require radiograph to confirm. Wording change: give guidance on when radiograph should be considered. Also, guidance on who should be referring for the radiograph –	Thank you for participating in the consultation process. The recommendation has been updated to Do not routinely do spinal imaging after VFA has identified a fracture unless there

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				referrer or report. Non-medical referral agreements will need to be in place which will require additional training.	is either: uncertainty whether a fracture is present, or concern about a differential diagnosis. Non-medical referral agreements are not part of the guideline scope. It is assumed that the person doing the assessment will have the necessary arrangements in place when requesting imaging.
Society and College of Radiographers	Guideline	11	009	Time since last fragility fracture – time needs defining. When does it need treating?	Thank you for participating in the consultation process. This recommendation relates to the criteria to take into account rather than specific thresholds for treatment. The assessing healthcare professional would need to weigh up all factors when discussing with the person.
Society and College of Radiographers	Guideline	11	016	We are concerned that treating people over 50 with a T-score of -2.5 or less will increase treatment overall (particularly to those with low fracture risk overall and less likely to benefit from treatment) and be less targeted to people with elevated fracture risk. Binary statement - -2.5 but also assessment of fracture risk linked to NOGG also to direct management.	Thank you for participating in the consultation process. The committee agreed that this group are likely to be at a high risk of fracture. To get to this point they will have had a previous fragility fracture or a 10 year risk of major osteoporotic fracture of 10% or more as well as a BMD of -2.5 or less. They also noted that most trials showing that treatment works have been in people with a BMD t score of <-2.5.
Society and College of Radiographers	Guideline	11	017	We are concerned that T-score -1.5 with fracture, steroids or factors associated with bone loss will under treat people with elevated fracture risk. This threshold appears right for ADT and AI treatment, however. Lowering the threshold will also increase DXA demand as there will be more patients that will require a follow up scan for treatment monitoring. Increasing the treatment cohort = increasing follow up scanning cohort. Suggest linking to NOGG.	Thank you for participating in the consultation process. The committee agreed to make the recommendations based on best practice and did not want to make a different recommendation purely because of capacity issues. Also, they note that the government announced 13 extra DXA scanners in

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				Also linking with guidance of breast and prostate cancer.	2025, and a further 20 scanners have been promised in March 2026 to help cut waiting times. The committee are not recommending the NOGG approach therefore a link to that guideline has not been included. We will consider linking to the applicable recommendations in the breast and prostate cancer guidelines for part 2 of this update when recommendations are made for specific pharmacological treatments.
Society and College of Radiographers	Guideline	12	005/006	There is evidence that when seeking advice for 30-50 yr olds the use of Z scores in younger patients as per ROS/ISCD recommendations would be helpful addition	Thank you for participating in the consultation process. The specific BMD score to use is not mentioned in the recommendation and the committee anticipate the healthcare professional assessing the person's risk will choose which score to use.
Society and College of Radiographers	Guideline	19	019	We agree with the inclusion of HRT but with the addition of systemic HRT and not vaginal	Thank you for participating in the consultation process. We have used the term systemic HRT within the recommendation as suggested.
Society and College of Radiographers	Guideline	21	025/026	Recommendation needs to reflect the 6-week threshold to avoid complex two-tier systems for DXA referrals	Thank you for participating in the consultation process. This has been added to the recommendation.
Society and College of Radiographers	Guideline	23	012/013	This rationale states that ' <i>VFA adds a few minutes to a DXA scan and a few additional minutes for reviewing and reporting.</i> ' This is not an accurate representation for all patients particularly those with limited mobility and are very frail. In practice adding a VFA to a DXA takes around 5-7 minutes for mobile patients and up to 10 for less able	Thank you for participating in the consultation process. The committee discussion in Evidence report G has been updated to acknowledge the additional time needed for less mobile patients and complex scans. Further discussion of training needs for implementation has also been added. The text

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				patients, and an additional 5-7 minutes to perform a good quality analysis and report the VFA in simple cases, in cases with multiple vertebral deformities, or further complex spinal pathologies/degeneration can take around 15 minutes to analyse and report. The proposal is suggesting VFA in all over 50s having DXA which is the majority of DXA patients so this will be a significant time burden. Also, VFA analysis requires additional training for both undertaking and reporting. VFA scanning and reporting training is currently not available through any provider. Furthermore, consideration of extra time for patient – patient may have to return for VFA following expert report review of PA/hip DXA scan where the operator is not trained or experienced to identify features on the scan which suggest vertebral fracture.	related to additional time has also been revised in the Rationale and Impact sections of the Short Guideline. The committee rediscussed the impact of new VFA recommendations and agreed to revise them to mitigate the impact from all people having DXA over 50 to men aged 70 and over and women aged 60 and over, with use under these ages limited to those with additional risk factors.
Society and College of Radiographers	Guideline	23	019	Recommendation for using NOGG criteria for considering VFA in practice.	Thank you for participating in the consultation process. We have updated our recommendations to consider a VFA with DXA to a higher age limit of men aged 70 and over and women aged 60 and over as the risk of vertebral fractures increases after these ages. For people below this age our criteria to consider a VFA when doing a DXA scan is similar to the NOGG approach. Both approaches include consideration of height loss, kyphosis, current or frequent use of systemic glucocorticoids, history of back pain with no identified cause and exceptionally low BMD for their age. Please see the updated recommendations about the use of VFA to diagnose vertebral fracture.

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Society and College of Radiographers	Guideline	24	015	This rationale states that '<i>Overall, a net reduction in spinal images is anticipated</i>' – disagree with this statement - to implement broader VFA in more people, more grade 1 fractures will be identified therefore spinal imaging will still be requested to determine if grade 1 fracture or normal variants/ congenital anomalies, therefore increased imaging demand. Furthermore, it has been suggested that the cost of VFA will be saved in the long run as there will less spinal imaging because of implementing VFA however a lot of DXA services do not sit within Radiology services across the country. There is some confusion regarding the tariff costing DXA plus/minus VFA. The VFA needs to be tariffed and resourced appropriately as staff/training costs money etc.	Thank you for participating in the consultation process. The committee agree that there will be an increase in confirmatory spinal radiographs when implementing VFA where currently neither VFA or spinal radiographs are being used to assess for vertebral fracture. However, where spinal radiographs are currently used in some people to assess for vertebral fracture, VFA will mostly displace these. In addition, in some areas where VFA is already in use, confirmatory spinal radiographs are being done in all people with positive VFA. In these cases, the recommendation to only do confirmatory spinal radiographs where there is uncertainty will reduce the number of spinal radiographs. Taking all these effects into account, the committee consider it likely that spinal radiographs will reduce overall in the NHS. DXA costs reported in the guideline are from the NHS National Cost Collection. This does not report separate costs for DXA with and without VFA and so will reflect a mix based on current practice. Note that resource impact is considered for the NHS in England overall, rather than for individual services and the NHS tariff is not within NICE's remit.
Society and College of Radiographers	Guideline	25	027-029	Outdated view - FRAX/NOGG disagree with this by considering all clinical risk factors for fracture. – Need to consider holistic approach at deciding on treatment options rather than T-score driven treatment pathways, appears to be	Thank you for participating in the consultation process. The committee do not think they have overlooked clinical risk factors for fractures. The recommendation included a clinical assessment of risk factors. We have further clarified this to specify that this would include those not specifically included in

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				a lack of consideration for other clinical risks and use of judgement. Acknowledge that access to anabolic therapy is T-score based, however the proposal is incredibly binary and overlooks clinical risk factors for fracture and will be less targeted, personalised management, ultimately not benefiting those who need bone protection.	the risk prediction tools. Please note that we have amended the recommendations to make clear that the shared decision to consider pharmacological treatment should be holistic and consider an individual's clinical risk profile by taking into account: FRAX or QFracture fracture risk and DXA-BMD (if available), number, timing, and skeletal sites of previous fragility fracture, clinical risk assessment of risk factors, including those not included in the relevant risk tool. The committee further specified the factors that should lead clinicians to consider pharmacological treatment, including (though not limited to BMD). For example, it is straightforwardly recommended that treatment for a man over 50, or a women who has experienced menopause, with a previous hip or vertebral fracture should be considered without need for a specific BMD.
The Robert Jones and Agnes Hunt Orthopaedic Hospital	Guideline	8	1.4.3	Rec 1.4.3. We welcome the "fast-tracking" of DXA for those needing anabolic treatment. However, without a clear definition of who is "likely to need" such treatment (e.g., imminent risk), this recommendation may be difficult to implement consistently in primary care.	Thank you for participating in the consultation process. The committee has provided an example in the rationale of criteria that may warrant fast-tracking. However, they do not have the evidence to make a statement for anyone with any recent major osteoporotic fracture and have decided that it should be down to the assessing clinician's judgement. It is expected that the multiple technology appraisal MTA covering osteoporosis pharmacological treatments due to be published next year provide more guidance in this area.
The Robert Jones and Agnes Hunt	Guideline	9	1.5.2	Rec 1.5.2. Regarding the use of "back pain" as a criterion for VFA: Back pain is highly non-specific in the 50+ population. Including this as a standalone trigger for VFA risks	Thank you for participating in the consultation process. The recommendation has been amended to read 'history of back pain or radiating rib pain with no identified cause'.

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Orthopaedic Hospital				overwhelming diagnostic services and the healthcare system with low-yield scans.	
The Robert Jones and Agnes Hunt Orthopaedic Hospital	Guideline	10	1.5.2	Rec 1.5.2. Regarding "physical height loss": We recommend clarifying this criterion with a specific threshold, such as 4 cm or more of height loss. Providing a concrete measurement ensures more objective referrals and better targets those at true risk of vertebral fracture.	Thank you for participating in the consultation process. The committee have added the International Society for Clinical Densitometry (ISCD) criteria of a height loss of more than 4 cm.
The Robert Jones and Agnes Hunt Orthopaedic Hospital	Guideline	11	1.7.2	Rec 1.7.2. The criteria for pharmacological treatment appear to add clinical complexity. Moving away from established NOGG/FRAX-based thresholds towards a fixed 10% risk may cause confusion in specialist referrals for anabolic agents like Romosozumab and Abaloparatide, which currently use different "very high risk" definitions.	Thank you for participating in the consultation process. The committee agree that the use of an age-based threshold in FRAX-NOGG to determine eligibility for pharmacological treatment leads to too few older people being treated and too many young people being treated because there is unclear long-term benefit for these treatments for a relatively small reduction in absolute risk. In contrast, the NICE approach is based on the premise that treatment is best targeted to people at high enough levels of risk who are likely to experience an absolute risk reduction in fracture that outweighs possible adverse events. It also incorporates BMD t score criteria in addition of absolute fracture risk as the committee agreed this is important to ensure treatment is targeted at those most likely to achieve a risk reduction. However, although BMD score is a key criterion they also recommend treatment is considered without the need for a BMD score for people have a previous hip or vertebral fracture, people on very-high dose steroids (around 15mg per day), where the score is not needed for treatment decisions

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					(e.g. elderly people living with frailty who have a few key risk factors, and where doing the DXA is not tolerated or feasible. These recommendations are designed to capture who should be considered for treatment. Categories for very high fracture risk will be dealt with in the multiple technology appraisal covering pharmacological treatments that will be incorporated into part 2 of this guideline.
The Robert Jones and Agnes Hunt Orthopaedic Hospital	Guideline	23	6-22 Rationale (Identifying vertebral fractures)	Rationale (Identifying vertebral fragility fractures). We anticipate that the current wording will lead to a substantial resource impact. Refining the triggers (specific height loss and qualifying back pain) is essential to ensure the NHS can manage the additional staff time and training costs required.	Thank you for participating in the consultation process. The committee rediscussed the impact of new VFA recommendations and agreed to revise them to mitigate the impact from all people having DXA over 50 to men aged 70 and over and women aged 60 and over, with use under these ages limited to those with additional risk factors. The committee agree there will be some increased costs related to VFA, in particular related to staff time for reporting, but there will also be reductions in use of spinal radiography and reduced fractures in the longer term from improved treatment that mean that overall a significant resource impact to the NHS is not anticipated.
The Robert Jones and Agnes Hunt Orthopaedic Hospital	Guideline	25	22–24	Rationale: Deciding whether treatment is appropriate. The guideline states BMD measurements allow clinicians to make "informed choices". This choice is undermined if economic constraints override clinical evidence regarding the efficacy of specific anabolic agents at different skeletal sites.	Thank you for participating in the consultation process. The committee reasoned that information about BMD can be helpful for clinicians when making decisions about treatment. Please note that the efficacy of specific anabolic agents will be addressed in Part 2 of the guideline.

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The Secretariat of the UK National Screening Committee	Guideline and Evidence Reviews	General	General	We appreciate the opportunity to comment on the updated osteoporosis guideline and are grateful to have been kept informed during its development. At this stage, we have no specific concerns regarding the direction of the recommendations. As outlined in Evidence Review A, we would like to emphasise that the introduction of a population-level screening programme falls outside the remit of NICE guidelines. The questions addressed in this update relate to clinical case-finding rather than screening. As NICE is aware, an update to the UK NSC’s recommendation on osteoporosis screening is currently under consideration by the UK NSC’s evidence team. It is important to distinguish that guideline recommendations are primarily intended to support decision-making for individual patient care. The nature and strength of evidence underpinning clinical guidance differs from those required to justify the implementation of a nationally delivered screening programme. We will continue to engage with NICE as both processes move forward.	Thank you for participating in the consultation process and support.
Theramex UK Ltd.	Guideline	007	008	In relation to the risk prediction tools, Theramex would like to recommend clarification (particularly in support of non-specialists) which explicitly acknowledges that FRAX and QFracture do not capture all clinically relevant risk factors (e.g. falls risk and history, recency of fracture, steroidal use etc..). Theramex suggest that it is clear that fracture risk	Thank you for participating in the consultation process. Recommendation 1.3.1 makes clear that a full clinical risk assessment should be conducted alongside the fracture risk assessment tools. The explanation that this is because they do not include all risk factors has been added to the rationale for this recommendation. A full discussion, including of the fact that

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				assessment should emphasise comprehensive clinical judgement alongside, not solely dependent on, bone mineral density (BMD) or a $\geq 10\%$ MOF threshold. (El Miedany Y. FRAX: re-adjust or re-think. <i>Arch Osteoporos</i>. 2020;15(1):150. Published 2020 Sep 28. doi:10.1007/s11657-020-00827-z) (Silverman SL, Calderon AD. The utility and limitations of FRAX: A US perspective. <i>Curr Osteoporos Rep</i>. 2010;8(4):192-197. doi:10.1007/s11914-010-0032-1) Additionally, we suggest clarification to ensure healthcare professionals (HCPs) are aware that patients may not meet the 10% FRAX threshold yet still warrant treatment based on additional risk factors that may not have been accounted for in the risk prediction tools (e.g. falls, steroidal use, recency of fracture) but which increase fracture risk. This aligns (in part) to guidance in Section 1.7.1 outlining criteria for treatment and would ensure that non-specialists can also support equitable access of the appropriate management to all patients.	not all risk factors are included in the tools, is provided in the committee discussion of Evidence review C. The recommendations concerning eligibility for pharmacological treatment make clear that the 10-year risk of MOF threshold of 10% is just one of the factors that should be taken into consideration when making a shared decision to have pharmacological treatment.
Theramex UK Ltd.	Evidence Review B	007	013	Theramex support the recommended guidelines for searching and analysing electronic health records to identify adults who should be assessed for fragility fracture risk. Outcomes in scope include those with prior fragility fracture. However, Theramex support the committee's position that it is important to also identify those who do not have a prior fracture as approximately only 50% of people who sustain a hip fracture have a history of prior fragility fracture as stated in page 7, line 22-23 of Evidence Review B.	Thank you for participating in the consultation process. The committee agree and have recommended research on searching and analysing electronic health records to identify people who should be assessed for fracture risk. The population includes all adults who are 50 years and older which will include people who have not had a previous fracture. It

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					aims to identify people at risk including people who have not had a prior fracture and would otherwise have been missed.
Theramex UK Ltd.	Guideline	007	014	Theramex suggest the inclusion of rationale to justify why it is appropriate to use QFracture when assessing fragility fracture risk in people aged between 30 and 39. Justification may include that the FRAX tool was validated in the UK in a prospective cohort of patients aged 40-85 years (Schini M, Johansson H, Harvey NC, Lorentzon M, Kanis JA, McCloskey EV. An overview of the use of the fracture risk assessment tool (FRAX) in osteoporosis. J Endocrinol Invest. 2024;47(3):501-511. doi:10.1007/s40618-023-02219-9). Theramex suggest NICE provide clear guidance for healthcare professionals (HCPs) around cutoffs for interpreting QFracture, and generally when using any scoring method, whether this be by direct reference to National Osteoporosis Guideline Group (NOGG) guidelines, or a proposition from NICE in particular on the T-score cut-off threshold. This will help to ensure clarity on how cut-offs are related to therapeutic decisions.	Thank you for participating in the consultation process. A brief rationale is provided below the recommendations and a fuller discussion in each of the separate evidence reviews. QFracture was developed and validated in a cohort aged 30 to 90, and this is why QFracture is recommended for people aged 30 to 39. The committee recommend that DXA is considered if the person has a 10-year risk of major osteoporotic fracture of 10% or more from either tool. The subsequent T-score is used to inform the treatment criteria in recommendations 1.7.3. Further detail on treatment decisions will be addressed in Part 2 of this guideline.
Theramex UK Ltd.	Evidence Review C	007	017	Theramex support the research recommendation for CFracture, as it accounts for competing mortality risk and may help guide appropriate treatment selection in older, comorbid populations.	Thank you for participating in the consultation process and for your comment.

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Theramex UK Ltd.	Guideline	008	018	Theramex agree with the requirement to fast-track people likely to require osteoanabolic therapy for a dual-energy x-ray absorptiometry (DXA) scan. However, clarification is required to understand the feasibility and potential impact on local healthcare services owing to limited resource. In the UK parliamentary report on Fracture liaison services (2025), access to diagnostic services varies by region, attributed to a shortage of dual energy X-ray absorptiometry (DEXA) bone density scanners and radiographers. Latest NHS England figures for July 2025 show 56,674 patients were waiting for a DEXA scan, and 13.6% had been waiting more than the targeted six weeks. (Collyer Merritt E. <i>Fracture liaison services: Towards universal coverage in England by 2030</i>. House of Lords Library In Focus. Published September 30, 2025. Accessed February 20, 2026. https://lordslibrary.parliament.uk/fracture-liaison-services-towards-universal-coverage-in-england-by-2030/)	Thank you for participating in the consultation process. The committee noted that not all areas have issues with DXA waiting times but made this recommendation to help areas where there are long delays to help minimise treatment delays. They agreed that ideally DXA capacity should be sufficient for timely assessment and noted that NHS England committed to funding 13 new scanners in 2025 and another 20 new scanners in 2026 – this included a mix of replacement machines and additional machines to increase capacity.
Theramex UK Ltd.	Guideline	009	002	Theramex request clarification on the population that would qualify for requiring ‘antiresorptive treatment if the person is unlikely to need anabolic treatment’. When clarifying that a person is ‘unlikely to require anabolic treatment’, it would be beneficial to explicitly acknowledge evidence demonstrating that starting treatment with antiresorptives can blunt the subsequent response to osteoanabolic therapies.	Thank you for participating in the consultation process. An example has been added to the rationale of someone likely to need anabolic treatment. People more likely to need an antiresorptive rather than an anabolic would be those who meet the criteria but are not high enough risk for an anabolic. This is likely to be more easily defined when the

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				For example: - For individuals assessed as unlikely to require osteoanabolic therapy, clinicians should nevertheless be aware that prior exposure to antiresorptive agents has been shown to attenuate the subsequent response to osteoanabolic treatments, as reported in Cosman et al., 2017. Cosman et al., 2017 suggests that the common practice of switching to osteoanabolic agents only after patients have an inadequate response to antiresorptive agents (e.g., intercurrent fracture of inadequate BMD response) is sub-optimal and may instead result in transient loss in bone mineral density (BMD) and bone strength. Evidence indicates that initiating treatment with an osteoanabolic agent in appropriate individuals before introducing antiresorptive therapy preserves the full osteoanabolic effect and optimises long-term skeletal outcomes. (Cosman F, Nieves JW, Dempster DW. Treatment Sequence Matters: Anabolic and Antiresorptive Therapy for Osteoporosis. J Bone Miner Res. 2017;32(2):198-202. doi:10.1002/jbmr.3051). - For patients meeting criteria for very high fracture risk, treatment pathways should ensure that therapies are sequenced appropriately with osteoanabolic therapy, when considered appropriate, before transitioning to	recommendations from the multiple technology appraisal (MTA) have been agreed The rationale notes that it is not advisable to start an antiresorptive treatment then stop and switch to an anabolic treatment. When the MTA recommendations are finalised they will be incorporated into part 2 of the guideline. The recommendations on treatment criteria in this guideline form the basis of the scope for the MTA. Recommendations on which pharmacological treatment for which group will come out of that MTA.

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				antiresorptives to maintain the gains in BMD. This approach aligns with international guideline trends and reflects the biological rationale that osteoanabolics are most effective when used early and in patients with the greatest fracture risk. This is reflected in the latest recommendations from the International Osteoporosis Foundation (IOF) and European Society for Clinical and Economic Aspects of Osteoporosis, Osteoarthritis and Musculoskeletal Diseases (ESCEO). This approach demonstrates a more rapid and greater fracture risk reduction (Curtis EM, Reginster JY, Al-Daghri N, et al. Management of patients at very high risk of osteoporotic fractures through sequential treatments. Aging Clin Exp Res. 2022;34(4):695-714. doi:10.1007/s40520-022-02100-4). These recommendations are in line with committee's discussions reported in Evidence Review E p46 ('The committee discussed that starting an antiresorptive treatment is not appropriate for people at high risk that may need to start osteoanabolic treatment as it would reduce effectiveness'). Theramex also ask to clarify how this guidance will be incorporated into the Multiple Technology Appraisal (MTA) scope.	

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Theramex UK Ltd.	Guideline	009	013	Theramex support the guideline recommendation for consideration of completing a vertebral fracture assessment (VFA) when doing a dual-energy x-ray absorptiometry (DXA) scan in people aged 50 and over.	Thank you for participating in the consultation process and for your support. The recommendation has been revised from all people over 50 having DXA to men over 70 and women over 65 (check ages in rec), with use under these ages limited to those with additional risk factors. This is to take into account the concern that not everyone over 50 needs a VFA.
Theramex UK Ltd.	Guideline	010	014	The Guideline recommends to “not routinely do spinal imaging after positive vertebral fracture assessment (VFA) to confirm the fracture”. Theramex suggest to add “unless otherwise clinically indicated” to the end of the statement. There may be clinical rationale for performing a subsequent scan (e.g. magnetic resonance imaging (MRI) of the vertebral spine) to delineate for any spinal cord impingement associated with a vertebral fracture.	Thank you for participating in the consultation process. We have updated the recommendation to not routinely do spinal imaging after VFA has confirmed a fracture to include exceptions as suggested. These include uncertainty of the fracture or concern about a differential diagnosis. Further details have been added to the evidence report G committee discussion to provide examples such as a borderline VFA scan or if the fracture could be caused by a different condition to osteoporosis.
Theramex UK Ltd.	Guideline	011	002	Theramex would like to raise a general clarification on how the definitions outlined in the Multiple Technology Appraisal (MTA) scope e.g. on criteria for very high risk, will be translated to the Guidelines. (See Draft scope of Multiple Technology Appraisal: Medicines for treating osteoporosis and reducing the risk of fragility fractures (review of TA160, TA161, TA204, TA464, TA791 and TA991) – Figure 2 page 4)	Thank you for participating in the consultation process. When the recommendations are finalised in the MTA guidance these will be incorporated as written into part 2 of this guideline update.
Theramex UK Ltd.	Guideline	011	002	Theramex would also like to suggest expanding the final criterion in Section 1.7.1 to provide examples of additional high-risk factors (e.g. long-term steroid use, frailty, risk and history of falls or recurrent falls), recognising these as major	Thank you for participating in the consultation process. The bullet point relating to clinical assessment of risk factors in recommendation 1.7.1 has been updated with the following text

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				independent predictors of fracture, particularly in older adults. Theramex understand these are important given that they are not included in the risk prediction tools, and their consideration is relevant for risk stratification	'including those not specifically included in the risk prediction tools'.
Theramex UK Ltd.	Guideline	011	003	The Guideline recommends criteria for treatment based on a 10-year risk of major osteoporotic fragility fracture (MOF). Theramex acknowledge the committee's rationale for preferring a single fixed threshold (simplicity and easier implementation for non-specialists), however, adopting age-dependent intervention thresholds allows for better prioritisation of highest-risk patients, improves equity (especially for younger very high risk patients for whom meeting the 10% fixed threshold is more challenging even though they may be at risk), and enhances cost-effective treatment stratification. Age-dependent risk assessment allows intervention eligibility based on comparable 10-year fracture risk at any age, regardless of prior fracture status, which also aligns with the proposed primary prevention segmentation in the associated Multiple Technology Appraisal (MTA). Fracture risk rises with age and the individuals' biology, trajectory, and urgency of access to appropriate treatment, and varies from a 50-year-old to an 80-year-old presenting with the same risk factors. In alignment with National Osteoporosis Guideline Group (NOGG) guidelines and supporting equity and long-term value of appropriate management approaches, we suggest an age-dependent risk consideration to allow greater	Thank you for participating in the consultation process. The committee agreed that the use of an age-based threshold in FRAX-NOGG to determine eligibility for pharmacological treatment leads to too few older people being treated and too many young people being treated because there is unclear long-term benefit for these treatments for a relatively small reduction in absolute risk. In contrast, the NICE approach is based on the premise that treatment is best targeted to people at high enough levels of risk who are likely to experience an absolute risk reduction in fracture that outweighs possible adverse events. It also incorporates BMD t score criteria in addition to absolute fracture risk as the committee thought this is important to ensure treatment is targeted at those most likely to achieve a risk reduction. However, although BMD score is a key criterion they also recommend treatment is considered without the need for a BMD score for people have a previous hip or vertebral fracture, people on very-high dose steroids (around 15mg per day), where the score is not needed for treatment decisions (e.g. elderly people living with frailty who have a few key risk factors, and where doing the DXA is not tolerated or feasible.

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				individualised accuracy of fracture risk and risk categorisation to guide management. Relevance of the over-simplicity of the 10-year risk threshold may be mitigated by the fact that neither FRAX nor QFracture account for all potential risk fractures and are known to potentially underestimate risk score in some patients.	Note that NOGG thresholds increase with age due to clinical reasons related to the risk of someone of that age with a fracture, and not due to cost-effectiveness considerations.
Theramex UK Ltd.	Guideline	011	004	In addition to the 10-year risk of major osteoporotic fragility fracture, Theramex suggest the inclusion of the 10-year risk of hip fractures that is derived from these risk-stratification tools. Hip fracture risk should not be subsumed within the risk of major osteoporotic fracture owing to the following reasons: a) Hip fracture carries uniquely high mortality that is incomparable to other major osteoporotic fractures (MOF). Specifically, the relative risk of mortality is 5-8 times greater within the first 3 months post-event. In absolute terms, mortality reaches 9% and 36% at month 1 and at 1 year, respectively (Nuti R, Brandi ML, Checchia G, et al. Guidelines for the management of osteoporosis and fragility fractures. Intern Emerg Med. 2019;14(1):85-102. doi:10.1007/s11739-018-1874-2). b) Hip fracture causes disproportionate loss of independence and institutionalisation, with only 52% living in their own home after 120 days, whilst 26% will die within 12 months of their fracture (National	Thank you for participating in the consultation process. The committee noted that hip fracture risk will be calculated with both QFracture and FRAX as well as MOF risk and so will be available to clinicians. They discussed whether to add additional hip fracture risk criteria into the recommendations but decided against doing so to avoid increased complexity. Additionally, in most cases it would not impact decisions. They agreed however that there may be individual circumstances where it would be reasonable to recommend treatment outside the guideline criteria and that this would require clinical judgement.

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				Osteoporosis Guideline Group (NOGG). Section 2: Introduction to osteoporosis and fragility fractures. NOGG Full Guideline. Published 2024. Accessed February 20, 2026. https://www.nogg.org.uk/full-guideline/section-2-introduction-osteoporosis-and-fragility-fractures). Furthermore, 50% of women with hip fracture suffer from a substantial reduction in their level of self-sufficiency which, in approximately 20% of cases, involved long-term institutionalisation (Nuti R, Brandi ML, Checchia G, et al. Guidelines for the management of osteoporosis and fragility fractures. Intern Emerg Med. 2019;14(1):85-102. doi:10.1007/s11739-018-1874-2). c) Hip fracture has a distinct epidemiological and age-related profile. The peak number of hip fractures occur at 75-79 years of age for both sexes, whereas for all other fractures, the peak number occurs at 50-59 years and decreases with age (International Osteoporosis Foundation. Epidemiology of osteoporosis and fragility fractures. Facts & Statistics. Accessed February 20, 2026. https://www.osteoporosis.foundation/facts-statistics/epidemiology-of-osteoporosis-and-fragility-fractures). Therefore, this carries the theoretical risk that in the very elderly, the 10-year risk of MOF probability may be driven by lower-severity fractures occurring earlier in life, whilst the absolute hip fracture	

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				risk continues to escalate. This may lead to scenarios where an older patient may have a borderline MOF probability which fails to meet treatment threshold yet carry a clinically significant and independently actionable 10-year risk of hip fracture probability which does meet treatment threshold. d) National Osteoporosis Guideline Group (NOGG) and international guidelines provide separate intervention thresholds for hip fracture. NOGG guideline explicitly publishes and applies two separate thresholds – one for major osteoporotic fracture (MOF) and one for hip fracture (National Osteoporosis Guideline Group (NOGG). Section 4: Intervention thresholds and strategy. NOGG Full Guideline. Published 2024. Accessed February 20, 2026. https://www.nogg.org.uk/full-guideline/section-4-intervention-thresholds-and-strategy). In the US, the American Association of Clinical Endocrinologists/ American College of Endocrinology Clinical Practice Guidelines for the Diagnosis and Treatment of postmenopausal osteoporosis (2020 update) similarly uses a 3% hip fracture threshold independent of the 20% MOF threshold (Camacho PM, Petak SM, Binkley N, et al. American Association Of Clinical Endocrinologists/American College Of Endocrinology Clinical Practice Guidelines For The Diagnosis And Treatment Of Postmenopausal Osteoporosis-2020	

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				Update. Endocr Pract. 2020;26(Suppl 1):1-46. doi:10.4158/GL-2020-0524SUPPL). Hip fracture causes a disproportionate NHS cost burden – in the UK, cost of fragility fractures to the NHS exceeds £4.7 billion per annum, with £1.1 billion directly attributable to hip fractures alone (National Osteoporosis Guideline Group (NOGG). Section 2: Introduction to osteoporosis and fragility fractures. NOGG Full Guideline. Published 2024. Accessed February 20, 2026. https://www.nogg.org.uk/full-guideline/section-2-introduction-osteoporosis-and-fragility-fractures). Identifying and treating patients at elevated hip fracture risk early is therefore not only a clinical imperative, but also a health-economic one.	
Theramex UK Ltd.	Guideline	011	011-029	Theramex would like to ask clarification on the “if / and” wording in Section 1.7.2. In particular, a previous hip or vertebral fragility fracture listed in Line 15 in Section 1.7.2 appears to already be part of the ‘dual-energy x-ray absorptiometry (DXA) criteria’ which must be met in order to then consider these additional factors (as outlined in 1.7.2) – it could appear confusing. We suggest removing this point in Section 1.7.2 or providing a clarifying comment. Additionally, the guidance in Section 1.7.2 suggests: - if they meet DXA criteria (i.e. Section 1.3.1 they are aged between 50 and 90 and have had a previous hip or vertebral fragility fracture OR 2 or more fragility fractures)	Thank you for participating in the consultation process. The recommendation has been updated and the text ‘if the criteria for a DXA scan are met’ has been removed as suggested.

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				- AND - meet criteria outlined in Section 1.4.1 (i.e. considered bone mineral density (BMD) to guide treatment decisions for people with 10-year risk of major osteoporotic fracture (MOF) 10% or more) - AND - They then meet the T-scores outlined (e.g. -2.5 or less or -1.5 + risk factors...) We recommend simplifying recommendations and reviewing whether provision of such specific T-score cutoffs for patients who have already demonstrated meeting DXA criteria AND having 10% or more MOF risk is beneficial for optimal therapeutic decision-making.	
Theramex UK Ltd.	Guideline	013	017	Theramex support the recommendation that dual-energy x-ray absorptiometry (DXA) scans should not be routinely repeated within 2 years of the person's most recent DXA scan unless there are exceptional circumstances. These exceptional circumstances may include specific clinical indications, such as initiation of treatment likely to affect bone density, a fragility fracture, or a significant change in clinical status.	Thank you for participating in the consultation process and for your comment.
Theramex UK Ltd.	Guideline	026	014	Theramex would like to clarify whether the committee's inability to make specific recommendations for trans and non-binary people, because of the limited information available at the time of development for these groups of people, limits the	Thank you for participating in the consultation process. The committee were unable to make specific recommendations because it is currently unclear whether a trans person's risk and need for subsequent treatment more closely resembles the recommendations related to men or women. Consequently, we

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				applicability of these treatment recommendations across populations.	have added a recommendation for trans or non-binary people on risk assessment and criteria for pharmacological treatment to ensure that sex registered at birth, hormonal history and any current hormone treatment are taken into account.
Theramex UK Ltd.	Evidence Review E	046	043	Theramex support the committee's positioning that starting an antiresorptive treatment is not appropriate for people at high risk that may need to start osteoanabolic treatment as it would blunt the subsequent response to osteoanabolic therapies and reduce effectiveness. Theramex request it be elevated from the rationale text directly into a "Recommendation" within the guideline itself to clarify the importance of the right decision from the outset. If this remains only in the "rationale" section, clinicians and general practitioners (GPs) may miss this important guidance.	Thank you for participating in the consultation process. Recommendations about the use of antiresorptive and anabolic treatments will be addressed in Part 2 of this guideline. This chapter refers to recommendations about DXA scans before starting treatment. Anabolic treatment is mentioned in the current context because patients who need anabolic treatment need DXA before starting. As such, the recommendation concerns the possibility that such a patient may need DXA to be fast tracked so that they can be treated without delay. The recommendation does also state that if there is likely to be a significant delay in doing a DXA scan before treatment, consider starting antiresorptive treatment if the person is unlikely to need anabolic treatment. The rationale section states that it is not advisable to start an antiresorptive treatment and then stop and switch to an anabolic treatment.
UCB Pharma	Guideline	General	General	In the guidelines the role of primary care in identification and management is not clear.	Thank you for participating in the consultation process. It is anticipated that part 1 of the guideline will apply predominantly to primary care as they are usually the first point of contact for patients.
UCB Pharma	Guideline	General	General	Imminent fracture risk within 24 months of the initial fracture should be explicitly referenced in recommendations about who needs urgent assessment, who should bypass routine	Thank you for participating in the consultation process. The committee discussed adding a timepoint for imminent risk but acknowledged that the magnitude of risk varies between

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				risk tools (FRAX) who should have accelerated DXA/imaging/follow up. Imminent fracture risk should be a recognised category that alters urgency, assessment and pathway decisions. To effectively guide clinical teams, the urgency (Imminent Fracture Risk), assessment, decision-making pathways, and the risk of an imminent fracture should be distinctly categorised in the guideline	fracture sites and at different ages. Therefore, this information has been added to the committee discussion in evidence report A but not within the recommendations. There is a recommendation (1.4.1) that people with a previous hip or vertebral or 2 or more fragility fractures should be offered a DXA scan. This recommendation would apply to anyone with these past fractures regardless of when they occurred.
UCB Pharma	Guideline	General	General	Treatment decisions and guideline recommendations must be based on fracture risk which is multifactorial. Risk cannot be explained by BMD alone; therefore, BMD thresholds should not be used as treatment criteria alone.	Thank you for participating in the consultation process. The recommendations have been amended to make clear that the criteria for pharmacological treatment should be based on a holistic consideration of the patient: fracture risk score and BMD if available, but also the number, timing and sites of previous fractures, and other risk factors not included in the risk assessment tool. A visual summary accompanies the recommendations to make this clearer.
UCB Pharma	Guideline	General	General	Very High Fracture Risk (VHFR) is defined consistently across guidelines. Definitions adopt an "if any" qualifying-criteria approach rather than a rigid additive formula. Alignment should therefore reflect this approach.	Thank you for participating in the consultation process. Very high risk will be defined further in part 2 of the guideline where it is likely to affect treatment options and subsequent recommendations. Part 1 of the guideline is aimed at identifying people at risk of fragility fracture.
UCB Pharma	Guideline	General	General	Osteoporosis is a long-term condition which would require explicit sequencing logic, for example anabolic first where appropriate, followed by antiresorptive maintenance to sustain benefit. Alignment should therefore reflect this approach.	Thank you for participating in the consultation process and for the citations. Please note that evidence for the use of anabolic treatments to prevent fracture will be addressed in Part 2 of the guideline.

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				An anabolic first treatment approach in patients at very high or imminent fracture risk is supported within UK guidance by NICE TA791, NOGG clinical and consensus guidance. National Osteoporosis Guideline Group (NOGG). <i>Clinical Guideline for the Prevention and Treatment of Osteoporosis.</i> NOGG; 2024. https://www.nogg.org.uk/sites/nogg/download/NOGG-Guideline-2024.pdf Anabolic first has been recommended by many international guidelines including:- • Spanish Society of Rheumatology on Osteoporosis (2019) Hernández AN, et al. • Rheumatol Clin. 2019;15:188–210; • Spanish Society for Bone and Mineral Metabolism Investigation Riancho JA, et al. Rev Clin Esp (Barc). 2022;222:432–439; • Italian Society for Osteoporosis, Mineral Metabolism and Bone Diseases; , • Italian Society of Orthopaedics; , • Italian Society of Physical and Rehabilitation Medicine; ,	

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				- Italian Society of Rheumatology. Nuti R, et al. Intern Emerg Med. 2019;14:85–102; - Endocrine Society (2020) D, et al. J Clin Endocrinol Metab. 2020;105:1–8; - American Association of Clinical Endocrinologists/American College of Endocrinology Camacho PM, et al. Endocr Pract. 2020;26(Suppl 1):1–46;	
UCB Pharma	Guideline	General	General	Recency of fracture is a strong and independent predictor of subsequent fracture risk, with evidence demonstrating that fracture risk is highest in the first 12–24 months following an index fragility fracture. This excess short-term risk is not captured by standard 10year risk prediction tools, supporting explicit consideration of fracture recency when assessing fracture risk. Kanis JA, Johansson H, Harvey NC, Gudnason V, Sigurdsson G, Siggeirsdottir K, Lorentzon M, Liu E, Vandenput L, McCloskey EV. The effect on subsequent fracture risk of age, sex, and prior fracture site by recency of prior fracture. Osteoporosis International. 2021;32:1547–1555. https://doi.org/10.1007/s00198-020-05803-4	Thank you for participating in the consultation process. Timing of fracture is a factor included as part of recommendation 1.7.1 on the criteria to take into account when discussing pharmacological treatment with the person.
UCB Pharma	Guideline	004	008	'Clinical risk factors' should be included within this section of the guideline.	Thank you for participating in the consultation process. Evidence was not available to make a strong recommendation

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					to assess risk with any risk factor other than previous fragility fractures or glucocorticoid use. Clinical risk factors are included as part of the 'consider' recommendation 1.1.2.
UCB Pharma	Guideline	004	020	With reference to Fracture table 1 there is a need for weighting of the risk factors in terms of fracture risk as is demonstrated in the FRAX Plus tool.	Thank you for participating in the consultation process. It was not possible to weight this table. The committee also noted that FRAX and QFracture take this into account in their risk prediction tools. The aim of this recommendation is to highlight who could be considered for risk assessment.
UCB Pharma	Guideline	007	007	There are evidence and a rationale for including in this section patients who have had a fracture within the last 2 years. - Kanis J. et al. Osteoporosis International (2020) 31:1–12. Should include risk factors for fracture	Thank you for participating in the consultation process. The committee decided to recommend a DXA scan for those who have had a hip or vertebral fracture or two or more fragility fractures at any time. The committee were aware of recent evidence that suggested an increased risk with a previous fracture at any timepoint so did not want to limit this group. Evidence review A on risk factors does discuss imminent risk as an additional increase in the early months post fracture. They noted that it varies between fracture sites and at different ages.
UCB Pharma	Guideline	008	016	Recent fragility fracture defines very high / imminent fracture risk and can justify immediate initiation of bone forming therapy when DXA is unavailable or delayed, with DXA used later for confirmation, monitoring and sequencing.	Thank you for participating in the consultation process. Recommendation 1.7.1 recommends taking timing of fracture into account when making a decision to treat. The recommendation relating to when DXA is unavailable or delayed has been moved to the section on treatment criteria. The committee's understanding is that anabolic treatment cannot be started without a BMD score first. They made the recommendation to fast-track patients who are likely to need an

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					anabolic treatment based on this knowledge to ensure those at very high risk of fracture get appropriate treatment as soon as possible.
UCB Pharma	Guideline	008	018	VHFR patients require treatment not fast-tracking for urgent DXA (treatment selection is based on risk tier and not solely BMD in many guidelines)	Thank you for participating in the consultation process. The committee's understanding is that people with a very high fracture risk are likely to need anabolic treatment which cannot be started without a BMD score first. They made the recommendation to fast-track patients who are likely to need an anabolic treatment based on this knowledge to ensure those at very high risk of fracture get appropriate treatment as soon as possible.
UCB Pharma	Guideline	008	018	Agree with fast-tracking for DXA if anabolic treatment needed. However, fast-tracking for recent MOFs again to identify patients for anabolic treatments should also be included	Thank you for participating in the consultation process. The committee has provided an example in the rationale of criteria that may warrant fast-tracking however, they do not have the evidence to make a statement for anyone with any recent major osteoporotic fracture and have left that for the assessing clinician to decide. It is expected that the multiple technology appraisal MTA covering osteoporosis pharmacological treatments due to be published next year provide more guidance in this area.
UCB Pharma	Guideline	010	014	Spinal imaging advice different to GIRFT VFF pathway which advises AP and lateral spine imaging and or MRI VFF pathway FINAL November 2025	Thank you for participating in the consultation process. This recommendation applies to people having DXA as part a full clinical fracture risk profile, rather than just to those who have symptoms (or where there are signs) of vertebral fracture. As vertebral fractures are often symptomless or missed the committee think this recommendation applies to a different population to the GIRFT guidance for initial presentation.

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UCB Pharma	Guideline	011	003	Imminent fracture risk (IFR) should be explicitly defined as a criterion for treatment, named and operationalised within this part of the guideline with clear implications for prioritisation, assessment approach and care pathways e.g. time since last fragility fracture	Thank you for participating in the consultation process. This recommendation relates to the criteria to take into account rather than specific thresholds for treatment. The assessing healthcare professional would need to weigh up all factors when discussing with the person. We have updated the recommendation from 'the number and skeletal sites of previous fragility fracture' to 'the number, timing and skeletal sites of previous fragility fractures'. We have added some additional text to the rationale to highlight the importance of time since last fracture.
UCB Pharma	Guideline	011	005	Important to mention NOGG thresholds for treatment here.	Thank you for participating in the consultation process. This guideline is not recommending using NOGG and therefore the NOGG thresholds are not mentioned in the recommendations.
UCB Pharma	Guideline	011	011	Treatment decision should include FRAX/ Qfracture risk factors not on BMD alone	Thank you for participating in the consultation process. The committee recommend taking into account all the factors in recommendation 1.7.1 when deciding with the person on whether to treat as well as using the BMD scores noted in recommendation 1.7.3.
UCB Pharma	Guideline	013	004	DXA and VFA should be included as many VFF are missed	Thank you for participating in the consultation process. We did not conduct a review of this but note that the recommendations covering VFA will also apply when reassessing a person's BMD with DXA.
UCB Pharma	Guideline	013	017	Frequency of repeat DXAs should be dependant on treatments – i.e. 12 months for romosozumab and 18 months for abaloparitide. Very important to provide HCP and patient feedback in a 'silent' condition. However, the	Thank you for participating in the consultation process. The committee agreed it would not be appropriate to repeat the BMD before 2 years because any sooner is too short a time period to show a clinically meaningful change in BMD. There

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				availability of DXA should not be a criterion to initiate treatment with an anabolic.	are exceptions to this, including if the person has a high risk of accelerated bone loss, for example if they are using medicines known to cause a reduction in bone density such as high-dose systemic glucocorticoids. The committee's understanding is that an anabolic treatment cannot be started unless a BMD score from DXA is obtained first.
UCB Pharma	Guideline	015	005	The use of QCT or other AI identification tools in BMD measurements should be included to alleviate pressure on access to DXA	Thank you for participating in the consultation process. A research recommendation for QCT has been added. A research recommendation related to other AI tools has not been added as these were not part of the guideline scope.
UCB Pharma	Guideline	016	019	Fracture Liaison Services (FLS) primarily identify individuals with recent or new fractures, as fractures occurring more than one year previously are typically not captured. This presents an opportunity to explicitly embed imminent fracture risk (IFR), reflecting the principle that fracture begets fracture and that fracture risk is highest in the period immediately following an index event.	Thank you for participating in the consultation process. This relates to the initial risk assessment of people at risk of fracture. The recommendation advises that anyone with a previous fragility fracture is assessed regardless of when that fracture occurred. Time since last fracture is covered in the treatment criteria recommendation and the rationale for this has been updated to note that how recently a fracture occurred, particularly a major osteoporotic fracture, is important with the greatest risk of a subsequent fragility fracture being in the first 2 years after an initial fragility fracture.
UCB Pharma	Evidence D	034	005	New evidence shows that TH BMD appears to be the most useful treatment target because it consistently predicts the risk of both vertebral and nonvertebral fractures, whereas the	Thank you for participating in the consultation process. Unfortunately, we were not able to check this comment as no citation was provided.

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				LS BMD level after treatment predicts the risk of vertebral fracture but does not predict the risk of nonvertebral fractures as consistently.	
UCB Pharma	Evidence D	044	022	Concur with the criteria for starting treatment including recency of fracture.	Thank you for participating in the consultation process and for your comment.
University of Southampton	Equality impact assessment	General	General	3.1 Have the potential equality issues identified during the scoping process been addressed by the Committee, and, if so, how? Page 2: ‘Gender reassignment <i>Stakeholder consultation on the EIA noted that hormone blockers used in gender reassignment are associated with bone loss and that transgender men’s risk of osteoporosis may be overlooked if they transitioned at a late age. Taking medicines that are associated with fracture risk is included as a risk factor for fragility fracture. The committee did not make specific recommendations for trans and non-binary people because the information available at the time of development for these groups of people was too limited.’</i> RESPONSE: The decision not to make specific recommendations due to limited evidence is understandable; however, the absence of even high-level operational guidance risks perpetuating inequity in practice. Fracture risk in transgender people may be influenced by biological sex,	Thank you for participating in the consultation process. The committee agree and have made additional recommendations for trans or non-binary people on risk assessment and criteria for pharmacological treatment to ensure that sex registered at birth, hormonal history and any current hormone treatment are taken into account. Thank you for pointing out and providing examples where we have used gendered terms (men and women) across the recommendations in reference to biological sex. We have added the statement below to the EIA to acknowledge this. We have also noted that NICE uses person-centred language, so recommendations use the terms ‘women’ and ‘men’ to reflect how people speak and how they wish to be referred to. Added to EIA: For clarification, in this document, gendered terms, such as women and men, are used widely and intended to correspond to biological sex terms female and male respectively unless otherwise explicitly stated (e.g. trans and non-binary people). This reflects common historical clinical and epidemiological reporting conventions.’

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				timing of transition, duration of endogenous hormone exposure, use of puberty blockers and current gender-affirming hormone therapy. These are clinically relevant variables that could reasonably be acknowledged without overstepping the evidence base. Further the EIA does not clearly distinguish between: i) Biological sex factors that directly influence bone physiology; and ii) Gender identity and lived experience, which may affect access to care, disclosure, and risk recognition. Failure to articulate this distinction increases the risk of confusion in clinical application and may contribute to under-recognition or inconsistent assessment of fracture risk in transgender and non-binary populations. While evidence limitations are appropriately noted, there is scope for proportionate mitigation e.g, explicitly advising clinicians to undertake individualised risk assessment that considers sex assigned at birth, hormonal history and current hormone treatment where clinically relevant. Sex	Race: The committee have recommended QFracture and FRAX risk tools that are validated in UK populations. The committee have recommended that a full clinical assessment is undertaken alongside the risk tools to consider all factors such as race. The committee noted that most of the population are white in the validation studies and therefore the variation for race is likely not to be fully accounted for in the risk assessment tools. We have added this to the equality impact assessment and committee discussion. Socio-economic background: We have added this to the equality impact assessment and relevant committee discussions.

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				<i>To take into account that the risk of fracture is higher for postmenopausal women than men of a similar age, the guideline has made recommendations for risk assessment by age group.'</i> RESPONSE: As written, there is ambiguity regarding whether recommendations are based on biological sex, gender identity, or both (ie the title is 'sex' but women and men are terms indicating gender identity). Overall, while the epidemiological basis for differential recommendations is sound, the language could be strengthened to ensure clinical precision, consistency and alignment with inclusive practice principles. Also, bone is sexually dimorphic across the life span and biases in preclinical knowledge and treatment exist for female bone while male bone biology is less understood. Across the evidence presented and including the Equality Impact Assessment there are inconsistencies in the use of the terms <i>women</i> and <i>men</i>, which at times appear to be used interchangeably to denote both biological sex and gender. This lack of clarity may create ambiguity in interpretation for clinical practice and patient care. Biological sex (female/male) and gender (women/men) are distinct constructs and may have different implications for bone health, fracture risk, epidemiology and treatment response.	

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				A 2025 systematic review of 223 NICE clinical guidelines (Hulme et al, 2025, BMJ “Are sex and gender dimensions accounted for in NICE guidelines? A systematic review of 223 clinical guidelines Public Health 2025” Jul 17;3(2):e002510. doi: 10.1136/bmjph-2024-002510) found that sex and gender are largely under-addressed, with only 41% of non-sex-specific guidelines referencing these factors outside pregnancy and childbirth. Only one guideline provided evidence-based, sex-specific recommendations, underscoring a substantial gap in personalised care. Here, whilst ‘sex’ as a variable has been highlighted, of concern are the inconsistent definitions and interchangeable use of sex and gender terminology. While inclusion of these variables is good practice, imprecise and inconsistent usage risks confusion in clinical application and may undermine care quality, particularly as guidance increasingly needs to account for transgender and gender-diverse populations. The terminology should therefore be defined clearly and applied consistently throughout the guidance and caveats in the evidence presented and highlighted to ensure scientific accuracy, transparency and alignment with NICE equality principles.	

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				We have highlighted example of poor practice here, but this is not exhaustive to these documents but to serve as a guide. Definitions for clarity are provided below. It is recommended that in the EIA there should be a section that clearly defines sex and gender, and the terminology used throughout the documents. An example is provided below Sex Sex refers to biological attributes, including chromosomal, hormonal, and anatomical characteristics, typically classified as male, female, or intersex. Sex should be used where clinical outcomes are directly influenced by biological mechanisms relevant to bone physiology, skeletal development, hormonal status, or treatment response. Where recommendations are based on sex, they should be interpreted as relating to biological sex characteristics relevant to fracture risk rather than gender identity. Gender Gender refers to the social, cultural and experiential aspects of being a man, woman, non-binary person, or another gender identity. Gender should be considered in in relation to	

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				patient experience, communication, access to services and person-centred care. Gender identity Gender identity refers to an individual's internal and personal sense of their gender, which may or may not be the same as the sex assigned at birth. Transgender and non-binary people Transgender and non-binary people are included within the population considered by this guideline. Where the evidence base is limited, the Committee recommends that clinical decisions should be based on individualised assessment. This should take into account relevant biological factors, including sex characteristics, age, hormonal history (for example, puberty suppression, gender-affirming hormone therapy, or menopause), and other established determinants of fracture risk. Page 3 3.3 Have the Committee's considerations of equality issues been described in the guideline for consultation, and, if so, where?	

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				<i>Recommendations have been made by age groups to reflect the increased risk of fragility fracture as people get older, and by sex to reflect the increased risk of fragility fracture in women, particularly following the menopause. This is discussed in the rationales and committee discussions in the evidence reviews.</i> RESPONSE : Specifically, the statement refers to recommendations made “by sex” but then cites “women” as the comparator group. This risks conflating biological sex (clinical determinant of bone physiology) with gender (social/identity-based construct). The increased fracture risk described is primarily attributable to biological factors, notably oestrogen deficiency following menopause. Page 2 Race ‘No evidence was available to suggest different recommendations were needed in the guidelines’ RESPONSE : A body of evidence demonstrates ethnic variation in bone mineral density (BMD), osteoporosis prevalence, and fracture outcomes. Population studies indicate that Asian women frequently have lower areal BMD and may therefore meet DXA-defined osteoporosis thresholds more often; however, hip fracture incidence is generally lower than in White women, suggesting differences in bone geometry and fracture risk calibration tools such as FRAX (Barrett-Connor et al., 2005; Lo, 2023). In contrast,	

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				Black women typically have higher BMD and lower overall fracture incidence compared with White women (Cauley, 2011; Shen et al., 2022), yet disparities have been reported in post-fracture morbidity and mortality, as well as in access to assessment and treatment (Noel et al., 2021). These findings highlight the importance of ensuring that risk assessment tools, diagnostic thresholds, and treatment recommendations are appropriately calibrated and equitably applied across ethnic groups, and that guideline language reflects both biological variation and structural determinants influencing bone health outcomes. Page 2 Socio economic background 'No evidence was available to suggest specific recommendations were needed for this group.' RESPONSE: Evidence indicates that socioeconomic deprivation is associated with lower bone mineral density (BMD) and increased risk of osteoporosis in adulthood (Brennan et al., 2011; Curtis et al., 2017). Population-based studies further demonstrate higher hip fracture incidence and increased post-fracture mortality among individuals living in more deprived areas (Zingmond et al., 2006; Wilkinson et al., 2019). In addition to biological risk, inequalities are observed in access to bone health services, including lower rates of DXA assessment, reduced initiation and persistence with anti-osteoporosis medication, and variable access to coordinated secondary fracture prevention services in	

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				deprived populations (Curtis et al., 2017; Public Health England, 2017). Early-life socioeconomic disadvantage may also influence peak bone mass attainment and lifelong fracture susceptibility (Curtis et al., 2017; World Health Organization, 2007). Deprivation is a recognised driver of health inequalities and may intersect with protected characteristics such as age, ethnicity, and disability. Consideration should therefore be given to ensuring equitable access to fracture risk assessment, diagnostic investigation, and secondary fracture prevention pathways, particularly in areas of high deprivation, to avoid widening health inequalities (Curtis et al., 2017; Public Health England, 2017).	
University of Southampton	Evidence C	General	General	This evidence details that sex is used in the tool; however, the non-appendices section frequently uses gender terminology. For example, the tables 5-15 have a heading title for sex but use gender terms men and women .	Thank you for participating in the consultation process. The statement below has been added to the equality impact assessment form to address the interchangeable use of gender terms and biological sex terms throughout the documents. Added to EIA: For clarification, in this document, gendered terms, such as women and men, are used widely and intended to correspond to biological sex terms female and male respectively unless otherwise explicitly stated (e.g. transgender, non-binary). This reflects common historical clinical and epidemiological reporting conventions.

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University of Southampton	General	General	General	From reading the documents, it is apparent that biological sex and gender terminology are used interchangeably, even though the impact of each characteristic on bone health differs. Using the terms interchangeably creates confusion when interpreting from a biological research perspective, as the conclusions drawn will differ depending on whether the samples are sex- or gender-linked. To address this and similar examples of such practice across the evidence and literature the following statement could be used in the EIA: <i>'For clarification, in this document, gendered terms, such as women and men, are used widely and intended to correspond to biological sex terms female and male respectively unless otherwise explicitly stated (e.g. transgender, non-binary). This reflects common historical clinical and epidemiological reporting conventions'</i>.	Thank you for participating in the consultation process. We agree and have added this statement to the EIA form.
University of Southampton	Evidence D	6	10-13	Studies in which the cohort was a general 10 unselected population were included if they were high risk as defined by the previous NICE 11 Osteoporosis guideline CG146 (published 2012), that is, if the mean age of the participants 12 was ≥65 years for women or ≥75 years for men. If a study did not report age by sex, then it 13 was excluded. RESPONSE: Use of gendered terms women and men and then biological sex.	Thank you for participating in the consultation process. The statement has been added to the equality impact assessment form to address the interchangeable use of gender terms and biological sex terms throughout the documents. Added to EIA: For clarification, in this document, gendered terms, such as women and men, are used widely and intended to correspond to biological sex terms female and male respectively unless otherwise explicitly stated (e.g.

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					transgender, non-binary). This reflects common historical clinical and epidemiological reporting conventions.

**None of the stakeholders who comments on this clinical guideline have declared any links to the tobacco industry.*